



ANNUAL REPORT

PINALAWAK AT MGA BAGONG BENEPISYO PARA SA MAMAMAYANG FILIPINO

VISION

“Bawat Filipino, Miyembro. Bawat Miyembro, Protektado. Kalusugan ng Lahat, Segurado.”

MISSION

“Benepisyong Pangkalusugan, Sapat at Dekalidad para sa Lahat.”

CORE VALUES

Integridad, Inobasyon, Agarang Serbisyo, Taus-pusong Paglilingkod, Pagmamalasakit, Angkop na Benepisyo, Panlipunang Pagkakabuklod.

Pinalawak at mga Bagong Benepisyo para sa Mamamayang Pilipino

In 2013, the Philippine Health Insurance Corporation (PhilHealth) transitioned from a fee-for-service model to a new payment mechanism known as All Case Rates (ACR). However, due to various factors, no significant increases or enhancements have been made to ACR since then.

The COVID-19 pandemic presented numerous challenges to improving healthcare services and enhancing access to health services, highlighting the need to revisit and rationalize existing benefit packages. In November 2023, the Corporation issued PhilHealth Circular (PC) No. 2023-0025, titled "Guiding Principles of the Rationalized Inpatient Case Rates," as a precursor and major step in enhancing its benefit packages. This issuance essentially lays the framework for improving financial coverage in healthcare delivery as the Corporation transitions its provider payment mechanism to Diagnosis-Related Groups (DRGs).

As prescribed by the Universal Health Care (UHC) Law, PhilHealth strives to provide a comprehensive, quality, and cost-effective set of promotive, preventive, curative, rehabilitative,

and palliative health services without causing financial hardship. PhilHealth's initiatives are also guided by other existing laws (e.g., Senior Citizen Act, National Integrated Cancer Control Act, Mental Health Act), burden of disease, conditions that lead to catastrophic health spending, and existing benefit packages of the National Health Insurance Program. These improvements are subject to periodic review.

Enhancements of existing benefits include rationalized packages for Pneumonia High Risk, Z Benefits for Colon and Rectal Cancer, and Orthopedic Implants. Additionally, the development of new benefits such as the Outpatient Mental Health package, Konsulta with SDGs, and PhilHealth GAMOT have led to improvements in facilities and an increase in healthcare providers, as evidenced by the accomplishments detailed in this Annual Report.

With this, PhilHealth remains committed to providing adequate and quality health insurance for all.

CONTENTS

Message from the President of the Philippines	2
Message from the Board Chairperson	3
Message from the President and CEO	4
PhilHealth's 28th Year Accomplishments	6
Corporate Governance Statement	10
Commission on Audit	12
Statement of Management's Responsibility for the Financial Statements	15
Statements of Financial Position	16
Statements of Comprehensive Income	17
Statements of Changes in Equity	17
Statements of Cash Flows	18
Notes to Financial Statements	19
Board of Directors	47
Executive Officers	52

FROM THE PRESIDENT OF THE PHILIPPINES



I express my sincerest congratulations to the Philippine Health Insurance Corporation (PhilHealth) as it publishes its 2023 Annual Report.

This publication narrates the institution's dedication to providing accessible and affordable healthcare services and transforming communities across the nation. In these pages, we see their efforts to ensure that no individual is left behind in times of medical need.

Each achievement outlined serves as a milestone in PhilHealth's journey towards creating a healthier and more equitable society. The report also allows us to see how the organization's past endeavors remain connected with its future aspirations, and how it continues to set the stage for a transformative evolution in healthcare delivery.

As we reflect on the feats of PhilHealth and the challenges that lie ahead, let us embrace a spirit of hope and solidarity. By fostering a culture of collaboration and inclusivity, I have faith that we can usher in a new era for Philippine healthcare where the commitment to dignity, compassion, and quality lead us to a Bagong Pilipinas and a better future for all.

I wish you the best in your noble pursuits.

A handwritten signature in black ink, appearing to be 'F. Marcos Jr.', written over a diagonal line.

Ferdinand "Bongbong" Marcos Jr.
President Republic of the Philippines

FROM THE OFFICE OF THE SECRETARY



As we enter another chapter in the journey toward achieving Universal Health Care in the Philippines, let us reflect on the milestones, challenges, and successes of PhilHealth in 2023.

This Annual Report highlights the significant steps needed to transform our healthcare system, ensuring that every Filipino, in every corner of our archipelago, feels the impact of our efforts: *"Bawat Pilipino, ramdam ang kalusugan."*

Healthcare financing should demonstrate our commitment to equitably pay for health services to improve the well-being of our people. By increasing the service and cost coverage of our benefit packages and expanding access, we continue to bring quality healthcare within reach of many Filipinos.

Our health sector's 8-Point Action Agenda guides us toward a stronger and more equitable health system. We have worked tirelessly to expand service and financial coverage, improve health service delivery, and ensure that our resources are utilized efficiently and effectively. These efforts emphasize our dedication to making health care a reality for every Filipino, regardless of their socio-economic status.

Looking ahead, we are committed to furthering these efforts. We will continue to innovate, collaborate, and enhance the necessary benefits that will propel us towards our vision of Universal Health Care.

As Chairperson of the PhilHealth Board of Directors, I assure you that our focus remains steadfast: to build a health system that not only meets the present needs of our people but also anticipates future demands. After all, the job of PhilHealth is to pay the health benefits of its members.

In the coming years, we shall work tirelessly to uphold our promise to every Filipino—to make Universal Health Care not just an aspiration, but a lived reality. *Dahil sa Bagong Pilipinas, Bawat Buhay Mahalaga!*


Teodoro J. Herbosa, MD
Secretary of Health



FROM THE PRESIDENT AND CEO

My heartfelt thanks goes out to all the tireless, committed, and selfless PhilHealth workforce for their contributions to the country's relentless pursuit of Universal Health Care (UHC). We are now living a once elusive dream — a healthy nation that is deliberately liberated from the burden of high health care costs.

The Corporation has again defied odds and launched new and enhanced benefits in 2023. Despite previous suspension of premium adjustments, last year proved to be a defining moment for us.

Now, Chronic Kidney Disease Stage 5 patients have been effectively guaranteed a year-round coverage for hemodialysis, alleviating worries of benefit exhaustion leading to out-of-pocket expenses. The previous 90-session coverage was expanded to 156 to a maximum annual benefit of P405,600.

PhilHealth further asserted itself as the country's premiere health insurer when it increased the benefit packages for some of the most claimed and burdensome cases.

Coverage for high-risk pneumonia was amplified nearly three-folds, while coverage for ischemic and hemorrhagic stroke were increased at an average of 138% between them.

Mental health patients were also finally given the long overdue attention due them. We all know too well the emotional stress that the disorder brings to the patients, their family, and community—apart from the economic distress caused by treatments and medicines.

PhilHealth now offers annual outpatient benefit packages that cover general and specialized mental health care ranging from P9,000 to P16,000. Patients needing treatment for mental conditions can now afford consultations, services, and medicines.

Not only PhilHealth empowers them financially, patients are now enabled to live their lives to the fullest.

All these enhancements have been carefully studied, purposely designed, and timely executed all in favor reaching new horizons of health care financing, thus, propelling UHC into new heights. But we are not yet done, are we?

The year 2024 promises to be another exciting year for the National Health Insurance Program. More new and enhanced benefit packages will be released, development of fresh and improved systems will unfold, and a restructured organization will surely support the Corporation's grand plans for our members and stakeholders—all in an effort to greatly contribute to the founding of a Bagong Pilipinas! Mabuhay tayong lahat!



Emmanuel R. Ledesma, Jr.

President and Chief Executive Officer



2023 OPERATIONAL HIGHLIGHTS

PhilHealth's CY2023 Highlights of Accomplishments



PhilHealth has consistently assisted its members over the years, keeping committed to ensuring that every Filipino has access to the full range of necessary, high-quality healthcare services. PhilHealth, guided by its mandate under the National Health Insurance (NHI) Act, the Universal Health Care (UHC) Act, NEDA's Ambisyon 2040 vision and Philippine Development Plan, and the DOH's National Health Sector Strategy, envisions a universal healthcare system that protects the poor and underprivileged from the high cost of healthcare services.

Despite the Covid-19 pandemic making the last three years challenging, PhilHealth persevered and ensured that UHC and its aims gained traction. PhilHealth remained ever receptive to Filipinos' healthcare needs through continued membership coverage and fast payment of claims benefits, never losing sight of its goal of ensuring "Bawat Filipino, Miyembro; Bawat Miyembro, Protektado; Kalusugan ng Lahat, Segurado".

At his 2024 State of the Nation Address, President Ferdinand R. Marcos Jr. discussed the need for a more comprehensive healthcare system. He underlined the need of bringing medical treatment closer to the people rather than making them attend hospitals or drive long distances to receive it. In response to the directive, PhilHealth increased its accreditation efforts and expanded its primary care coverage to ensure that every Filipino, including those in distant areas, had access to medical treatment.

Population Coverage

All Filipinos Covered

The adoption of the Universal Health Care (UHC) program resulted in automatic insurance coverage for all Filipinos through the National Health Insurance (NHI) Program, with PhilHealth obtaining a 100% coverage rate. The Philippines' expected population in 2023 was 112,892,781, which means that approximately 113 million Filipinos now have secured and rapid access to PhilHealth benefits because of UHC.

The Corporation regularly tracks the Registration Rate of NHIP beneficiaries recorded in the PhilHealth membership database to ensure comprehensive enrollment of all Filipinos. As of December 31, 2023, the estimated population of Filipinos stands at 112,892,781, based on the updated projections from the 2015 Population Census by the PSA, with 96% of this population is now registered with PhilHealth. A detailed breakdown of the 108.5 million registered individuals is provided below.

Registered Members and Dependents

As of December 31, 2023

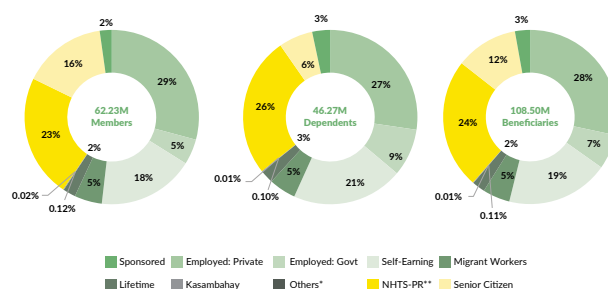
Membership Category	Members	Dependents	Beneficiaries (Total)
Direct Contributors	36,895,531	29,770,581	66,666,112
Employed: Private	18,115,215	12,645,478	30,760,693
Employed: Government	2,936,619	4,081,707	7,018,326
Informal / Self-Earning	11,132,556	9,542,930	20,675,486
OFWs / Migrant Workers	3,173,450	2,310,831	5,484,281
Lifetime Members	1,453,692	1,138,628	2,592,320
Kasambahay	74,462	46,815	121,277
Others*	9,537	4,192	13,729
Indirect Contributors	25,340,992	16,498,063	41,839,055
Indigents / NHTS-PR**	14,168,031	12,022,852	26,190,883
Senior Citizens	9,651,725	2,931,958	12,583,683
Sponsored Program	1,521,236	1,543,253	3,064,489
Total	62,236,523	46,268,644	108,505,167

Note/s:

* Includes Family Drivers, Filipinos w/ Dual Citizenship, Naturalized Filipino Citizens, PRA Foreign Retirees, Citizens of other Countries working / residing / studying in the Philippines

** Indigent count of members and dependents are based on DSWD LisTahanan database and are subject for further validation.

Distribution by Members, Dependents & Combined



Sustaining the Program: Premium Collections

The expected premium increases for 2023 were postponed, which definitely hampered PhilHealth's collection operations and may have negatively impact UHC deadlines. Despite this change, PhilHealth was able to collect more than Php 237 billion in premium payments for 2023.

This represents a 9.4% increase above the Php 216 billion collected (as restated) in 2022. The distribution of the Php237 billion is as follows: Direct Contributors accounted for approximately 67% of the total, while Indirect Contributors account for 33%.

Premium Contributions

January 1 to December 31, 2023

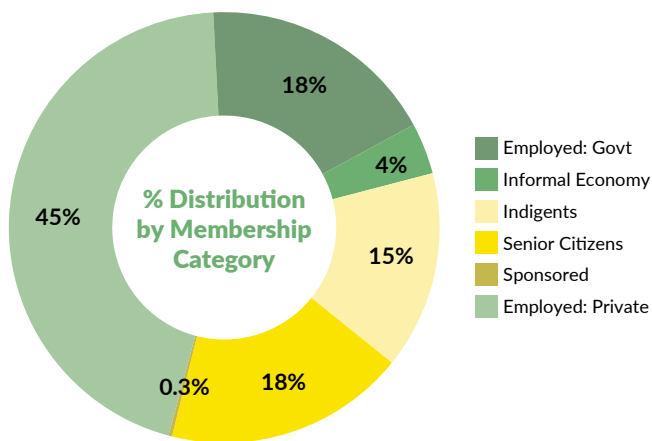
Distribution by Membership Category

Membership Category	Amount
Direct Contributors	Php 157,353,034,317
Employed: Private	105,907,437,477
Employed: Government	42,302,905,260
Informal Economy	9,142,691,580
Indirect Contributors	Php 78,805,615,998
Indigents / NHTS-PR	35,256,167,000
Senior Citizens	42,931,355,000
Sponsored*	618,093,998
Total	Php 236,158,650,315

Note/s:

*Includes Special Government Programs (PAMANA and Bangsamoro), NGAs, POS, PWD-unemployed, LGU, etc.

Source: 2023 Financial Statements as of December 2023



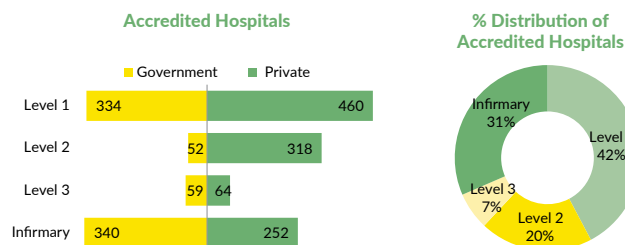
Guaranteed Availability of Healthcare Services

The principle of assuring the availability of healthcare services states that every Filipino has the right to timely access to necessary medical care whenever they need it, regardless of geographic location or provider availability. This notion is critical to developing a fair and successful healthcare system. As a result, certified healthcare facilities must be widely accessible in order to provide critical medical treatments when needed.

In 2023, PhilHealth accredited 11,904 healthcare facilities across the country, representing a 9% increase from the 10,897 healthcare facilities accredited in 2022. PhilHealth's accredited healthcare facilities include the following:

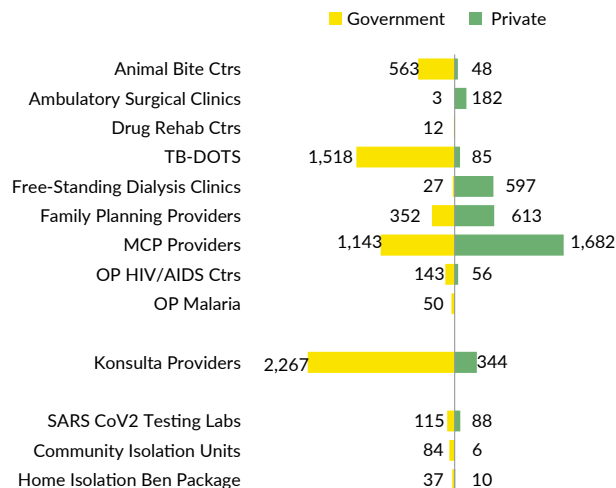
- Hospitals
- Infirmaries/Dispensaries
- Animal Bite Centers
- Ambulatory Surgical Clinics
- Drug Rehabilitation Centers
- Free-Standing Dialysis Clinics
- Family Planning Providers
- Konsulta Providers
- Maternity Care Package Providers
- Outpatient HIV/AIDS Centers
- Outpatient Malaria Providers
- TB-DOTS Providers
- Covid-19 Testing Laboratories
- Covid-19 Community Isolation Units
- Covid-19 Home Isolation Benefit Package

In 2023, PhilHealth accredited a total of 1,879 hospitals nationwide. Of these accredited facilities, 58% are private hospitals, while the remaining 42% are public institutions.



In addition to hospitals, PhilHealth has accredited 10,025 healthcare facilities that offer specific benefits and services, which includes 2,611 accredited Konsulta Providers.

Other Accredited Facilities

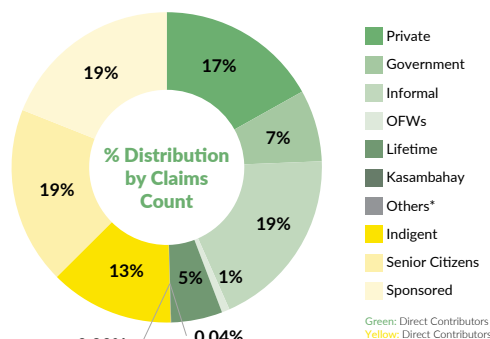


In 2023, the overall number of accredited healthcare professionals was 49,179, with physicians numbering 45,150 of that total. The remaining 4,029 professionals comprised 716 dentists, 103 nurses, and 3,210 midwives.

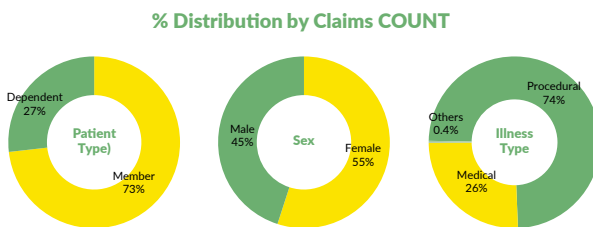
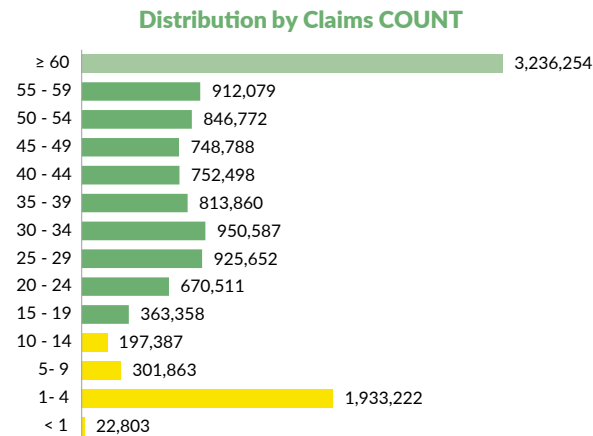
Prompt Claims Processing

PhilHealth is committed to swiftly processing legitimate hospital claims that comply with relevant laws and regulations. In the realm of claims disbursement, a total of 12,675,634 claims were settled in 2023, resulting in an expenditure exceeding Php122 billion. The total claims paid to Indirect Contributors experienced a modest rise, accounting for 50.4%, whereas the claims payment to Direct Contributors stood at 49.6%.

Membership Category	Claims Amount	Claims Count
Direct Contributor	Php 62,045,524,816	6,287,618
Employed: Private	22,021,316,640	2,139,785
Employed: Government	8,829,831,816	949,652
Informal/Self Earning	22,232,262,203	2,409,192
OFWs/ Migrant Workers	1,153,327,993	104,029
Lifetime Members	7,720,793,661	676,129
Kasambahay	53,940,889	4,717
Others*	34,051,613	4,114
Indirect Contributor	Php 60,337,478,276	6,388,016
Indigent/NHTS PR	14,303,917,078	1,638,248
Senior Citizen	26,279,897,266	2,344,525
Sponsored	19,753,663,932	2,405,243
Total	Php 122,383,003,091	12,675,634



Distribution of Claims Paid by Age Group



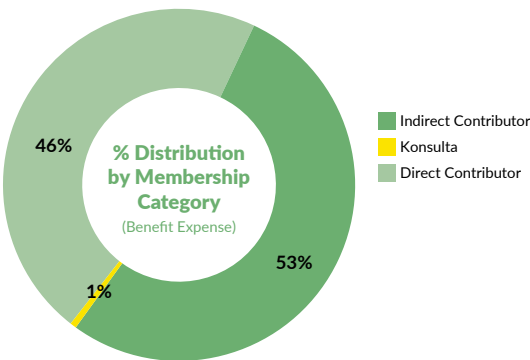
Benefit Expense

January 1 to December 31, 2023

Benefit Claims Expenses for 2023 are recognized at the time of admission. The decrease in benefit expense is due to the adjustment made for Incurred But Not Yet Paid (IBNP) as computed by Actuary. The adjustment is a change in accounting estimate and per PAS 8, change in accounting estimate shall be taken up on the period of change. Benefit expense before the adjustment is at 116.6B. After affecting the adjustment of 40.84B, it is now at 75.77B.

This account is composed of the following:

Distribution by Membership Category	
Membership Category	Amount
Members' Benefits for Direct Contributors	Php 34,473,633,624
Members' Benefits for Indirect Contributors	Php 39,303,979,133
Konsulta	Php 429,575,508
Total	Php 74,207,188,265



CORPORATE GOVERNANCE STATEMENT



“...A Better and More Efficient PhilHealth”

The 2023 Board Accomplishment Report

As the applauses from President Marcos Jr.'s praises of PhilHealth in his SONA subsided, our commitment and dedication to serve the Filipino people only intensified. As your partner in health, we promised to deliver quality healthcare service to every Juan and Juana of every corner of our country, and on that we shall deliver. We remain committed to our mandate of providing all Filipinos with equitable access to adequate, affordable, and acceptable health care services, anytime and anywhere they need them.

This year, we conducted 19 Board Meetings and passed 106 Board Resolutions to support PhilHealth Management, provided guidance in the enhancement of PhilHealth policies, and further improved PhilHealth's services. With these, rest assured that we will always put you—our partners in health—at the center of our decision-making processes.

“Pinalawak at mga Bagong Benepisyo para sa Mamamayang Filipino”

As our country steadily recovers from the effects of the pandemic, we gradually shifted our focus from COVID-19 benefits to the improvement of our other existing benefit packages, including the introduction of new case rate packages.

With the advancements in healthcare delivery systems, we made sure that PhilHealth remains up to date in its policies. The PhilHealth Board approved the Guiding Principles for Integrated UHC Benefits, including the Provider Payments Reform Sandbox Sites to conduct pilot implementation in determining optimal forms of health financing policies for national scale-up and to ensure effective financial coverage for a comprehensive and proper continuum of care. In line with these efforts, the Board also approved the Guidelines for the Contracting of Public Health Care Provider Networks to streamline the payment for the delivery of individual-based health services through prospective payment mechanisms.

To improve financial risk protection for our members, we looked into the enhancement of benefit packages which account for the highest percentage of claims, and determined that among these claims, high-risk pneumonia and acute stroke are among the priority conditions in rationalizing or improving the All Case Rates. In response to these findings, we approved the increased coverage for these diseases. PhilHealth now reimburse up to Php 90,100 for High-risk Pneumonia, Php 76,000 for Acute Stroke (Ischemic), and Php 80,000 for Acute Stroke (Hemorrhagic). With this increase in financial coverage, we hope to reduce the out-of-pocket expenditure of our members and their dependents.

With the introduction of social distancing measures and lockdowns in reducing COVID-19 transmission, incidences of loneliness, anxiety, and depression skyrocketed. This and the heightened focus on public health highlighted the importance of mental well-being as an integral part in the holistic approach to health. PhilHealth recognizes this need and addressed the clamor of mental health institutions to provide coverage for outpatient mental health benefits. Thus, we approved the Outpatient Mental Health Benefit Package covering up to Php 9,000 per year for General Mental Health Service and up to Php 16,000 per year for Specialty Mental Health Service.

Along with our other outpatient benefit packages, we also introduced this year our coverage for most common drugs used in primary care as well as those included in our KonSulTa benefit. This program, called PhilHealth Guaranteed Accessible Medications for Outpatient Treatment (GAMOT), aims to decrease out-of-pocket spending for medications used in outpatient disease management as part of our mandate under the UHC Act.

After receiving positive feedback from our members and stakeholders, we also resolved to institutionalize the increase in the number of reimbursable Hemodialysis Session from 90 to 156 sessions starting CY 2023. The expansion and institutionalization of this benefit was made possible with the help of funding from PAGCOR and PCSO. With this, patients on hemodialysis can avail themselves of as much as P405,600 per year at P2,600 per session. The enhancement of coverage for this life-saving treatment for patients with kidney failure is a welcome change in our benefit plans but PhilHealth remains committed to advocating for peritoneal dialysis (PD) as the initial line of treatment for these patients. These packages are only a few of the benefit expansions we have in the pipeline. Have no doubt that we will continue to re-evaluate and enhance our current benefits as well as introduce new ones to be more responsive to the needs of our members, to support our mandate to provide financial risk protection, and to achieve universal health coverage.

Safeguarding the Fund through Effective Financial Management and Proactive Anti-Fraud Measures

The National Health Insurance Fund is composed of various sources of funding. These include member contributions, government subsidies, and budget appropriations among others. As the administrator of the fund, it is our responsibility to ensure that any amount spent ultimately lead to better and more responsive services for Filipinos, down to the last peso. We meticulously examined the Corporate Operating Budget for CY 2023 and subsequently approved the same after thorough deliberation and consultation with financial experts within the Board. With this, we guaranteed sufficient funding for our operations, programs, and projects throughout the year.

As mandated under RA 11223, funding from PCSO and PAGCOR earnings will be utilized in the enhancement of our benefit packages. Thus, we approved the budgetary requirement for submission to the Department of Budget and Management amounting to Php 72.88 billion for the purpose. This funding is expected to boost our fiscal space and further enhance our existing benefits in our pursuit of universal health care.

Finally, to protect PhilHealth's fund from unlawful and illegal activities within the healthcare system, the PhilHealth Board, in the exercise of its quasi-judicial powers, resolved a total of 18 administrative cases against erring healthcare providers. It is our hope that by holding accountable those who defraud the National Health Insurance Program, we will be able to stave off such incidences and instill in our stakeholders our core values of integrity and accountability.



Republic of the Philippines

COMMISSION ON AUDIT

Commonwealth Avenue, Quezon City, Philippines

INDEPENDENT AUDITOR'S REPORT

THE BOARD OF DIRECTORS

Philippine Health Insurance Corporation
Citystate Centre Building
709 Shaw Boulevard
Pasig City

Report on the Audit of the Financial Statements

Qualified Opinion

We were mandated to audit the Financial Statements (FSS) of the **Philippine Health Insurance Corporation (PhilHealth)**, which comprise the Statements of Financial Position as at December 31, 2023 and 2022, and the Statements of Comprehensive Income, Statements of Changes in Equity and Statements of Cash Flows for the years then ended, and Notes to the FSs, including a summary of significant accounting policies.

In our opinion, except for the matters discussed in the *Bases for Qualified Opinion* section of our report, the accompanying financial statements present fairly, in all material aspects, the financial position of **PhilHealth** as at December 31, 2023 and 2022, and its financial performance and cash flows for the years ended, in accordance with Philippine Financial Reporting Standards (PFRS).

Bases for Qualified Opinion

The faithful representation in the FSS of Provision for Insurance Contract Liabilities (ICL) amounting to P1.150 trillion and P266.873 billion for the Calendar Years (CYS) 2023 and 2022, respectively, cannot be ascertained due to the following factors: a) the completeness and accuracy of the claims and premium data cannot be established, casting doubt on the reliability of the reported figures, b) the evaluation of the CY 2023 Actuarial Valuation Report, intended to assess the adequacy of the ICL, revealed inconsistencies and irregularities that affects its reliability and validity to third parties, c) the use of different measurement bases for the Provision for ICL diminishes the comparability of information in the FSs, hindering the users from making informed economic decisions, and d) the absence of reasonableness test on expense loading further undermines the reliability of the ICL account in the FSs, which are inconsistent with Philippine Accounting Standard (PAS) 1 Presentation of FSs, Philippine Financial Reporting Standard (PFRS) 4 Insurance Contracts, and Revised Conceptual Framework for Financial Reporting (CFFR).

Likewise, the completeness and accuracy of the Contributions and Premium Receivable, Members' Contributions, and Fines & Penalties - Business Income accounts, amounting to P48.538 billion, P236.159 billion, and P700.502 million, respectively, could not be reasonably ascertained due to the non-recognition of an estimated P2.295 billion unremitted income and undetermined amounts of corresponding interests and surcharges from 184,434 non-remitting operational employers. This deprives PhilHealth of the funds necessary to carry out its mandate, specifically in the administration of the National Health Insurance Program, and affects the faithful representation of account

balances in the FSs, which is not in accordance with Paragraph 15 of PAS 1 - Presentation of FSs and Paragraphs 2.12 and 2.13 of the Revised CFFR.

Similarly, the faithful representation of the Premium Contributions - Direct Contributor account in the FSs, with year-end balance of P157.353 billion, cannot be ascertained due to the following deficiencies, contrary to the requirements of fair presentation outlined in PAS 1 and CFFR: a) the Statement of Premium Accounts transacted online for CY 2023, amounting to P90.349 billion, cannot be validated due to the unavailability of the repository database and restrictions in access to the Electronic Premium Reporting System, PhilHealth Members Portal, and Electronic Collection Reporting System. These limitations hinder the verification of amounts paid by members, restrict audit capabilities, and prevent the determination of the accuracy and existence of remitted collection by Accredited Collecting Agents; b) the non-sequential series number of ePAR posted in reports per Treasury Data Editing Module impose limitations on the audit process, hindering the validation of the completeness of collections from ACAS transacted online; and c) control deficiencies within the collection process further contribute to the uncertainty surrounding the accuracy and reliability of the Premium Contributions - Direct Contributor account, which highlight weaknesses in the internal control systems and procedures designed to safeguard the integrity of the collection process.

Further, the faithful representation in the FSS of Provision for Health Benefits Incurred But Not Yet Received (IBNR) account amounting to P57.507 billion for the CY 2023 is unreliable due to several issues: a) differences in the components of Incurred But Not Paid (IBNP) liabilities presented by the Comptrollership Department and Actuarial Consultant, such as the inclusion of Claims Handling Expenses and Provision for Denied Claims, diminish the reliability and comparability of information in the FSs, thus hindering the users from making informed economic decisions, and b) in PhilHealth Regional Office (PRO) NCR and Rizal, the belated tagging for the Motions for Reconsideration/Appeals received during CY 2023 for the previously denied claims resulted in the overstatement of the Provision for Health Benefits - IBNR account by an unidentified amount. These deficiencies are not in accordance with the relevant provisions of PAS 1 Presentation of FSs and Revised CFFR, which emphasize the faithful presentation of information in the FSs.

Meanwhile, the Premium Receivable from Direct Contributors, with a year-end balance of P45.365 billion, net, was overstated due to the improper recognition of collection of premiums with an unidentified amount arising from the mandated one percent premium increment for CY 2022 as prescribed under the Universal Health Care (UHC) Act, which is inconsistent with the requirements set forth in PAS 1 and the Conceptual Framework for General Purpose Financial Reporting (CFGPR), emphasizing the importance of faithfully representing transactions in the appropriate accounting period. Likewise, the balance

of the account was further undermined by an increase of probability of default risk on premium receivables due to the deferred collection of increments for CYS 2021 and 2023 amounting to P15.156 billion and P16.219 billion, respectively, and an unidentified amount in CY 2022. Thus indicates a lax collection strategy on the part of PhilHealth, which is not in accordance with the Republic Act No. 11223 or the UHC Act and the Revised Implementing Rules and Regulations of the National Health Insurance Act of 2013.

Also, the faithful representation in the FSs of Financial Liabilities - ABP-ICS, amounting to P29.817 billion for the CY 2023 cannot be ascertained due to the different approaches by the Actuarial Consultant and the Comptrollership Department in the computation of ABP-ICS, with respect to claims and the periods covered, resulted in a significant difference of P24.929 billion. These discrepancies diminish the reliability and comparability of information in the FSs, which are inconsistent with PAS 1 and CFFR, thus, hindering the users from making informed economic decisions.

Further, the accumulation of P2.424 billion in Undistributed Collections account as at December 31, 2023, due to the delay in reclassifying proceeds, non-usage or improper usage of the Undistributed Collections account, and over-reversal of previously recorded Payables resulted in the misstatement of an undetermined amount of the Receivable, Other Payable, and/or Revenue accounts, contrary to the PAS 1 requirement for the fair presentation of FSS and OB2 of CFFR.

Additionally, the faithful representation in the FSS of Financial Liabilities - ABP-ICS-RTH amounting to P2.220 billion for CY 2023 is unreliable due to the 105,033 claims amounting to P1.066 billion with deficiencies and irregularities. These noted discrepancies are inconsistent with the PAS 1 and CFFR.

Similarly, the presentation and disclosures of provisions and contingencies are not in compliance with the minimum requirements of PAS 1 and 37 due to the following issues: a) provisions and contingent liabilities for appealed benefit claims are not adequately presented or disclosed in the FSS and its accompanying Notes due to an insufficient monitoring and reporting mechanism, resulting in the misstatement of Accrued Benefit Claims Payable and Provision for Health Benefits - IBNR by an unidentified amount; and b) contingent assets arising from outstanding legal cases are improperly disclosed in the Notes to FSs due to the lack of policy on specific classification criteria for estimating the outcome of the inflow of economic benefits. Consequently, these misstatements and limitations undermine the primary objectives of both PAS 1 and 37, where PAS 1 seeks to ensure that the effects of transactions are faithfully represented within the FSs, while PAS 37 aims to inform users/readers about the financial impact of provisions and contingencies.

Moreover, the persistent utilization of fragmented systems for data management, resulting from the delayed implementation of an integrated IT System, and the compromised application servers of PhilHealth due to weak system security measures, have significantly impacted the data capture of various transactions. This manifests a significant weakness in internal control over financial reporting, exposing the information presented in the FSS to the heightened risks of misstatements and questionable integrity and reliability.

Furthermore, the faithful representation of the CYS 2023 and 2022 Trial Balance (TB) account balances could not be ascertained due to the following significant noted deficiencies, casting doubt on the reliability of the balances of accounts presented in the FSs and its accompanying Notes, which is not in accordance with the PAS 1 and QC4 and QC12 of CFGPFR: a) non-reconciliation of the accounts with abnormal balances totaling P26.463 billion and P13.306 billion in the TB - Head Office

and combined TB, respectively; b) inconsistent use of account titles across the PhilHealth Regional Offices, contrary to Sections 2.1 and 2.4 of COA Circular No. 2020-002; and c) interchanged use of accounts due to the presence of multiple sub-accounts for a single class of loans under the Trust Liabilities- PhilHealth Provident Fund account, exposing the accounts to the risk of accumulation of outstanding or negative balances, contrary to sound financial reporting. These observations are further undermined by several accounts that are not supported with detailed subsidiary ledgers or schedules, precluding the verification of transactions, thus constituting a scope limitation in the conduct of audit of PhilHealth's transactions for CY 2023.

Lastly, the presentation in the FSs and disclosures in the Notes to FSS were not adequate to conform with the minimum requirements of the PFRSs, thereby depriving the users/readers of the FSS of relevant information in making economic decisions, contrary to Paragraphs 9, 112 and 113 of PAS 1.

PhilHealth Management **has fully acknowledged and accepted** the audit observations, recognizing the critical nature of the issues identified. While PhilHealth Management has demonstrated a commitment to addressing these concerns, the necessary actions to rectify the audit findings have not yet been fully implemented. Consequently, the corrective measures recommended have not been reflected in the Financial Statements and accompanying notes for the year ended December 31, 2023. This ongoing effort by Management, though positive, underscores the importance of continued diligence to ensure the full and timely resolution of these matters in future reporting periods.

Other Matter

In our previous report dated June 29, 2023, covering the calendar year ending December 31, 2022, we did not express an opinion on the fairness of the FSS due to the significant limitations in verifying the faithful representation of the Provision for ICL, Premium Contributions Direct Contributors, Provision for Health Benefits - IBNR amounting to P266.873 billion, P139.475 billion, P95.907 billion, respectively. Additionally, other asset, liability, expense, and revenue accounts were similarly affected.

While the bases for the prior year's disclaimer of opinion have not been fully resolved in CY 2023, we acknowledge that PhilHealth Management has taken significant steps towards addressing the identified audit and scope limitations. This ongoing corrective action by Management has led to a shift in our audit approach and opinion for the current year, reflecting a more detailed assessment of the financial statements in light of these ongoing efforts.

Responsibilities of Management and Those Charged with Governance for the Financial Statements

Management is responsible for the preparation and fair presentation of these FSs in accordance with PFRSS, and for such internal control as management determines is necessary to enable the preparation of FSs that are free from material misstatement, whether due to fraud or error.

In preparing the FSs, management is responsible for assessing the PhilHealth's ability to continue as a going concern, disclosing, as applicable, matters related to going concern and using the going concern basis of accounting unless management either intends to liquidate the PhilHealth or to cease operations, or has no realistic alternative but to do so.

Those charged with governance are responsible for overseeing the PhilHealth's financial reporting process.

Auditor's Responsibilities for the Audit of the Financial Statements

Our responsibility is to conduct an audit of the PhilHealth's FSs in accordance with International Standards of Supreme Audit Institutions and to issue an auditor's report. Because of the matters described in the *Bases for Qualified Opinion* section of our report, we were able to obtain sufficient appropriate audit evidence to provide a basis for an audit opinion on these FSs.

We are independent of the PhilHealth in accordance with the Revised Code of Conduct and Ethical Standards for the Commission on Audit Officials and Employees (Code of Ethics) together with the ethical requirements that are relevant to our audit of the FSS in the Philippines, and we have fulfilled our other ethical responsibilities in accordance with these requirements and the Code of Ethics.

Report on Other Legal and Regulatory Requirements

Our audits were conducted for the purpose of forming an opinion on the basic FSs taken as a whole. The supplementary information for the year ended December 31, 2023 required by the Bureau of Internal Revenue as disclosed in Note 35 to the FSs is presented for purposes of additional analysis and is not a required part of the basic FSS prepared in accordance with PFRSS. Such supplementary information is the responsibility of management.

COMMISSION ON AUDIT

By:



NORMABELLA F. PORTILES

OIC-Supervising Auditor

Audit Group F - Philippine Health Insurance Corporation

Cluster 6-Social, Cultural, Trading, Promotional and Other Services

Corporate Government Audit Sector

September 11, 2024

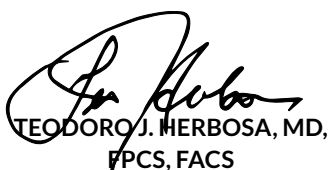


STATEMENT OF MANAGEMENT'S RESPONSIBILITY FOR FINANCIAL STATEMENTS

The Management of the Philippine Health Insurance Corporation is responsible for the preparation of the financial statements as at December 31, 2023 and 2022, including the additional components attached thereto in accordance with the prescribed financial reporting framework indicated therein. The responsibility includes designing and implementing internal controls relevant to the preparation and fair presentation of financial statements that are free from material misstatement whether due to fraud or error, selecting and applying appropriate accounting policies and making accounting estimates that are reasonable in the circumstances.

The Board of Directors reviews and approves the financial statements before such statements are issued to the regulators, creditors and other users.

The Commission on Audit has audited the financial statements of the Philippine Health Insurance Corporation in accordance with the Philippine Financial Reporting Standards (PFRS) where applicable, as well as government accounting standards and other pertinent rules and regulations, and has expressed its opinion on the fairness of presentation upon completion of such audit, in its report to the Board of Directors.



TEODORO J. HERBOSA, MD,
FPCS, FACS

DOH Secretary/ Chairperson
of the Board



EMMANUEL R. LEDESMA, JR.

President and
Chief Executive Officer



RENATO L. LIMSIACO, JR.,
CPA, DM, CESE

SVP, Fund Management Sector

PHILIPPINE HEALTH INSURANCE CORPORATION

STATEMENTS OF FINANCIAL POSITION

As at December 31, 2023 and 2022 (In Philippine Peso)

	Notes	2023	2022 (As Restated)
ASSETS			
Current Assets			
Cash and cash equivalents	5	30,779,119,082	6,423,703,001
Investment in time deposits	6	135,606,097,538	126,426,398,154
Receivables, net	7	80,602,658,375	37,964,072,573
Other receivable, IRM	8	–	8,230,950
Inventories	9	125,716,250	122,846,004
Other current assets	10	2,722,550,287	28,369,588
Total Current Assets		249,836,141,532	170,973,620,270
Non-Current Assets			
Investment securities at amortized cost	11	336,127,635,854	281,181,882,805
Investment property	12	855,663,291	855,663,291
Property and equipment, net	13	827,659,343	824,386,979
Right-of-use of assets	14	432,170,366	505,427,949
Intangible assets, net	15	259,353,119	264,741,753
Other non-current assets, net	16	208,180,216	193,270,284
Total Non-Current Assets		338,710,662,189	283,825,373,061
TOTAL ASSETS		588,546,803,721	454,798,993,331
LIABILITIES			
Current Liabilities			
Financial liabilities	17	38,728,256,966	56,387,901,742
Inter-agency payables	18	762,080,485	716,088,056
Trust liabilities	19	1,056,381,446	1,009,307,576
Provision for health benefits	20	57,507,818,227	95,907,073,327
Other payables	21	2,458,148,334	1,035,151,202
Total Current Liabilities		100,512,685,458	155,055,521,903
Non-Current Liabilities			
Deferred credits/Unearned income	22	818,228,649	952,574,650
Lease payable	23	451,812,395	535,743,247
Leave benefits payable	24	814,468,334	807,441,849
Provision for insurance contract liabilities	25	1,149,656,439,006	266,873,312,531
Total Non-Current Liabilities		1,151,740,948,384	269,169,072,277
TOTAL LIABILITIES		1,252,253,633,842	424,224,594,180
EQUITY			
Members' equity	26	(663,706,830,121)	30,574,399,151
Total Equity		(663,706,830,121)	30,574,399,151
TOTAL LIABILITIES AND EQUITY		588,546,803,721	454,798,993,331

PHILIPPINE HEALTH INSURANCE CORPORATION

STATEMENTS OF COMPREHENSIVE INCOME

For the Years Ended December 31, 2023 and 2022 (In Philippine Peso)

	Notes	2023	2022 (As Restated)
Premium contributions	27	236,158,650,315	219,539,373,046
Less: Benefit claims expenses	29	74,207,188,265	143,560,402,394
GROSS MARGIN FROM OPERATIONS		161,951,462,050	75,978,970,652
Operating Expenses			
Personnel services	30	5,348,565,974	4,972,582,892
Other operating expenses	31	4,054,742,038	4,210,737,670
TOTAL OPERATING EXPENSES		9,403,308,012	9,183,320,562
NET OPERATING INCOME (LOSS)		152,548,154,038	66,795,650,090
Add: Interest and other income	28	21,516,226,869	12,242,892,152
NET INCOME BEFORE CHANGE IN ICL		174,064,380,907	79,038,542,242
OTHER GAINS	28.3	-	72,436,425,208
OTHER LOSSES	31.4	(882,783,126,475)	-
NET INCOME (LOSS)		(708,718,745,568)	151,474,967,450

PHILIPPINE HEALTH INSURANCE CORPORATION

STATEMENTS OF CHANGES IN EQUITY

For the Years Ended December 31, 2023 and 2022 (In Philippine Peso)

	Notes	2023	2022 (As Restated)
SURPLUS	26		
Surplus at January 1		-	-
Net income before change in ICL		174,064,380,907	79,038,542,242
Other Gains		-	72,436,425,208
Other Losses		(882,783,126,475)	-
Net Income		(708,718,745,568)	151,474,967,450
Prior year adjustment		14,437,516,296	5,248,548,177
Total Surplus (Deficit)		(694,281,229,272)	156,723,515,627
Surplus transferred to reserves		(188,501,897,203)	(84,287,090,419)
Surplus applied to provision for ICL		882,783,126,475	(72,436,425,208)
Surplus at December 31		-	-
RESERVE FUND	26		
Reserve at January 1		275,785,094,946	191,498,004,527
Net income		174,064,380,907	79,038,542,242
Surplus transferred to reserves		14,437,516,296	5,248,548,177
Reserve Fund at December 31		464,286,992,149	275,785,094,946
Provision for insurance contract liability	25	(1,127,993,822,270)	(245,210,695,795)
TOTAL MEMBERS' EQUITY		(663,706,830,121)	30,574,399,151

PHILIPPINE HEALTH INSURANCE CORPORATION

STATEMENTS OF CASH FLOWS

For the Years Ended December 31, 2023 and 2022 (In Philippine Peso)

	Note	2023	2022
CASH FLOWS FROM OPERATING ACTIVITIES			
Cash Inflows			
Cash received from premium contributions		197,809,239,822	207,196,436,071
Collection of other income		1,237,371,504	375,592,222
Gain (Loss) on foreign exchange		40,267	43,466
Collection of rent		600,000	638,791
Total Cash Inflows		199,047,251,593	207,572,710,550
Cash Outflows			
Payment of benefit claims		(118,900,340,656)	(130,557,999,599)
Payment of operating expenses		(8,106,244,684)	(6,841,221,259)
Payment of financial charges		(29,555,380)	(29,860,033)
Loss on Foreign Exchange		(44,764)	
Total Cash Outflows		(127,036,185,484)	(137,429,080,891)
Net Cash Provided by/(Used in) Operating Activities		72,011,066,109	70,143,629,659
CASH FLOWS FROM INVESTING ACTIVITIES			
Cash Inflows			
Matured time deposits		157,103,588,752	120,189,963,980
Matured treasury bills		9,350,077,535	2,864,435,324
Matured treasury bonds		41,679,228,000	20,122,998,342
Interest received from investments		18,713,680,940	11,610,930,177
Proceeds from disposal of assets		380,100	302,911
Total Cash Inflows		226,846,955,327	154,788,630,734
Cash Outflows			
Placement on time deposits		(166,283,301,101)	(180,377,570,871)
Placement on treasury bills		–	(12,214,512,859)
Placement on treasury bonds		(106,631,289,408)	(31,787,669,037)
Interest paid on placement of bonds		(1,011,889,408)	(8,114,167)
Fixed assets purchase		(314,822,914)	(269,768,890)
Total Cash Outflows		(274,241,302,831)	(224,657,635,824)
Net Cash Provided by/(Used in) Investing Activities		(47,394,347,504)	(69,869,005,090)
CASH FLOWS FROM FINANCING ACTIVITIES			
Cash Inflows			
Trust receipts		473,565,282	459,281,712
Total Cash Inflows		473,565,282	459,281,712
Cash Outflows			
Trust disbursements		(448,699,239)	(408,284,323)
Finance lease payments		(286,168,567)	(265,713,475)
Total Cash Outflows		(734,867,806)	(673,997,798)
Net Cash Provided By/(Used In) Financing Activities		(261,302,524)	(214,716,086)
INCREASE/(DECREASE) IN CASH AND CASH EQUIVALENTS		24,355,416,081	59,908,483
CASH AND CASH EQUIVALENTS, JANUARY 1	5	6,423,703,001	6,363,794,518
CASH AND CASH EQUIVALENTS, DECEMBER 31	5	30,779,119,082	6,423,703,001

PHILIPPINE HEALTH INSURANCE CORPORATION

Notes to the Financial Statements

(All amounts in Philippine Peso unless otherwise stated)

1. GENERAL INFORMATION

The National Health Insurance Act of 1995 or Republic Act (RA) No. 7875, and its amendments, otherwise known as the "National Health Insurance Act of 2013", instituted a National Health Insurance Program (NHIP) that shall provide health insurance coverage and ensure affordable, acceptable, available and accessible health care services for all Filipinos. The Philippine Health Insurance Corporation (PhilHealth), a tax-exempt Government-Owned and/or Controlled Corporation (GOCC), was established to administer the Program at the central and local levels. The Head Office is located at 709 CityState Centre Building, Barangay Oranbo, Shaw Blvd., Pasig City.

On February 20, 2019, RA No. 11223 or the Universal Health Care (UHC) Act was enacted. This law automatically provides social health risk protection for all Filipino citizens in the NHIP.

The Corporation is an attached agency of the Department of Health (DOH) for policy coordination and guidance, governed by a Board of Directors (BODs) composed of 13 members and has the powers and functions provided for in Article IV, Section 16 of RA No. 7875 as amended. The Corporation has the power, among others, to formulate and promulgate policies, set standards, rules and regulations necessary to ensure quality of care and overall accomplishment of program objectives, formulate and implement guidelines contributions and benefits; establish branch offices, manage grants and donations, acquire property, collect, deposit, invest, administer and disburse the National Health Insurance Fund (NHIF), to enter into contract with health care institutions, to fix a reasonable compensation, allowances and benefits of all positions including the President and Chief Executive Officer (PCEO), based on a comprehensive job analysis and audit of actual duties and responsibilities subject to the approval of the President of the Philippines.

The NHIF as amended shall consist of premiums from direct or indirect contributors; other appropriations earmarked by the national and local governments purposely for the implementation of the program; subsequent appropriations provided for under Sections 46 and 47 of RA No. 7875, as amended; donations and grants-in-aid; and all accruals thereof. Under Section 26, Article VI of RA No. 7875, as amended, the use, disposition, investment, administration and management of the NHIF, including any subsidy, grant or donation received for the program operations shall be governed by applicable laws, and in the absence thereof, existing resolution of the BODs of the Corporation subject to limitations prescribed in the Act.

The financial statements of the Corporation for the year ended December 31, 2023 were approved and authorized for issue by the BODs on September 11, 2024.

2. SUMMARY OF SIGNIFICANT ACCOUNTING POLICIES

2.1 Basis of Preparation and Statement of Compliance

Presently, PhilHealth is now classified as a Commercial Public Sector Entities (CPSE) per COA Resolution No. 2020-013 dated January 31, 202, re: Renaming Government Business Entities (GBEs) and Non-Government Business Entities (non-GBEs) into CPSEs and Non-CPSEs pursuant to the 2018 Edition of the Handbook of International Public Sector Accounting Pronouncements (HIPSAP) published by the International Federation of Accountants (IFAC) and the International Public Sector Accounting Standard Board (IPSASB) and its adoption of the Philippine Financial Reporting Standards (PFRSs) as its financial reporting framework.

2.2 Measurement Bases

The financial statements are measured in Philippine Peso (P), which is also the Corporation's functional and presentation currency. All values are rounded-off to the nearest peso values, unless otherwise stated. The financial statements have been prepared on a historical cost basis.

Historical cost is generally based on the fair value of the consideration given in exchange of an asset and fair value of the consideration received in exchange for incurring a liability. Fair value is the price that would be received to sell an asset or paid to transfer a liability in an orderly transaction between market participants at the measurement date. The Corporation uses market observable data to a possible extent when measuring the fair value of an asset or a liability. Fair values are categorized into different levels in a fair value hierarchy based on inputs used in the valuation techniques as follows:

- Level 1 - Quoted (unadjusted) market prices in active markets for identical assets or liabilities.
- Level 2 - Valuation techniques for which the lowest level input that is significant to the fair value measurement is directly or indirectly observable.
- Level 3 - Valuation techniques for which the lowest level input that is significant to the fair value measurement is unobservable.

2.3 Adoption of New and Amended PFRS

The accounting policies adopted are consistent with those of the previous financial year, except for the adoption of the following new and amended PFRS which the Corporation adopted effective for annual periods beginning January 1, 2023. Unless otherwise indicated, the adoption of the new and amended PFRS did not have any material effect on the financial statements. Additional disclosures have been included in the notes to financial statements, as applicable.

Amendments to PAS 8, Definition of Accounting Estimates – The amendments introduce a new definition of accounting estimates, clarify the distinction between changes in accounting estimates and changes in accounting policy and correction of errors. It also clarifies how entities use measurement techniques and inputs to develop accounting estimates.

Amendments to PAS 1 and PFRS Practice Statement 2, Disclosure of Accounting Policy – The amendments aim to help entities provide accounting policy disclosures that are more useful by replacing the requirement for entities to disclose their 'significant' accounting policies with a requirement to disclose their 'material' accounting policies and adding guidance on how to apply the concept of materiality in making decisions about accounting policy disclosures.

2.4 New and Amended PFRS Issued But Not Yet Effective

Relevant new and amended PFRSs, which are not yet effective as at December 31, 2023 and have not been applied in preparing the financial statements, are summarized below. Effective for annual periods beginning on or after January 1, 2025:

PFRS 17, Insurance Contracts – This standard will replace PFRS 4, Insurance Contracts. It requires insurance liabilities to be measured at current fulfillment value and provides a more uniform measurement and presentation approach to achieve consistent, principle-based accounting for all insurance contracts. It also requires similar principles to be applied to reinsurance contracts held and investment contracts with discretionary participation features issued.

In response to the challenges brought by the Covid-19 pandemic, the Insurance Commission issued Circular Letter 2020-062, Amendment of Section 1 of Circular Letter No. 2018-69, Deferral of IFRS 17 Implementation, which provides a two-year deferral on the implementation of the standard from the 2023 effectivity date. Therefore, all life and nonlife insurance companies in the Philippines shall adopt PFRS 17 for annual periods beginning on or after January 1, 2025.

Under prevailing circumstances, the adoption of the foregoing new and amended PFRS is not expected to have any material effect on the financial statements of the Corporation. Additional disclosures were included in the financial statements, as applicable.

2.5 Financial Assets and Liabilities

Date of Recognition

The Corporation recognizes a financial asset or a financial liability in the statements of financial position when it becomes a party to the

contractual provisions of a financial instrument. In the case of a regular way purchase or sale of financial assets, recognition and derecognition, as applicable is done using settlement date accounting.

Initial Recognition

Financial instruments are recognized initially at fair value, which is the fair value of the consideration given (in case of an asset) or received (in case of a liability). The initial measurement of financial instruments, except for those designated at Fair Value through Profit or Loss (FVPL), includes transaction cost.

"Day 1" Difference

Where the transaction in a non-active market is different from the fair value of other observable current market transactions in the same instrument or based on a valuation technique whose variables include only data from observable market, the Corporation recognizes the difference between the transaction price and fair value (a "Day 1" difference) in profit or loss.

In cases where there is no observable data on inception, the Corporation deems the transactions price as the best estimate of fair value and recognizes "Day 1" difference in profit or loss when the inputs become observable or when the instrument is derecognized. For each transaction, the Corporation determines the appropriate method of recognizing the "Day 1" difference.

Classification of Financial Instruments

The Corporation classifies its financial assets at initial recognition under the following categories: (a) financial assets at FVPL, (b) financial assets at amortized cost and, (c) financial assets at Fair Value through Other Comprehensive Income (FVOCI). Financial liabilities, on the other hand, are classified as either financial liabilities at FVPL or other financial liabilities at amortized cost. The classification of a financial instrument largely depends on the Corporation's business model.

As at December 31, 2022 and December 31, 2021, the Corporation does not have financial assets and liabilities at FVPL and debt instruments measured at FVOCI.

Financial Assets at Amortized Cost

A financial asset shall be measured at amortized cost if both of the following conditions are met:

- The financial asset is held within a business model whose objective is to hold financial assets in order to collect contractual cash flows; and
- The contractual terms of the financial asset give rise on specified dates to cash flows that are solely payments of principal and interest on the principal amount outstanding.

After initial recognition, financial assets at amortized cost are subsequently measured at amortized cost using the effective interest method, less allowance for impairment, if any. Amortized cost is calculated by taking into account any discount or premium on acquisition and fees that are an integral part of the effective interest rate. Gains and losses are recognized in profit or loss when the financial assets are derecognized and through amortization process. Financial assets at amortized cost are included under current assets if realizability or collectability is within 12 months after the reporting period. Otherwise, these are classified as noncurrent assets.

Included in this category are cash and cash equivalents, receivables and other receivables.

Financial Liabilities at Amortized Cost

Financial liabilities are categorized as financial liabilities at amortized cost when the substance of the contractual arrangement results in the Corporation having an obligation either to deliver cash or another financial asset to the holder, or to settle the obligation other than by the exchange of a fixed amount of cash or another financial asset for a fixed number of its own equity instruments.

These financial liabilities are initially recognized at fair value less any directly attributable transaction costs. After initial recognition, these financial liabilities are subsequently measured at amortized cost using the effective interest method. Amortized cost is calculated by taking

into account any discount or premium on the issue and fees that are an integral part of the effective interest rate. Gains and losses are recognized in profit or loss when the liabilities are derecognized or impaired or through the amortization process.

This category includes financial liabilities, inter-agency payables, trust liabilities, leave benefits payable and other payables.

2.6 Impairment of Financial Assets at Amortized Cost

The Corporation records an allowance for Expected Credit Loss (ECL). The ECL is based on the difference between the contractual cash flows due in accordance with the contract and all the cash flows that the Corporation expects to receive. The difference is then discounted at an approximation to the asset's original effective interest rate.

2.7 Derecognition of Financial Assets and Liabilities

Financial Assets

A financial asset (or, where applicable a part of a financial asset or part of a group of similar financial assets) is derecognized when:

- The right to receive cash flows from the asset has expired;
- The Corporation retains the right to receive cash flows from the asset, but has assumed an obligation to pay them in full without material delay to a third party under a "pass-through" arrangement; or
- The Corporation has transferred its right to receive cash flows from the asset and either; (a) has transferred substantially all the risks and rewards of the asset, or (b) has neither transferred nor retained substantially all the risks and rewards of the asset, but has transferred control of the asset.

When the Corporation has transferred its right to receive cash flows from an asset and has neither transferred nor retained substantially all the risks and rewards of the asset nor transferred control of the asset, the asset is recognized to the extent of the Corporation's continuing involvement in the asset. Continuing involvement that takes the form of a guarantee over the transferred asset is measured at the lower of the original carrying amount of the asset and the maximum amount of consideration that the Corporation could be required to repay.

Financial Liabilities

A financial liability is derecognized when the obligation under the liability is discharged, cancelled or expired. When an existing financial liability is replaced by another from the same lender on substantially different terms, or the terms of an existing liability are substantially modified, such an exchange or modification is treated as a derecognition of the original liability and the recognition of a new liability. The difference in the respective carrying amounts is recognized in profit or loss.

2.8 Offsetting of Financial Assets and Liabilities

Financial assets and liabilities are offset and the net amount is reported in the statements of financial position if, and only if, there is a currently enforceable legal right to offset the recognized amounts and there is an intention to settle on a net basis, or to realize the assets and settle the liabilities simultaneously. This is not generally the case with master netting agreements, and the related assets and liabilities are presented gross in the statements of financial position.

2.9 Classification of Financial Instrument between Liability and Equity

A financial instrument is classified as liability if it provides for a contractual obligation to:

- Deliver cash or another financial asset to another entity;
- Exchange financial assets or financial liabilities with another entity under conditions that are potentially unfavorable to the Corporation; or
- Satisfy the obligation other than by the exchange of a fixed amount of cash or another financial asset for a fixed number of own equity shares.

If the Corporation does not have an unconditional right to avoid delivering cash or another financial asset to settle its contractual obligation, the obligation meets the definition of a financial liability.

2.10 Inventories

Inventories include office supplies and materials inventory, semi-expendable machinery and equipment, and semi-expendable furniture, fixtures and books. These are initially measured at cost. At each reporting date, inventories are assessed for impairment. If an item of inventory is impaired, its carrying value is reduced to net realizable value, and an impairment loss is recognized immediately in profit and loss. Any reversal of impairment is recognized also in profit and loss.

2.11 Other Current Assets

Other current assets represent assets of the Corporation which are expected to be realized within one year or within the Corporation's normal operating cycle whichever is longer. Other current assets are presented in the Statement of Financial Position at net realizable value.

2.12 Property and Equipment

Property and equipment are tangible assets that are held for use in the production or supply of goods or services, for rental to others, or for administrative purposes, and are expected to be used during more than one period.

Items of property and equipment are initially measured at cost. Such cost includes purchase price and all incidental costs necessary to bring the asset to its location and condition. Subsequent to initial recognition, items of property and equipment are measured in the statement of financial position at cost less any accumulated depreciation and any accumulated impairment losses. Depreciation, which is computed on a straight-line basis, is recognized so as to allocate the cost of assets less their residual values over their estimated useful lives.

The depreciation periods for property and equipment, based on above policies, are as follows:

Particulars	Estimated Useful Life (in years)
Land improvements	10
Building and building improvements	30
Leasehold improvements	10 or contract duration whichever is shorter
IT equipment	5
Furniture and fixtures	10
Office equipment	5
Communication equipment	10
Library books	5
Medical equipment	10
Transportation equipment	7

The estimated useful lives and depreciation method are reviewed periodically to ensure that the periods and method of depreciation is consistent with the expected pattern of economic benefits from items of property and equipment.

Land is not depreciated. If there is an indication that there has been a significant change in useful life or residual value of an asset, the depreciation of that asset is revised prospectively to reflect the new expectations.

When assets are sold, retired or otherwise disposed of, their costs and related accumulated depreciation and impairment losses, if any, are removed from the accounts and any resulting gain or loss is reflected in profit or loss for the period.

2.13 Intangible Assets

Intangible asset represents computer software and licenses. This is initially measured at cost and is presented in the Statement of Financial Position at cost less accumulated amortization and any accumulated impairment losses.

Computer software with finite life is amortized over its estimated useful life of five (5) years using the straight-line method while the licenses

are amortized over the life of the license. The amortization period and method are reviewed periodically to ensure that these are consistent with expected pattern of economic benefits from the intangible assets. If there is an indication that there has been a significant change in the useful life or residual value of an intangible asset, the amortization is revised prospectively to reflect the new expectations.

Intangibles that are classified with indefinite life are not amortized but are subject to impairment test annually. Part of the intangibles with indefinite life are internally developed software that are being used in the operations.

When assets are sold, retired or otherwise disposed of, their cost and related accumulated amortization and impairment losses, if any, are removed from the accounts and any resulting gain or loss is reflected in profit or loss for the period.

2.14 Impairment of Non-Financial Assets

At each reporting date, property and equipment and intangible asset accounts are reviewed to determine whether there is any indication that those assets have suffered an impairment loss. If there is an indication of possible impairment, the recoverable amount of any affected asset (or group of related assets) is estimated and compared with its carrying amount. If estimated recoverable amount is lower, the carrying amount is reduced to its estimated recoverable amount, and an impairment loss is recognized immediately in profit and loss.

If an impairment loss subsequently reverses, the carrying amount of the asset is increased to the revised estimate of its recoverable amount, but not in excess of the amount that would have been determined had no impairment loss been recognized for the asset in prior years. A reversal of an impairment loss is recognized immediately in profit or loss.

2.15 Reserve Fund

Reserve Fund represents the cumulative results of normal and continuous operations profit of the Corporation including prior period adjustments, effects of changes in accounting and other adjustments.

2.16 Revenue Recognition

Revenue is recognized to the extent that is probable that the economic benefits will flow to the Corporation and the revenue can be reliably measured. Revenue is measured at the fair value of the consideration received, excluding discounts and rebates. Revenue is recognized either at a point in time or over a period of time.

Premium contributions

Revenue is recognized as the member contributions become due. Fines and penalties are recognized when the event that triggers the fine or penalty occurs. If the collectability is not reasonably assured, fines and penalties are recognized only when cash is received.

Interest Income

Interest income is recognized as the interest accrues taking into account the effective interest.

Rent Income

Income from rental of property is recognized on a straight-line basis over the lease term.

2.17 Expense Recognition

Expenses are decreases in economic benefits during the accounting period in the form of outflows or depletions of assets or incurrence of liabilities that result in decrease in equity, other than those relating to distributions of equity to participants.

Benefit Claims Expense

This represents benefits incurred by the Corporation for health care services, in-patient, out-patient, Primary Care Benefit (PCB) and Z benefit packages availed of by the members and their dependents. Benefit Claims Expense is recognized at the date of admission (per Corporate Order No. 2021-0051).

Operating Expenses

These include personnel services and maintenance and other operating expenses which are recognized as expense in the period they are incurred.

2.18 Leases

A lease is a contract that conveys the right to use an identified asset for a period of time in exchange for a consideration.

Determination as to whether a contract is, or contains, a lease is made at the inception of the lease. Accordingly, the Corporation assesses whether the contract meets three key evaluations which are:

- The contract contains an identified asset, which is either explicitly identified in the contract or implicitly specified by being identified at the time the asset is made available to the lessee;
- The lessee has the right to obtain substantially all of the economic benefits from use of the identified asset throughout the period of use, considering its rights within the defined scope of the contract; and,
- The lessee has the right to direct the use of the identified asset throughout the period of use. The lessee assesses whether it has the right to direct 'how and for what purpose' the asset is used throughout the period of use.

Corporation as a Lessee

Leases are recognized as a Right-of-Use (ROU) asset and a corresponding liability at the date at which the leased asset is available for use by the Corporation.

The Corporation's leasing activity is mainly for office spaces and warehouses. The non-cancellable contracts period for leases is three to five years and has no extension clause. Upon expiration of the lease contract, it is extended on a month to month basis only until a new contract is executed or a new office space will be leased.

ROU Assets

At commencement date, the Corporation measures ROU assets at cost. The cost comprises:

- i. The amount of the initial measurement of lease liabilities;
- ii. Any lease payments made at or before the commencement date less any lease incentives received;
- iii. Any initial direct costs; and
- iv. An estimation of costs to be incurred by the Corporation in dismantling and removing the underlying asset, when applicable.

The ROU assets are recognized at the present value of the liability at the commencement date of the lease, adding any directly attributable costs. After the commencement date, the ROU assets are carried at cost less any accumulated amortization and accumulated impairment losses, and adjusted for any re-measurement of the related lease liabilities. The ROU assets are amortized over the shorter of the lease terms including renewals or the useful lives of the underlying assets. In addition, the ROU asset is periodically reduced by impairment losses, if any and adjusted for certain measurement of the lease liability.

ROU assets are office spaces and warehouses being leased by PhilHealth with a multi-year contract.

Below is the summary of information regarding ROU asset of the Corporation:

	2023	2022
Carrying amount of Right-of-Use asset	432,170,366	505,427,949
Depreciation expense	272,458,495	272,589,986
Interest expense	19,037,558	22,873,017
Short-term leases	325,222,188	282,550,111
Total	1,048,888,607	1,083,441,063

Lease Liabilities

At commencement date, the Corporation measures a lease liability at the present value of future lease payments using the interest rate implicit in the lease, if that rate can be readily determined. Otherwise, the Corporation uses its incremental borrowing rate.

Lease payments included in the measurement of a lease liability comprise the following:

- i. Fixed payments, including in-substance fixed payments;
- ii. Variable lease payments that depend on an index or a rate, initially measured using the index or rate as at the commencement date;
- iii. Amounts expected to be payable by the lessee under residual value guarantees; and
- iv. The exercise price under a purchase option that the Company is reasonably certain to exercise; lease payments in an optional renewal period if the Company is reasonably certain to exercise an extension option; and penalties for early termination of a lease unless the Company is reasonably certain not to terminate early.

A lease liability is subsequently measured at amortized cost. Interest on the lease liability and any variable lease payments not included in the measurement of lease liability are recognized in profit or loss unless these are capitalized as costs of another asset. Variable lease payments not included in the measurement of the lease liability are recognized in profit or loss when the event or condition that triggers those payments occurs.

If there is a change in the lease term or if there is a change in the assessment of an option to purchase the underlying asset, the lease liability is re-measured using a revised discount rate considering the revised lease payments on the basis of the revised lease term or reflecting the change in amounts payable under the purchase option.

Short-Term Leases and Leases of Low-Value Assets

Short-term leases and lease of low value assets are exempted from the recognition of ROU asset and lease liability as allowed under PFRS 16. These are recognized in the profit or loss when incurred.

The Corporation does not have leases of low-value assets. Classified under short-term leases are office spaces that are on a month to month basis renewal and have no multi-year contract.

Corporation as a Lessor

Leases where the Corporation does not transfer substantially all the risks and benefits of ownership of the assets are classified as operating leases. Initial direct costs incurred in negotiating operating leases are added to the carrying amount of the leased asset and amortized over the lease term on the same basis as the rental income. Contingent rents are recognized as revenue in the period in which these are earned. Operating leases are recognized as income in the statements of comprehensive income on a straight-line basis over the lease term.

The Corporation determines whether an arrangement is, or contains a lease based on the substance of the arrangement. It makes an assessment of whether the fulfillment of the arrangement is dependent on the use of a specific asset or assets and the arrangement conveys a right to use the asset.

2.19 Employee Benefits

Short-term Benefits

Short-term benefits given by the Corporation to its employees include salaries and wages, compensated absences, 13th month pay, Mid-year bonus, employer share contributions and other de minimis benefits, among others. These are granted pursuant to E.O. 150, or the Compensation and Position Classification System for Government Corporations.

These are recognized as expense in the period the employees render services to the Corporation.

2.20 Related Parties

Related party relationship exists when one party has the ability to control, directly, or indirectly through one or more intermediaries, the other party or exercises significant influence over the other party in making financial and operating decisions. Such relationships also exist between and/or among entities which are under common control with the reporting enterprise, or between, and/or among the reporting enterprise and its key management personnel, directors, or its major

shareholders. In considering each possible related party relationship, attention is directed to the substance of the relationship, and not merely the legal form.

2.21 Provisions

Provisions are recognized when the Corporation has a present obligation (legal or constructive) as a result of a past event, it is probable that an outflow of resources embodying economic benefits will be required to settle the obligation, and a reliable estimate can be made of the amount of the obligation.

Provisions are made using the best estimates of the amount required to settle the obligation and discounted to present values using a pre-tax rate that reflects current market assessments of the time value of money and the risks specific to the obligation. Changes in estimates are reflected in profit or loss in the period they arise.

2.22 Provisions for Insurance Contract Liabilities (ICL)

Provision for ICL is the estimate of the expected future claims payments to be incurred less expected premium contributions in relation to the liabilities that the Corporation is contractually obligated to pay to our members. It is computed as the expected net of present value of expenses (claims + expenses) and present value of collection plus margin for adverse deviation. Since 2005, PFRS 4 have mandated that these liabilities must be properly recognized and reported in the financial statements.

2.23 Contingencies

Contingent liabilities are not recognized in the financial statements. These are disclosed in the notes to financial statements unless the possibility of an outflow of resources embodying economic benefits is remote. Contingent assets are not recognized in the financial statements but are disclosed when an inflow of economic benefits is probable.

2.24 Events after the Reporting Period

Post year-end events that provide additional information about the Corporation's financial position at reporting date (adjusting events) are reflected in the financial statements. Post year-end events that are not adjusting events are disclosed in the notes to financial statements when material.

3. SIGNIFICANT JUDGMENTS, ACCOUNTING ESTIMATES AND ASSUMPTIONS

The preparation of the financial statements in accordance with the PFRS requires Corporation to exercise judgment, make accounting estimates and assumptions that affect the reported amounts in the financial statements and related notes. The judgment and estimates used in the financial statements are based upon Corporation's evaluation of relevant facts and circumstances as of the date of the financial statements. Actual results could differ from such estimates.

Judgment and estimates are continually evaluated and are based on historical experiences and other factors, including expectations of future events that are believed to be reasonable under the circumstances.

3.1 Judgments

In the process of applying the Corporation's accounting policies, management has made certain judgment, apart from those involving estimations, which have the most significant effect on the amounts recognized in the financial statements.

3.1 Estimates and Assumptions

The key assumptions concerning the future and other key source of estimation uncertainty at the financial reporting date, that have significant risk of causing a material adjustment to the carrying amounts of assets and liabilities within the next financial year are discussed below.

Estimating Premium Income from the Direct Contributors - Employed Sector
Estimate is based on SPA generated by employers. The amount estimated shall be based on the applicable period as at the reporting period. The generated SPA shall come from the extraction of data bases and shall cover all employers registered all over the country.

Estimating Allowance for Impairment of Receivables

The Corporation maintains allowance for impairment losses at a level considered adequate to provide for potential uncollectible receivables. This amount is evaluated based on such factors that affect the collectability of the accounts. These factors include, the age of the receivables, the length of the Corporation's relationship with the customer, the customer's payment behavior and known market factors. The amount and timing of recorded expenses for any period would differ if the Corporation made different judgments or utilized different estimates. An increase in allowance for impairment losses would increase the recorded operating expenses and decrease current assets.

Estimating Useful Lives of Property and Equipment

The Corporation estimates the useful lives of the property and equipment based on the period over which these assets are expected to be available for use. The estimated useful lives are reviewed periodically and are updated if expectations differ from previous estimates due to physical wear and tear, technical or commercial obsolescence and legal or other limits on the use of these assets. In addition, estimation of the useful lives is based on collective assessment of industry practice, internal technical evaluation and experience with similar assets. It is possible, however, that future results of operations could be materially affected by changes in estimates brought about by changes in factors mentioned above. The amounts and timing of recorded expenses for any period would be affected by changes in these factors and circumstances.

There is no change in the estimated useful lives of depreciable property and equipment in 2023 and 2022.

Estimating Benefit Claims Payables

One of the accounting estimates being made is the setting up of accrued benefit expense for Incurred But Not Yet Paid (IBNP) claims which consist of the following:

- In-Course of Settlement (ICS) - these are claims still in process at the end of the reporting period. It includes claims already approved for payment awaiting Automatic Debit Arrangement (ADA) and Return to Hospitals (RTH) of the current year. The estimate is based on the case rate amount extracted from the N-Claims.
- Provision for Denied Claims - Denied claims are claims determined to be invalid and unworthy of payment due to an absolute deficiency that cannot be remedied through return to Health Facility or due to a finding of an unmet requirement. However, a provision is necessary for those claims with pending motion for reconsiderations which are highly probable that payment shall be made after evaluation.
- Incurred But Not Yet Reported (IBNR) - These are claims which are estimated to be in the possession of the Health Care Institutions (HCIs) as of the end of the year and have yet to be submitted to the Corporation. The IBNR is the balance between IBNP and ICS when IBNP are those claims yet to be paid by PhilHealth. The amount to be recorded is actuarially estimated. The IBNR is computed as follows:

$$\text{IBNR} = \text{IBNP} - \text{ICS} - \text{Provision for Denied}$$

Methodology for Estimating the IBNP

The method applied in estimating IBNP claims is called claims development (or lag) method. It is an estimation technique under which historical claim data, such as the number and amount of claims are grouped into the lag time periods in which claims were incurred and the time periods in which they were paid. The development method uses these groupings to create a claims processing or lag development pattern, which is used to determine completion factors to help estimate the unpaid portion of incurred claims and come up with the ultimate value of claims. The paid claims are then subtracted from the ultimate value of claims by admission month to estimate the IBNP reserves.

The IBNP was estimated by admission month, and separately by covid and non-covid claims. The ultimate value of claims was estimated by admission months and subtracted the cumulative paid claim to arrive at the estimated IBNP claims.

Calculation of IBNP Claims – Non-Covid Claims

Ultimate claims for Non-Covid claims were estimated using the Paid Chain Ladder/Development Method (PDA), wherein the reported paid claims by admission month are divided by the appropriate completion factors. We based the completion factors on historical experience except for the immediate few months for which another method needs to be employed as completion factors may not be credible during these months. The paid claims are then subtracted from the ultimate value of claims by admission months to estimate the IBNP reserves.

Calculation of IBNP Claims – Covid Claims

IBNP claims for covid claims were calculated separately for “testing” and “other than testing,” utilizing distinct illness codes. However, the ultimate value of Covid claims was estimated by multiplying average paid per case (AVPC) to the ultimate claim count taking into account the frequency and severity of Covid claims instead of historical claims data due to lack of a well-established payment pattern and insufficient experience to use a credible lag method. The AVPC is obtained by dividing the cumulative paid claims amount by the paid claims count. The ultimate claim count was based on the number of reported claims to PhilHealth, and the official DOH reported number of Covid cases and number of administered tests. For 2023, since more stable paid claims information for the pandemic years had become available, lesser uncertainty margins had been considered.

Further, the following are included in the computation of the IBNP:

- Claims Handling Expense**
 It refers to the cost incurred by Philhealth in managing and processing claims. Five percent claims handling expense loading was applied to 100% of the IBNR and 50% of the ICS.
- Provision for Denied Claims**
 Denied claims are claims determined to be invalid and unworthy of payment due to an absolute deficiency that cannot be remedied through return to Health Facility or due to a finding of an unmet requirement. In projecting the provision for denied claims, the historical ratio of total paid claims at various development periods for previously denied claims to the total denied claims as of December 31, 2023 was first determined. After which, the selected ratio for future developments was multiplied by the total denied claims to come up with the provision.
- Margin for Adverse Deviation**
 It reflects the degree of uncertainty of the best estimate assumption. Because of the high level of uncertainty inherent in estimating IBNP reserves, and to reflect a higher level of comfort in the recorded number than that in the best estimate of IBNP, a 10% margin for adverse deviation was added in the computation.

4. FINANCIAL RISK MANAGEMENT

4.1 Financial Risk Factors

This is the risk that results from unexpected changes in external markets, prices, rates and liquidity supply and demand.

a. Credit risk

This refers to the risk of loss arising from the Corporation's counterparty to perform contractual obligations in a timely manner. This includes risk due to (a) failure of a counterparty to make required payments on their obligations when due (Default Risk) and (b) default of a counterparty before any transfer of securities or funds or once final transfer of securities or funds has begun but not been completed (Settlement Risk).

The Corporation's placements in fixed term deposits with Authorized Government Depository Banks (AGDBs) are in accordance with Sec. 5.2 of Department of Finance (DOF) Circular No. 002-2022, on the Revised Guidelines on Authorized Government Depository Banks, which mandates all Government Owned and Controlled Corporations (GOCCs), National Government Agencies (NGAs) and Local Government Units (LGUs) to maintain and deposit government funds only with AGDBs.

The Corporation implements a structured and standardized evaluation guideline, credit ratings and approval processes. Investments undergo technical evaluation to determine their viability/acceptability. Due Diligence process (i.e., yield versus comparable financial instruments, term of the indebtedness including redemption feature, liquidity of the issuance, monitoring of issuer and counterparty risks including risks arising from implementation, and other portfolio and strategic considerations) and information from third party (e.g., Philippine Ratings Services Corporation and Credit Ratings and Investors Services Philippines, Inc.) are used to determine if counterparties are credit-worthy. With respect to bond underwriters and selling agents, PhilHealth as a qualified institutional buyer of debt securities under Section 10.1 (L) of the Securities Regulation Code, deals only with domestic universal bank or domestic investment house duly assigned by the issuer as underwriter or selling agent for the distribution of debt issuances.

Purchase of peso denominated government securities under the Non-Restricted Trading Environment (NRTE) of the Philippine Dealing Exchange is based on Treasury Circular No. 04-2014, Implementing DOF Department Order No. 068-2014 entitled, “Revised Rules and Regulations for the Issuance, Placement, Sale, Service and Redemption of Treasury Bills and Bonds under R.A. No. 245, As Amended”. Evaluation of counterparty trading participant in the NRTE platform, at the minimum, is measured on the Government Securities Eligible Dealer's (i) good standing in the Philippine Dealing Exchange, (ii) Risk-Based Capital Adequacy Ratio (CAR), (iii) Financial Standing on Earnings and Profitability, (iv) compliance to Securities Regulation Code and Code of Ethics Governing Financial Market Activities in the Philippines, and (iv) positive track record of service with other government agencies.

To avoid significant concentrations of exposures to specific industries or group of issuers and borrowers, PhilHealth investments are regularly monitored to ensure that these are always within the prescribed cumulative ceilings specified in Sec. V.D.2 of CO 2021-0057 PhilHealth Omnibus Guidelines on Fund Investments (Corporate Order No. 2021-0057). The said Corporate Order outlines the policies and guidelines in determining and managing exposure limits to investment instruments including the negative list of investments that are considered not ethically and socially responsible by the Corporation such as companies with exposures in sin products like tobacco, liquor, alcohol, including gaming, and mining and quarrying.

Investments in government securities are not impaired. Only investments in corporate bonds are subject to impairment. Allowance for impairment is computed as follows:

EAD		*PD		*LGD		ECL
Exposure at Default	X	Probability of Default	X	Loss Given Default	X	Expected Credit Loss

**Probability of Default and Loss Given Default is based on Bloomberg*

As regards criteria for accreditation of applicant collecting agents for premium collections, PhilHealth requires submission of three-year Audited Financial Statements, as one of the documentary requirements for accreditation. Financial evaluation of the applicants comprised the analysis on liquidity ratio, solvency ratio, capital adequacy ratio and profitability ratio vis-a-vis the current Bangko Sentral ng Pilipinas (BSP) Performance Indicators for banks, including non-banks with quasi-banking function. Once accredited, on an annual basis, submission of their yearly financial statements is required to monitor financial standing of the ACAs to mitigate any losses that may be incurred by the concerned ACAs. Other documentary requirements such as Articles of Incorporation, Certificate of Good Standing with Existing Industry Association, Electronic Banking Authority, among others are likewise submitted by PhilHealth ACAs when renewing their accreditation to assess their capabilities as collecting agents.

On Cash Management: The Corporation maintains its accounts in any of the Authorized Government Depository Banks (AGDBs) in compliance with DOF Circular No. 01-2017 namely: Land Bank of the Philippines

and Development Bank of the Philippines. These are the depository and servicing banks for the Corporation's over-the-counter collections, disbursements, funding and settlement accounts which are covered by Payroll Servicing Agreement and Memorandum of Agreement on One Fund Disbursement Account, respectively. Hence, lesser vulnerability of default given the Corporation's compliance to regulatory agencies and the service agreements with the AGDBs.

b. Liquidity Risk

This refers to the risk of loss, though solvent, due to insufficient financial resources to cover for liabilities as they fall due. It also involves the risk of excessive costs in securing such resources. This risk also refers to (a) unanticipated changes in liquidity supply and demand that may affect the organization through inability to meet contractual obligations or default (Funding Liquidity Risk) and (b) asset illiquidity or the risk of loss arising from inability to realize the value of assets, without significant reduction in price, due to bad market conditions (Market Liquidity Risk).

PhilHealth manages this risk through daily monitoring of cash flows in consideration of future payment due dates and regular premium collection receipts. PhilHealth also maintains sufficient portfolio of highly marketable assets that can easily be liquidated as protection against unforeseen interruption to cash flow such as Special Savings Deposits with AGDBs, Treasury Bills and Government Securities with remaining life of less than one (1) year.

As to management of ACAs, they are required to remit their collections to PhilHealth on a daily basis in accordance with the DOF Circular Nos. 01-2015 dated June 1, 2015 and 01-2017 dated May 11, 2017. Compliance to existing covenants in the Collection and Remittance Agreements with ACAs is strictly covenants in the Collection and Remittance Agreements with ACAs is strictly monitored by the Corporation and close monitoring through daily reconciliation of collections and remittances by ACAs are being done and implemented including the imposition of penalties and interests for late remittances, under remittances and late submission of required reports and documents.

On Cash Management, the Corporation also maintains its collections account with the AGDBs as prescribed in DOF Circular No. 01-2017. The collection comprises remittances from its collecting agents and other receipts which include fund support coming from the National Government via funds releases through the Bureau of Treasury (BTr) and the Department of Budget and Management (DBM). These accounts are covered by service agreements: Memorandum of Agreement on Opening of One-Way Deposit Collection Account and Sweeping of Available Balance to Mother Accounts, Collection and Deposit Pick up Agreement, hence ensuring the steady inflow of funds to meet the daily fund requirements of the Corporation.

c. Interest Rate Risk

Interest rate risk is the risk that the fair value or future cash flows of a financial instrument will fluctuate because of changes in market interest rates.

5. CASH AND CASH EQUIVALENTS

This account is composed of the following:

	2023	2022
Collecting officers	92,901,541	346,093,288
Petty cash fund	4,587,625	3,660,932
Cash in bank	4,140,608,917	4,250,198,797
Special savings deposit (SSD)	26,541,020,999	1,823,749,984
Total	30,779,119,082	6,423,703,001

5.1 Collecting officers represent collections at the end of the month made by collecting officers which are to be deposited on the following working day.

5.2 The Corporation uses the imprest fund system in handling its petty cash fund. This fund is for the use of miscellaneous expenditures

which cannot be conveniently paid by check wherein an officer holding the fund is also properly bonded in accordance with law.

5.3 Cash in banks represent various bank deposits that are unrestricted and available for current operations. Included in this account are cash denominated in foreign currencies which are translated to peso using the closing rate as of reporting dates.

Cash in bank earns interest based on the prevailing interest rates. Interest earned on bank deposits amounted to P1.752 million and P1.660 million in 2023 and 2022 respectively.

5.4 The SSDs are term deposits of one (1) day up to ninety (90) days with interest rates higher than the regular savings deposit rates and evidenced by either passbook or schedule of deposits. The interest rates ranging from 5.151 percent to 5.300 percent and 0.4500 percent to 0.5500 percent as of December 31, 2023 and December 31, 2022, respectively.

6. INVESTMENTS IN TIME DEPOSITS

	2023	2022
Investment in time deposits - local	135,604,790,555	126,425,079,472
Investment in time deposits - foreign	1,306,983	1,318,682
Total	135,606,097,538	126,426,398,154

Investments in Time Deposits are term deposits of 91 to 364 days with interest rate ranging from 5.8000 percent to 6.350 percent and 1.5000 percent to 6.0000 percent as of December 31, 2023 and December 31, 2022, respectively.

7. RECEIVABLES

This account is composed of the following:

	2023	2022 (As Restated)
Receivable from direct contributors	48,538,095,465	36,935,770,827
Due from NGAs	49,237,566,382	21,145,709,574
Due from LGUs	221,579,356	255,783,121
Due from LGUs - PCPN	168,215,436	-
Accrued interest receivable from investment	6,450,894,713	2,837,220,804
Other receivables	48,617,228	48,508,762
Receivables	104,664,968,580	61,222,993,088
Allowance for impairment	(24,062,310,205)	(23,258,920,515)
Receivables, net	80,602,658,375	37,964,072,573

7.1 Receivable from direct contributors represent accruals of premium contributions from direct contributor members based on ACAs and Over-the-counter Collections.

	2023	2022 (As Restated)
Receivable from direct contributors	48,538,095,465	36,935,770,827
Allowance for impairment	(3,173,108,592)	(2,353,248,669)
Receivables, net	45,364,986,873	34,582,522,158

The Expected Credit Loss (ECL) is computed by multiplying the exposure at default by the Loss Rate (1-Collection Efficiency Rate as determine by the Member Management Group).

2023	2022
48,370,557,799 x (1-9344) = 3,173,108,592	33,617,838,133 x (1-9300) = 2,353,248,669

Exposure Default includes only receivable that are deemed at risk of default.

7.2 Due from NGAs account represents premium contributions for the following:

	2023	2022 (As Restated)
NHTS		
No. of enrollees	12,618,921	–
Amount	28,076,492,000	–
Senior Citizen		
No. of enrollees	8,557,139	8,557,139
Amount	20,537,134,800	20,537,134,800
Point-of-Service (POS)		
No. of enrollees	187,623	187,623
Amount	450,295,200	450,295,200
Fortuitous event		
No. of enrollees	56,135	56,135
Amount	134,724,000	134,724,000
Department of Public Works and Highways (DPWH)		
Amount	–	652,684
DBM-Procurement	38,749,501	22,902,889
Due from NGAs – BIR	170,881	–
Total	49,237,566,382	21,145,709,573
Allowance for impairment	(20,671,858,800)	(20,671,858,800)
Net amount	28,565,707,582	473,850,773
Details of Due from NGA's:		
Enrollment of 12,618,921 indigent members under the Listahan 2 of the DSWD categorized	28,076,492,000	
DBM-Procurement	38,749,501	
Due from NGAs – BIR	170,881	
Total 2023	28,115,412,382	
Enrollment of 187,623 qualified members under the 2019 POS program for the period		
January to December 2018 per billing to DBM	450,295,200	
Total 2018	450,295,200	
Enrollment of 2,258,757 Senior Citizens which were automatically renewed for the period January to December 2016 per billing to DBM through DOH [P12,998,827,200 - P7,577,809,200 (Collection)]		
	5,421,018,000	
Less: Allowance for impairment	(5,421,018,000)	
Total 2016	–	
Enrollment of 5,416,178 Senior Citizens for CY 2015 per billing to DBM		
	12,998,827,200	
Less: Allowance for Impairment	(12,998,827,200)	
Total 2015	–	

Enrollment of 56,135 families of which 55,474 families came from Region VIII who were casualties of Typhoon Yolanda and for the extended insurance coverage under the Fortuitous Event Program implemented as a component of the NHIP for the billing period January to December 2014.

134,724,000

Enrollment of 882,204 Senior Citizens for the period of October to December 2014 per billing to DBM

2,117,289,600

Less: Allowance for Impairment

(2,252,013,600)

Total 2014 (Net)

–

Grand Total

28,565,707,582

7.3 Due from LGUs is the account representing the outstanding accounts receivable on premium contributions from various LGUs. The details of the account are as follows:

Region	2023	2022
National Capital Region (NCR)	6,040	6,040
Cordillera Administrative Region (CAR)	105,365	105,365
II	–	1,192,600
III	69,349,028	98,908,028
IVA	127,600	–
V	795,000	795,000
VI	147,530,923	151,000,288
VII	3,665,400	3,775,800
Total	221,579,356	255,783,121
Less: Allowance for Impairment	(215,725,656)	(231,892,977)
Due from LGUs, net	5,853,700	23,890,144

Expected Credit Loss is 97.36% of the exposure default amounting to 221,579,356 as at December 31, 2023 and 90.66% of the exposure default amounting to 255,783,121 as at December 31, 2022.

7.4 Due from LGU- PCPN are fund transferred through frontloaded capitation to contracted Public Network for primary care services to our members. As of reporting period, only four Philhealth Regional Offices have engaged Public Network. The balance of the frontloaded capitation is at P168,215,436.

7.5 Accrued Interest Receivable from investment amounting to P6,450,894,713 and P2,837,220,804 as at December 31, 2023 and December 31, 2022, respectively, represent interest earned as at the reporting date from special savings deposit, time deposit, investment securities at amortized cost which are due for collection on the following year.

7.6 Other Receivables amounting to P48,617,228 and P48,508,762 as at December 31, 2023 and December 31, 2022, respectively, represents disallowances and due from officers and employees.

Allowance for impairment of P1,617,157 was provided due to the ECL as of December 31, 2023. Allowance for impairment for disallowances was reversed as recommended by COA.

8. OTHER RECEIVABLE - INTERIM REIMBURSEMENT MECHANISM (IRM)

8.1 On January 30, 2020, the BODs issued PBR No. 2496, s. 2020 which approved the inclusion of the IRM among the special privileges that may be conferred to the members during fortuitous events and emergencies. It provides:

WHEREFORE, premises considered, the Board RESOLVES, as it is hereby RESOLVED, to APPROVE the inclusion of Interim Reimbursement Mechanism among the special privileges that may be conferred during fortuitous events and emergencies, thereby amending PhilHealth Board Resolution No. 1848, s. 2013 for this purpose;

8.2 Subsequently, the BODs issued PBR No. 2515, s. 2020 dated March 31, 2020 to ratify the implementation of IRM nationwide due to the COVID-19 pandemic. Thus:

WHEREFORE, premises considered, the Board RESOLVES, as it is hereby RESOLVED to RATIFY the implementation of Interim Reimbursement Mechanism nationwide due to the corona virus disease (COVID-19).

8.3 On May 7, 2020, PhilHealth issued CO No. 2020-0033 signed by then PCEO BGen. Ricardo C. Morales. It provides the guidelines and procedures in the recording and reporting of benefit claims under the IRM.

8.4 As at December 31, 2023, a total of P14.97 billion or 100 percent has already been liquidated. As at December 31, 2022, a total of P14.963 billion or 99.9450 percent has already been liquidated out of the P14.971 billion released IRM, leaving a balance of P8.231 million or 0.0550 percent nationwide.

Region	No. of HCIs	Released	Liquidated	Dec 31, 2023 Balance
NCR	162	4,353,410,549	4,353,410,549	-
CAR	19	484,137,869	484,137,869	-
I	49	782,476,917	782,476,917	-
II	13	338,564,867	338,564,867	-
III	105	1,956,388,846	1,956,388,846	-
IV-A	89	1,248,048,505	1,248,048,505	-
IV-B	41	690,075,500	690,075,500	-
V	61	724,616,526	724,616,526	-
VI	1	121,372,688	121,372,688	-
VII	50	1,101,824,043	1,101,824,043	-
VIII	36	505,534,202	505,534,202	-
IX	13	378,753,947	378,753,947	-
X	18	737,824,644	737,824,644	-
XI	28	890,240,858	890,240,858	-
XII	17	483,567,834	483,567,834	-
CARAGA	2	89,487,449	89,487,449	-
BARMM	7	84,524,850	84,524,850	-
Total		14,970,850,094	14,970,850,094	-

Region	No. of HCIs	Released	Liquidated	Dec 31, 2022 Balance
NCR	162	4,353,410,549	4,347,354,599	6,055,950
CAR	19	484,137,869	484,137,869	-
I	49	782,476,917	780,301,917	2,175,000
II	13	338,564,867	338,564,867	-
III	105	1,956,388,846	1,956,388,846	-
IV-A	89	1,248,048,505	1,248,048,505	-
IV-B	41	690,075,499	690,075,499	-
V	61	724,616,526	724,616,526	-
VI	1	121,372,688	121,372,688	-
VII	50	1,101,824,043	1,101,824,043	-
VIII	36	505,534,202	505,534,202	-
IX	13	378,753,947	378,753,947	-
X	18	737,824,644	737,824,644	-
XI	28	890,240,858	890,240,858	-
XII	17	483,567,834	483,567,834	-
BARMM	2	89,487,449	89,487,449	-
CARAGA	7	84,524,850	84,524,850	-
Total		14,970,850,093	14,962,619,143	8,230,950

9. INVENTORIES

Inventories as of December 31, 2023 and December 31, 2022 were stated at cost less any allowance for impairment.

This account includes the following:

	2023	2022
Office supplies and materials inventory	89,600,570	91,980,768
Semi-expendable machinery and equipment	19,294,077	8,842,411
Semi-expendable furniture, fixture and books	18,273,914	23,200,560
Inventories	127,168,561	124,023,739
Less: Allowance for Impairment	(1,452,311)	(1,177,735)
Inventories, net	125,716,250	122,846,004

9.1 Office Supplies and Materials Inventory account amounting to P89.6 million and P91.981 million as at December 31, 2023 and 2022, respectively, represents small tangible items that are expected to be used within one year from the reporting date.

9.2 Semi-Expendable Machinery and Equipment account amounting to P19.294 million and P8.842 million as at December 31, 2023 and 2022, respectively, represents office equipment, information and communication technology costing less than P50,000.

9.3 Semi-Expendable Furniture, Fixtures, and Books account amounting to P18.273 million and P23.201 million as at December 31, 2023 and 2022, respectively, represents furniture, fixtures and books costing less than P50,000.

10. OTHER CURRENT ASSETS

This account is composed of the following:

	2023	2022
Other Receivables		
Creditable Withholding Tax	-	1,493,156
Other Current Assets	2,776,735,267	70,833,859
Other Current Assets	2,776,735,267	72,327,015
Less: Allowance for Impairment	(54,184,980)	(43,957,427)
Other Current Assets, net	2,722,550,287	28,369,588

10.1 Remittance of taxes withheld from the released IRM amounted to nil and P1.493 million as at December 31, 2023 and 2022, respectively.

10.2 Other Current Assets amounting to P2.777 billion and P70.834 million, as at December 31, 2023 and 2022, respectively, represent due from officers/employees and fines/penalties from ACAs and Fines and Penalties from ACAs, payment recovery, DCPM amounting to P2.701 billion and compromise penalties of hospitals amounting to P24.264 million for the taxable year 2003-2004 per Bureau of Internal Revenue (BIR) decision with reference No. P06-15 dated April 14, 2015.

The Debit-Credit Payment Method (DCPM) was introduced in 2021 during the pandemic to comply with Section 47(l) of the Revised Implementing Rules and Regulations of the National Health Insurance Act of 2013 of Republic Act No. 7875, as amended by R.A. Nos. 9241 and 10606, prescribes that "all completed claims, except those under investigation, shall be paid within the sixty (60) calendar days from the receipt of the Corporation. As such, for in process claims ageing 60 days and beyond were paid and tagged as DCPM population subject to processing and reconciliation.

The DCPM recorded as Other Current Assets amounting to 2,701,167,046.00 are now subject to reconciliation and Payment Recovery Policy of the Corporation.

10.3 Allowance for Impairment amounting to P54.185 million was provided due to the ECL as at December 31, 2023. Details as follows:

	Total Amount	Allowance	Net Amount
Other Receivables - Accrued Fines & Penalties from ACAs	3,015,562	(2,987,172)	28,390
Other Receivables - Personal Calls	69,237	(69,043)	194
Other Receivables - Refund from Benefit Payment – Employer	30,960,520	(29,574,738)	1,385,782
Other Receivables - Filing Fees	210,500	-	210,500
Other Receivables – GPAI	67,473	(67,473)	-
Other Receivables – Payment Recovery	13,907,670	-	13,907,670
Other Receivables - DCPM	2,701,167,046	-	2,701,167,046
Other Receivables – Others	27,337,259	(21,486,554)	5,850,705
Total	2,776,735,267	(54,184,980)	2,722,550,287

11. INVESTMENT SECURITIES AT AMORTIZED COST

This account is composed of the following:

	2023	2022
Investment in Treasury Bills	-	9,350,077,536
Investment in Government Bonds	331,680,257,444	270,732,356,462
Investment in Corporate Bonds	4,447,910,000	1,100,000,000
Total	336,128,167,444	281,182,433,998
Less: Allowance for Impairment	(531,590)	(551,193)
Investment, net	336,127,635,854	281,181,882,805

11.1 Investment in Treasury Bills with a maturity period of 91 to 364 days and with an average interest rate of 3.6475 percent as at December 31, 2022. Treasury Bills has matured and no new placement as at December 31, 2023:

	Interest Rate	2023	2022
Treasury Bills	3.4850%	-	5,615,365,067
Treasury Bills	3.4850%	-	1,429,808,756
Treasury Bills	3.8100%	-	1,962,204,668
Treasury Bills	3.8100%	-	342,699,045
Sub-Total		-	9,350,077,536

11.2 Investment in Government Bonds with a maturity period of more than one year and with an average interest rate of 3.8995 and 3.9794 percent as at December 31, 2023 and 2022, respectively.

	Interest Rate	2023	2022
Retail T-bonds	4.7083%	71,000,000,000	78,316,890,000
RTB non-restricted environment	5.3151%	258,180,257,444	192,415,466,462
Tokenized Treasure Bonds	6.5000%	2,500,000,000	-
Sub-Total		331,680,257,444	270,732,356,462

11.3 Investment in Corporate Bonds with a maturity period of more than one year and with interest rates ranging from 4.8256 percent to 5.9024 percent:

	Interest Rate	2023	2022
7-yr Ayala Land Inc. Fixed Rate Bonds	3.8915	-	1,100,000,000

2.5 yrs SM Prime Holdings Fixed Rate Bonds	6.2069	1,000,000,000	-
4 yrs SM Prime Holdings Fixed Rate Bonds	6.2151	1,000,000,000	-
6 yrs SM Prime Holdings Fixed Rate Bonds	6.3275	500,000,000	-
3 yrs Robinson Land Corp. Fixed Rate Bonds	6.0972	270,000,000	-
3 yrs Robinson Land Corp. Fixed Rate Bonds	6.1663	856,910,000	-
1.5 yr DBP Fixed Rate Bonds	6.4126	821,000,000	-
Sub-Total		4,447,910,000	1,100,000,000
Less: Allowance for Impairment		(531,590)	(551,193)
Total		4,447,378,410	1,099,448,807

11.4 Interest earned on Investment Securities at Amortized Cost amounted to P12.163 billion and P9.920 billion as at December 31, 2023 and December 31, 2022, respectively.

12. INVESTMENT PROPERTY

This account is composed of the following:

12.1 A parcel of land situated in Quezon City with a total area of 17,231 square meters (sq. m.)

This property has a carrying amount of P439.378 million. The Head Office of PhilHealth shall be constructed in this lot.

Relative to the construction of the proposed PhilHealth Corporate Center, the Corporation acquired an additional 145 sq. m. lot from the National Housing Authority located in Barangay Pinyahan, East Avenue, Quezon City at a cost of P2.440 million. It has a fair market value of P3.274 billion as of July 30, 2021

12.2 A property with a total area of 4,355 sq. m. acquired from Fort Bonifacio Development Corporation (FBDC), in exchange for the Corporation's investments in FBDC located in Fort Bonifacio, Taguig City. This asset with a carrying amount of P413.846 million was appraised with a fair market value of P3.354 billion as at August 6, 2021.

13. PROPERTY AND EQUIPMENT

This account is composed of the following: (refer to the table on next page)

Included under Land and Buildings accounts are the following:

13.1 A parcel of lot with total area of 1,544 sq. m. and a building with a total floor area of 3,287 sq. m. purchased by PRO No. III for its permanent Regional Office in San Fernando, Pampanga at a cost of P25.520 million. The land and building were appraised to have a fair market value of P73.710 million as at August 4, 2021. PRO No. III also purchased a warehouse in San Fernando City, Pampanga with a total area of 1,831 sq. m. and was appraised with a fair market value of P22.025 million as at August 4, 2021.

13.2 A parcel of lot with a total area of 2,897 sq. m. located in Tuguegarao City which will be used as the PRO No. II's Regional Office. This property was received by way of donation, through a MOA executed between DPWH Region II and the Corporation. This property which is carried in the books at P4.056 million was appraised with a fair market value of P51.280 million as at August 10, 2021. In compliance with the condition set forth in the MOA to construct PhilHealth's office building within two years from the execution of the Deed of Donation, the Regional Development Council (RDC) II has extended, upon the request of PRO No. II, the construction of the building for another two years per RDC-2 Resolution No. 02-25, series of 2017.

13.3 A parcel of lot with a total area of 600 sq. m. located in Gov. Democrito O. Plaza Government Center, Patin-ay, Prosperidad, Agusan del Sur which will be used as the PRO CARAGA's Local Health Insurance Office (LHIO) San Francisco Corporate Office. This property was received by way of donation, through Deed of

2023

	Land and land improvements	Building and structure / leasehold improvements	Infra. Assets	Construction in progress	Furniture & fixtures equipment & books	Motor vehicles	Total
Cost							
Balance at beginning of year	20,514,062	264,490,613	-	29,017,394	2,248,131,498	333,671,182	2,895,824,749
Additions	-	12,175,674	161,613	1,691,463	101,233,775	259,730,500	374,993,025
Adjustments	-	(5,223,843)	-	(67,479)	(96,798,220)	(151,592,631)	(253,682,173)
Balance at end of year	20,514,062	271,442,444	161,613	30,641,378	2,252,567,053	441,809,051	3,017,135,601
Accumulated Depreciation							
Balance at beginning of year	1,498,376	175,073,281	-	-	1,699,918,231	194,947,882	2,071,437,770
Depreciation	-	23,620,225	-	-	163,212,719	36,501,497	223,334,441
Adjustments	-	(5,599,515)	-	-	(75,574,606)	(24,121,832)	(105,295,953)
Balance at end of year	1,498,376	193,093,991	-	-	1,787,556,344	207,327,547	2,189,476,258
Net Book Value, Dec. 31, 2023	19,015,686	78,348,453	161,613	30,641,378	465,010,709	234,481,504	827,659,343

2022 (As Restated)

	Land and land improvements	Building and structure / leasehold improvements	Infra. Assets	Construction in progress	Furniture & fixtures equipment & books	Motor vehicles	Total
Cost							
Balance at beginning of year	20,514,062	252,045,198	-	27,870,253	2,287,996,767	230,094,371	2,818,520,651
Additions	-	19,093,988	-	1,147,141	83,353,275	106,190,000	209,784,404
Adjustments	-	(6,648,573)	-	0.00	(123,218,544)	(2,613,189)	(132,480,306)
Balance at end of year	20,514,062	264,490,613	-	29,017,394	2,248,131,498	333,671,182	2,895,824,749
Accumulated Depreciation							
Balance at beginning of year	1,498,376	153,134,467	-	-	1,542,123,679	183,055,928	1,879,812,450
Depreciation	-	28,212,563	-	-	213,565,542	14,493,515	256,271,620
Adjustments	-	(6,273,749)	-	-	(55,770,990)	-	(64,646,300)
Balance at end of year	1,498,376	175,073,281	-	-	1,699,918,231	194,947,882	2,071,437,770
Net Book Value, Dec. 31, 2023	19,015,686	89,417,332	-	29,017,394	548,213,267	138,723,300	824,386,979

Donation executed by the Provincial Government of Agusan del Sur in favor of PhilHealth office. This property is recorded in the books at P1.086 million.

14. RIGHT-OF-USE ASSETS

Right-of-Use (ROU) assets are office spaces and warehouses being leased by PhilHealth with a multi-year contract. Summary of ROU Assets follows:

2023			
	Total Amount	Accumulated Depreciation	Net Amount
Head Office	441,721,703	(395,366,236)	46,355,467
PRO NCR	149,228,616	(74,035,177)	75,193,439
PRO CAR	19,802,526	(4,950,887)	14,851,639
PRO I	5,433,703	(2,074,372)	3,359,331
PRO III	141,479,810	(49,160,411)	92,319,399
PRO IV-A	82,520,259	(55,394,278)	27,125,981
PRO IV-B	107,158,529	(62,564,750)	44,593,779
PRO V	78,146,519	(3,606,841)	74,539,678
PRO VI	22,285,297	(4,433,338)	17,851,959
PRO VII	87,505,524	(83,885,597)	3,619,927
PRO VIII	12,941,045	(10,127,694)	2,813,351
PRO XI	50,112,775	(43,754,111)	6,358,664
PRO CARAGA	70,722,319	(47,534,567)	23,187,752
PRO BARM	16,704,773	(16,704,773)	-
Total	1,285,763,398	(853,593,032)	432,170,366

2022			
	Total Amount	Accumulated Depreciation	Net Amount
Head Office	441,721,774	(293,281,906)	148,439,868
RO NCR	149,228,616	(74,035,177)	75,193,439
PRO CAR	22,643,591	(16,413,950)	6,229,641
PRO I	1,149,859	(263,138)	886,721
PRO III	118,134,934	(24,292,816)	93,842,118
PRO IV-A	75,637,436	(49,445,053)	26,192,383
PRO IV-B	101,425,110	(46,308,742)	55,116,368
PRO VI	6,212,331	(5,917,680)	294,651
PRO VII	87,505,524	(57,171,314)	30,334,210
PRO VIII	10,961,211	(7,068,469)	3,892,742
PRO X	92,574,846	(85,355,036)	7,219,810
PRO XI	50,243,144	(34,898,007)	15,345,137
PRO CARAGA	76,467,794	(37,367,887)	39,099,907
PRO BARM	16,704,773	(13,363,819)	3,340,954
Total	1,250,610,943	(745,182,994)	505,427,949

Details of ROU Assets as at December 31, 2023 are as follows:

Lessor	Use	Lease Period	Amount
Head Office			
Zapanta Realty & Dev't. Corp.	Office Space	06/01/2019 to 05/31/2024	198,829,376
Columbia Estate Properties, Inc.	Office Space	01/01/2021 to 05/31/2024	129,128,366
Columbia Estate Properties, Inc.	Warehouse	03/01/2019 to 02/29/2024	26,963,394
Fortune General Ins. Corp.	Office Space	06/01/2019 to 05/31/2024	14,095,120
Fortune Life Ins. Co., Inc.	Office Space	06/01/2019 to 05/31/2024	37,264,465
Columbia Estate Properties, Inc.	Office Space	01/01/2019 to 05/31/2024	35,440,982
Sub-Total Head Office			441,721,703
PRO NCR			
Westco Electrical & Equipt. Corp.	PRO Space	12/01/2017 to 11/30/2022	18,795,649
Metro Dry Clean Ventures, Inc.	PRO Warehouse 1	01/01/2019 to 04/30/2021	2,708,024
CAS Properties, Inc.	PRO Warehouse 2	07/16/2018 to 07/15/2023	4,207,385
Columbia Estate Properties	Central Branch Office & LHIO QC	01/01/2018 to 12/31/2022	34,835,751
Fibertex Corporation	LHIO Rizal	03/01/2017 to 02/28/2022	6,689,639
Our Workshop Sales (OWS)	LHIO Fairview	06/01/2017 to 05/31/2022	6,148,054
Intraland Resource, Inc.	North Branch Office	01/01/2019 to 12/31/2021	903,969
Guru Property Dev't. & Mgt. Corp.	LHIO Caloocan	01/01/2019 to 11/30/2021	6,901,710
Lica Management, Inc.	LHIO Mandaluyong	05/02/2018 to 04/30/2023	8,816,271
Cromagen Land Corporation	South Branch Office	07/01/2019 to 06/30/2024	40,845,095
Industrial Timber Corporation	LHIO Makati	01/01/2019 to 03/31/2021	903,969
Iriz One Properties Inc.	LHIO Pasig	01/01/2019 to 04/30/2021	6,964,698
Editha Building	LHIO Las Pinas	01/01/2019 to 10/31/2019	1,671,983
BNC Properties, Inc.	LHIO Paranaque	08/01/2018 to 07/31/2022	8,836,419
Sub-Total PRO NCR			149,228,616
PRO CAR			
Lester Astudillo	Abra Office Space	04/01/2022 to 03/03/2027	5,276,892
Montanosa Pastoral Resources Corp	Baguio Office Space	04/01/2023 to 03/03/2025	12,399,666
NCIP KALINGA		01/01/2019 to 02/28/2023	1,086,791
Precious Sarah Residence-Apayao		07/16/2023 to 07/15/2025	1,039,177
Sub-Total PRO CAR			19,802,526
PRO I			
Municipality of Laao, Pangasinan	LHIO Eastern Pangasinan Warehouse	03/01/2022 to 02/28/2025	339,610

B & D Bldg, Allied Properties	LHIO Western Pangasinan	05/16/2022 to 05/15/2025	810,249
Marmor Realty	LHIO Western Pangasinan	01/01/2023 to 12/31/2025	1,903,116
Henady Real Estate Lessor	LHIO Ilocos Sur	01/01/2023 to 12/31/2025	2,380,728
Sub-Total PRO I			5,433,703
PRO III			
3Aces Commercial Building	LHIO Sta. Maria	06/01/2023 to 07/31/2028	14,769,206
Tarlac Metrotown Corporation	LHIO Tarlac	12/15/2023 to 12/14/2028	8,575,670
Dona Rita Realty Corp.	Branch Office	07/06/2022 to 07/05/2027	53,503,199
Analita T. Mangahas	LHIO Gapan	08/01/2022 to 07/31/2027	10,418,399
Vicente Lim	Warehouse	04/10/2019 to 03/31/2024	13,562,850
Oscar Santos	LHIO Olongapo	08/01/2022 to 07/31/2027	18,232,199
JCB Building	LHIO Gapan satellite office	11/09/2020 to 11/28/2025	1,426,970
Juan D. Nepomuceno Sons, Inc.	LHIO Angeles	04/30/2021 to 04/29/2026	14,730,365
Alice B. Gonzales	LHIO Iba	04/30/2021 to 04/29/2026	6,260,952
Sub-Total PRO III			141,479,810
PRO IV-A			
Lucena Grand Central Terminal, Inc.	Storage Space	10/01/2019 to 09/30/2024	4,231,115
Lucena Grand Central Terminal, Inc.	PRO Office Space	12/01/2019 to 11/30/2024	25,183,214
Calamba Medical Center	LHIO Calamba	11/01/2019 to 10/31/2024	10,484,741
Calamba Medical Center	LHIO Calamba	07/01/2020 to 06/30/2025	2,491,822
Premium Link Devt. Corp.	LHIO Dasmarias	05/01/2019 to 04/30/2024	14,027,937
One FS Industrial Corp.	LHIO Dasmarias Storage Space	08/01/2019 to 07/31/2024	1,411,917
One FS Industrial Corp.	LHIO Trece Martires Storage Space	08/01/2019 to 07/31/2024	1,268,189
Manuelito Lorica	LHIO Gumaca Office Space	02/01/2021 to 01/31/2026	6,676,068
James Uy	LHIO SPC Office Space	08/01/2023 to 07/31/2028	16,745,256
Sub-Total PRO IV-A			82,520,259
PRO IV-B			
Willy D. Tan	LHIO Lipa Warehouse	09/05/2023 to 09/04/2028	1,644,537
Millorey, Inc.	LHIO Batangas City	09/05/2019 to 08/31/2024	7,252,250
Edgardo P. Perez	LHIO Tanauan	06/02/2022 to 06/02/2027	17,562,077
Feature Realty Holdings & Dev't. Corp.	LHIO Lemery	11/27/2019 to 11/31/2024	5,539,757
XRC Mall Developer	Regional Office	07/01/2020 to 06/30/2025	59,818,372

Karangyan Builders & Traders	LHIO Oriental Mindoro	06/01/2021 to 05/31/2026	6,425,658
Bernardo B. Go, Jr.	LHIO-Palawan	06/29/2023 to 06/28/2028	8,915,878
Sub-Total PRO IV-B			107,158,529
PRO V			
ANST Corp.	Office Space	12/31/2023 to 11/30/2026	45,757,070
SORCOM Leasing	Office Space	12/31/2023 to 11/30/2026	6,539,562
Westhub	Office Space	10/01/2023 to 09/30/2026	10,845,164
Delos Santos	Office Space	10/01/2023 to 09/30/2026	15,004,723
Sub-Total PRO V			78,146,519
PRO VI			
HEVA Management & Devt. Corp.	LHIO Ilollo	01/01/2023 to 12/31/2027	17,394,808
Barangay Caticlan	Philhealth Express	06/16/2023 to 06/15/2028	258,777
Northern Negros Planters Asso	LHIO Sagay	01/01/2023 to 12/31/2027	4,631,712
Sub-Total PRO VI			22,285,297
PRO VII			
Province of Siquijor	Office Space	01/01/2017 to 12/31/2026	2,585,425
Elsita A. Panizales (Bais)	Office Space	07/01/2022 to 06/30/2024	1,547,470
Carcar City - LGU	Office Space	07/22/2019 to 06/21/2024	6,160,726
JGY Land Corporation	Office Space	09/16/2019 to 09/15/2024	14,396,243
Talibon DC General Merchandise	Warehouse	03/16/2020 to 03/15/2025	215,756
Hancit Realty & Dev't Corp.	Office Space	01/01/2020 to 12/31/2023	19,159,822
Skytower Realty & Dev't Corp.	Office Space	01/01/2020 to 12/31/2023	43,440,082
Sub-Total PRO VII			87,505,524
PRO VIII			
Hugh Trevor Realty Warehouse	Office Space	12/15/2022 to 12/15/2025	1,979,835
R.K. Eng'g. & Construction Works	Office Space	07/01/2019 to 06/30/2024	1,853,819
MAV Properties	Office Space	04/16/2019 to 04/15/2024	6,016,069
EC Cereno Rental Services	Office Space	07/01/2019 to 06/30/2024	1,744,771
CINCO Real Property Estate Leasing	Office Space	12/09/2022 to 11/18/2025	1,346,551
Sub-Total PRO VIII			12,941,045
PRO XI			
DDIS Inc.	Warehouse	7/1/2022 to 06/30/2027	5,767,069
828 Commercial Building	Office Space	07/01/2022 to 06/30/2027	1,228,796
Valgosons Realty Incorporated	LHIO Tanauan	01/16/2019 to 01/15/2024	43,116,910
Sub-Total PRO XI			50,112,775

PRO CARAGA

Abcalo Commercial Building	Office Space	07/01/2020 to 06/30/2025	43,598,300
Mn Bayalas Commercial	LHIO Bislig	03/01/2020 to 02/28/2025	4,877,827
CARAGA Ramlizdy Corp.	LHIO Butuan City	12/01/2020 to 11/30/2025	12,129,493
Saint Francis Realty	Office Space	06/01/2022 to 05/31/2025	7,060,208
JTP Realty	LHIO Tandag	03/01/2022 to 02/28/2025	3,056,491
Sub-Total PRO CARAGA			70,722,319

PRO BARMM

Kouzgary Builders	PRO Space	01/01/2019 to 12/31/2023	16,704,773
Sub Total PRO BARMM			16,704,773
GRAND TOTAL			1,285,763,398

15. INTANGIBLE ASSETS - Net

This account is composed of the following:

	2023			
	Computer Software	Website	Devt. In Progress - Computer Software	Total
Cost				
Balance at beginning of year	478,285,622	31,940	47,651,497	525,969,059
Additions	-	-	-	-
Adjustments	55,091,737	-	-	55,091,737
Balance at end of year	533,377,359	31,940	47,651,497	581,060,796
Accumulated Amortization				
Balance at beginning of year	261,227,306	-	-	261,227,306
Amortization	5,468,226	-	-	5,468,226
Adjustments				
Balance at end of year	266,695,532	-	-	266,695,532
Accumulated Impairment Loss	55,012,145	-	-	55,012,145
Carrying Amount	211,669,682	31,940	47,651,497	259,353,119

	2022 As Restated			
	Computer Software	Website	Devt. In Progress - Computer Software	Total
Cost				
Balance at beginning of year	467,617,382			467,617,382
Additions	-	-	47,651,497	47,651,497
Adjustments	10,668,240	31,940	-	10,700,180
Balance at end of year	478,285,622	31,940	47,651,497	525,969,059
Accumulated Amortization				
Balance at beginning of year	250,247,248	-	-	250,247,248
Amortization	10,206,770	-	-	10,206,770
Adjustments	773,288			773,288
Balance at end of year	261,227,206	-	-	261,227,206
Carrying Amount	217,058,316	31,940	47,651,497	264,741,753

This account mainly pertains to various software applications/ programs, the majority of which are licenses to fully utilize the capability of a software system to operate in a virtual environment. It includes computer development in progress – computer software which up to now is for customization.

From the total cost amount, intangibles with finite life is valued at P271,042,377 while intangibles with indefinite life is at P207,354,777 of which are internally developed and are being used in the operation amounting to P11,046,970. The balance is for development in progress – computer software.

16. OTHER NON-CURRENT ASSETS

This account is composed of the following:

	2023	2022 (Restated)
Advances to Special Disbursing Officers	72,967	–
Advances to Officers & Employees	1,536,198	350,378
Prepayments	22,596,881	15,120,828
Guaranty Deposits	123,759,591	111,682,753
Other Deposits	3,007,765	3,155,548
Sub-Total	150,973,402	130,309,507
DBM (transfer of NHIP Program from GSIS to PHIC)	155,235,240	155,235,240
PDIC (per MB Reso. 459 dated 04/07/2005)	327,103	327,103
PROs (from Various Health Providers-DCS)	700,555	700,555
Unserviceable Equipment	47,818,721	48,874,315
Serviceable equipment	1,540,186	1,095,251
Receivable from NGAs	394,749,368	394,741,430
Receivable from LGUs	448,977,370	481,189,230
Other assets - Intangibles	–	6,287,168
Gross Long-term receivable	1,049,348,543	1,088,450,292
Less: Allowance for Impairment	(992,141,729)	(1,025,489,515)
Sub-Total	57,206,814	62,960,777
Grand Total	208,180,216	193,270,284

- 16.1 The Special Disbursing Officer is an Accountable Officer duly designated, responsible and accountable for the proper management of funds for a specific legal purpose or activity and is properly bonded in accordance with law. The amount is P72,967 and nil as at December 31, 2023 and December 31, 2022, respectively. The amount represents the funds on hand from the said officers as of reporting date.
- 16.2 Advances to Officers and Employees amounting to P1.536 million and P350,378 as at December 31, 2023 and 2022, respectively, represent cash advances granted to authorized officers and employees for legal authorized purpose such as local and foreign travels.
- 16.3 Prepayments amounting to P22.599 million and P15.121 million as at December 31, 2023 and 2022, respectively, represent authorized payments made for insurance of motor vehicles of the Corporation from the Government Service Insurance System (GSIS), prepaid rent, prepaid registrations and other prepayments.
- 16.4 Guaranty Deposits amounting to P123.760 million and P111.683 million as at December 31, 2023 and 2022, respectively, represent transactions made by the Head Office and PROs in compliance with the requirements provided in the contracts for office rentals. These deposits are made for the faithful performance of the provisions of the lease agreements and

shall cover possible damages to the leased premises. These are refundable at the end of the service agreement.

- 16.5 Long Term Receivable from the DBM amounting to P155.235 million as at December 31, 2023 and 2022 represents surcharges for late remittance of the employer counterpart for premium contribution. However, allowance for impairment account of the same amount was provided after evaluation of factors such as aging of accounts, collection experience in relation to particular receivable and identified doubtful accounts.
- 16.6 Long Term Receivable from Philippine Deposit Insurance Corporation (PDIC) amounting to P327,103 as at December 31, 2023 and 2022 was in pursuant to Monetary Board (MB) Resolution No. 459 dated April 7, 2005 placing Hermosa Savings and Loan Bank, Inc. under liquidation. As of last communication last March 10, 2023 as per directive of Audit Committee, no collection has been received. Nevertheless, PDIC has mention that we on the priority list for payment upon liquidation of assets.
- 16.7 The Debit-Credit Scheme (DCS) amounting to P0.701 million as at December 31, 2023 and 2022, refers to the balance of advance payment to Health Care Provider (HCPs) for the year 1999. Allowance for impairment of P0.701 million was provided for due to closure of the hospital facilities and as prescribed by PFRS 9.
- 16.8 Unserviceable Equipment account amounting to P47.819 million and P48.874 million and as at December 31, 2023 and 2022, respectively, represent equipment that are already for disposal. This account shall be further reclassified as Non-Current Asset – Held for Sale, once the requirements set upon by the Standard are met.
- 16.9 Serviceable Equipment account amounting to P1.540 and P1.095 million as at December 31, 2023 and 2022, respectively, represent pieces of equipment which are still functional but already obsolete and fully depreciated and ready for disposal. This account shall be further reclassified as Non-Current Asset – Held for Sale, once the requirements set upon by the Standard are met.
- 16.10 Receivable from NGAs amounting to P394.749 million and P394.741 million as at December 31, 2023 and 2022 represents deficiency in employer share of the Health Insurance Premium Contributions to the Corporation by different government agencies nationwide for CYs 2001 to 2008.
- 16.11 Receivable from LGUs represents long overdue premium contributions from various cities and municipalities all over the Philippines for the enrollment of their respective constituents under the Sponsored Program of the Corporation. It was reclassified from Current Receivable to Other Assets in compliance with COA recommendation per Audit.

Observation Memorandum (AOM) No. 2015-22 (14) dated May 5, 2015, details as shown below.

PRO	2023	2022
NCR	1,307,600	1,307,600
CAR	20,893,161	22,806,503
I	35,962,267	35,962,267
II	14,914,961	15,914,961
IV-A	14,124,485	14,124,485
IV-B	938,740	938,740
V	278,980,773	281,853,173
VIII	6,079,686	6,940,502
X	1,342,545	21,456,375
XI	13,036,341	16,546,341
CARAGA	59,480,259	61,421,731
BARMM	1,916,552	1,916,552
Total	448,977,370	481,189,230

16.12 Other Assets – Intangibles – These are reclassified from Intangible Assets which were fully amortized and was left with their residual value only amounting to nil and P6.287 million as at December 31, 2023 and 2022, respectively. They no longer received support from the supplier and are no longer used in the operations.

Derecognition of intangibles was recommended after re-assessment/ impairment test. All intangibles that were derecognized are either obsolete or retired and longer receive support from the supplier.

Continuous assessment or impairment test is being done to determine if the remaining intangibles will be derecognized when found to be no future economic benefit or services are expected from its use or disposal.

16.13 Allowance for impairment was provided for the following uncollectible receivable accounts as recommended by COA per AOM No. 17-003 dated February 28, 2017 in accordance with COA Circular No. 2016-005 dated December 19, 2016 and based on the Corporation's policy on the setting up of allowance for doubtful/uncollectible/impairment accounts receivable per PhilHealth Corporate Order No. 2017-0010 dated January 20, 2017.

Per PhilHealth CO No. 2017-0010, the Corporation shall establish and maintain an Allowance for Doubtful/Impairment Accounts representing an estimate of the amount of accounts receivable that will not be collected. The goal in recording this allowance is to show, as accurately as possible, the net realizable value of accounts receivable on the financial reports. The allowance for uncollectible accounts shall be updated at the end of the fiscal year. The Corporation shall adopt the Allowance Method in recording bad debts in its books of account and use the Specific Identification Method in determining the amount of uncollectible in setting up the allowance for doubtful accounts.

PRO	2023	2022
DBM (transfer of NHIP Program from GSIS to PhilHealth)	155,235,240	155,235,240
PDIC (per MB Resolution 459 dated April 7, 2005)	327,103	327,103
PROs (from various Health Providers-DCS)	700,555	700,555
Receivable from NGAs	394,741,430	394,741,430
Receivable from LGUs	441,137,401	474,485,187
Total Allowance for Impairment	992,141,729	1,025,489,515

17. FINANCIAL LIABILITIES

This account is composed of the following:

	2023	2022 (As Restated)
Accounts payable	1,008,249,743	941,329,027
Accrued benefits payable	37,238,607,745	54,977,762,554
Due to officers and employees	476,810,765	464,537,810
Operating lease payable	4,573,713	4,257,351
Other financial liabilities	15,000	15,000
Total Allowance for Impairment	38,728,256,966	56,387,901,742

Below is the breakdown of the Accounts Payable:

	2023	2022
Personnel Services	28,162,954	32,124,909
MOOE	919,290,262	778,920,888
CAPEX	60,796,527	130,283,230
Total Allowance for Impairment	1,008,249,743	941,329,027

17.1 Accounts Payable – Personnel Services amounting to P28.163 million and P32.125 million as at December 31, 2023 and 2022, respectively represent per diems and allowances of the BODs and Committee members which are due for payment. This also includes the GSIS premium amounting to P20.605 million which was part and parcel of the salary adjustments given to PhilHealth employees but subsequently disallowed by COA. The said amount is still outstanding and not yet remitted to GSIS per Opinion No. 56 Series of 2018 from the Office of the Government Corporate Counsel (OGCC) dated March 26, 2018 stating that PhilHealth should keep the amount originally intended to be remitted to GSIS as premiums corresponding to the adjusted salaries of its employees without prejudice to its remittance in the event the Notice of Disallowance (ND) is lifted.

17.2 Accounts Payable – Maintenance and Other Operating Expenses (MOOE) amounting to P919.290 million and P778.921 million as at December 31, 2023 and 2022, respectively, represent procurement of goods or services which are due for payment.

17.3 Accounts Payable – Capital Expenditure (CAPEX) amounting to P60.797 million and P130.283 million as at December 31, 2023 and 2022, respectively, represents procurement of property and equipment which are due for payment.

17.4 Accrued Benefits Payable are accrued expenses from claims filed by members or HCIs and received by the corporation but not yet processed and unpaid as at reporting period.

	2023	2022 (As Restated)
Benefit Claims Processed	4,750,316,499	7,656,040,443
Benefit Claims Processed-Primary Care Benefit (PCB)	2,670,515	6,847,697
Benefit Claims Processed-Konsulta	40,087,748	793,488
In-Course of Settlement (ICS)	32,037,165,698	38,179,864,717
ICS – in process	29,817,405,775	32,649,449,704
ICS – RTH	2,219,759,923	5,530,415,013
PCB	132,940,453	8,930,105,106
Konsulta	275,426,832	204,111,103
Total	37,238,607,745	54,977,762,554

17.5 Accrued Benefit Payable – Claims Processed amounting to P4.750 billion and P7.656 billion as at December 31, 2023 and December 31, 2022, respectively, represent benefit claims awaiting payment for the next crediting date. It also includes pending claims payment due to non-submission of official receipt from the HCI as well non-negotiated checks by the HCI as of reporting date.

17.6 Accrued Benefit Payable – Primary Care Benefit (PCB) processed amounting to P2.670 million and P6.847 million as at December 31, 2023 and December 31, 2022, respectively, represent PCB awaiting payment.

17.7 Accrued Benefit Payable – Konsulta processed amounting to P40.088 million and P793,488 represents benefit claims for Konsulta package awaiting payment as at December 31, 2023 and December 31, 2022, respectively.

17.8 Accrued Benefits Payable – ICS amounting to P32.037 billion and P38.180 billion as at December 31, 2023 and 2022, respectively, are estimated benefit claims still in process as of the reporting period. The amount recorded is from the valuation report of Actuary. The remaining prior year ICS was transferred to IBNR to conform with the provision of Incurred But Not yet Paid (IBNP) per valuation report.

17.9 Accrued Benefits Payable – PCB amounting to P.132 billion and P8.930 billion as at December 31, 2023 and 2022, respectively, are claims which are actuarially estimated to be outstanding as of

report date. The decrease was in compliance with the DBM and COA Joint Circular No.99-6 and No. 1, s.2021 on reversion of payables which remained outstanding for 2 years to accumulated surplus or deficit.

- 17.10 Due to Officers and Employees amounting to P476.811 million and P464.538 million as at December 31, 2023 and 2022, respectively, represent payables to officers and employees for salaries, benefits and other emoluments including authorized expenses paid in advance by the officers and employees.

Included in the accounts payable is the accrued Personnel Services for Welfare Support Assistance (WESA) from CYs 2012 to 2020. Below is the basis of accruing the said assistance, to wit:

The PhilHealth BODs issued PBR No. 385, s. 2001, granting the payment of the WESA of P4,000 each to all qualified employees effective January 1, 2001. This is in lieu of the subsistence and laundry allowances paid to public health workers under RA No. 7305, otherwise known as the Magna Carta of Public Health Workers. However, On February 7, 2008, the COA issued ND No. PHIC 2008-003 (2004) disallowing the payment of WESA contending that the payment was made without legal basis in the absence of approval from the Office of the President. On Appeal, the COA en banc sustained the disallowance.

PhilHealth then filed a Petition before the Supreme Court (SC) seeking to reverse and set aside the Decision of the COA. The SC in its Decision dated November 29, 2016, held that the Health Secretary approved the grant of the WESA together with 10 other members of the Board does not make the act any short of the approval required under the law. As far as the Magna Carta and its Revised Implementing Rules and Regulations (RIRR) are concerned, the then Health Secretary Dr. Alberto G. Romualdez, Jr. voted in favor of the WESA's issuance, and for as long as there exists no deception or coercion that may vitiate his consent, the concurring votes of his fellow Board members does not change the fact of his approval. PhilHealth's grant of the WESA was aptly sanctioned not only by Section 12 of the Salary Standardization Law which explicitly identifies laundry and subsistence allowance as excluded from the integrated salary, but also by statutory authority, particularly, Sections 22 and 24 of the Magna Carta. In view of such fact, the PhilHealth officers cannot be found to have approved the issuance of the same in bad faith or in gross negligence amounting to bad faith for it was well within the parameters set by law.

As a result of the decision of the SC (GR No. 213453) reversing the ND and COA decision on WESA on November 29, 2016, PhilHealth Management, with the concurrence of the BODs, decided that such WESA should be given retroactive effect so that the rate of WESA, in lieu of Subsistence and Laundry Allowance, at P4,000 shall be deemed continuously granted starting February 2012 after its restoration by virtue of the PhilHealth Board of Directors Resolution dated December 2019.

Such an interpretation is consistent with the constitutionally-protected right of labor and that any doubt in the interpretation of laws, including compensation laws, should be resolved in favor of labor. Section 3 of Article XIII of the 1987 Constitution on Social Justice and Human Rights reads: "The State shall afford full protection to labor, local and overseas, organized and unorganized, and promote full employment and equality of employment opportunities for all." Section 18, Article II of the 1987 Constitution states that "[t]he State affirms labor as a primary social economic force. It shall protect the rights of workers and promote their welfare" and Section 10 declares that "[t]he State shall promote social justice in all phases of national development."

As at December 31, 2022, the remaining balance for WESA amounting to P95.765 million are for retired and resigned employees.

- 17.11 Operating Lease liability amounting to P4.574 million and P4.257 million as at December 31, 2023 and 2022, respectively, are for Rental expense with no contract but extended on a month to month basis and contract with 1 year or less term.

- 17.12 Other Financial Liabilities – this account is used to recognize the amount of other financial liabilities incurred which cannot be appropriately classified under any of the specific financial liability accounts.

18. INTER-AGENCY PAYABLE

This account consists the following:

	2023	2022 (As Restated)
Due to BIR	203,657,423	160,737,152
Due to GSIS	98,020,839	88,447,900
Due to Pag-IBIG	4,560,725	3,701,875
Due to PhilHealth	14,884,305	13,194,106
Due to NGAs	10,339	10,540,301
Due to Government Corporations	427,443,702	427,593,702
Due to LGUs	829,628	1,063,400
Due to Treasurer of the Phil	12,673,524	10,809,620
Total Allowance for Impairment	762,080,485	716,088,056

- 18.1 Due to BIR consists of liability for taxes withheld from employees' compensation and income payment to suppliers of goods and services which will be remitted in the subsequent period.

- 18.2 Due to GSIS, Due to Pag-IBIG and Due to PhilHealth comprise of amortization of loans availments, life and retirement insurance premiums payable to GSIS, contributions and amortization of loan availments payable to Pag-IBIG fund, and medical insurance premiums payable to PhilHealth deducted from the salaries of the Corporation's officials and employees and the employer's counterpart which will be remitted to the appropriate agencies in the subsequent periods.

- 18.3 Due to NGAs refers to Sigurado at Garantisadong Insurance Pangkalusugan and Philippine Health Information Exchange projects of the DOH.

- 18.4 18.4. Due to Government Corporations refer to accrual of UMID billing from GSIS and SSS as well as loan payment to National Home Mortgage Finance Corporation

- 18.5 Due to LGUs refers to overpayment of premium contributions under sponsored programs which shall be applied to their additional enrollment during the year, if any. It also includes local tax withheld located outside the region for remittance to LGU.

- 18.6 Due to Treasurer of the Philippines are payables for auditing services for the months of November and December.

19. TRUST LIABILITIES

Trust Liabilities refer to funds from other sources which are held in trust for a specific purpose. A separate bank account is opened if specified or stated in the Agreement.

This account consists the following:

	2023	2022 (As Restated)
UNFPA Project	38,057	37,786
Unclaimed Refund from HCPs	312,345,456	312,162,816
AHP - Protest Bond	76,125,320	41,985,000
Donations	8,623,879	8,710,840
Bail bond payable/guaranty/ security deposit pay	62,978,128	32,773,702
Retention fee	5,295,341	36,736,180

Global Development Project	1,958,886	1,958,886
Philippine Training Institute	6,540,391	5,222,074
Philippine Training Institute- NSSF	622,675	619,138
PhilHealth Run 2013	900	900
PhilHealth Run 2015	87,219	87,219
Calamity fund	80,752	127,437
PhilHealth Provident Fund	25,271,964	36,622,000
COVID-19 National Vaccine Indemnity Fund	521,995,906	498,833,762
LBP Electronic Salary Loan	667,464	-
Disallowance/Charges	389,767	194,500
Others	33,359,341	33,235,336
Total	1,056,381,446	1,009,307,576

19.1 Donations include funds received from the following entities, including earned interest

	2023	2022
Westmont Investment Corporation	2,945,656	2,945,656
Strategies and Alliance Corporation	3,476,458	3,563,419
Land Bank of the Philippines	110,000	110,000
Donation received by PROs	2,091,765	2,091,765
Total	8,623,879	8,710,840

20. PROVISION FOR HEALTH BENEFITS

20.1 Provision for Health Benefits – IBNR amounted to P54.994 billion and P95.907 billion as at December 31, 2023 and December 31, 2022, respectively. They are claims which are actuarially estimated to be in the possession of the HCIs as of the end of the month and have yet to be submitted to the Corporation within the allowable 60-day period after the date of discharge per CO No. 2015-0017 dated December 8, 2015.

Provision for IBNR, December 31, 2022	95,098,573,409
Add: Adjustment due to understatement	808,499,918
IBNR, adjusted balance	95,907,073,327
Less: Claims paid applied to balance	(17,290,843,633)
IBNR, balance for 2023	78,616,229,694
Less: Change as at December 31, 2023	(23,621,594,351)
Provision for IBNR, December 31, 2023	54,994,635,343

Provision for IBNR, December 31, 2021	55,453,415,291
Add: Adjustment on Restatement	1,447,209,967
IBNR, adjusted balance	56,900,625,258
Less: Claims paid applied to balance	(17,731,629,071)
IBNR, balance for 2021	39,168,996,187
Add: Change as at December 31, 2022	55,929,577,222
Provision for IBNR, December 31, 2022	95,098,573,409
Add: Adjustment due to understatement	808,499,918
Provision for IBNR, December 31, 2022	95,907,073,327

The method applied in estimating IBNP claims is called claims development (or lag) method. It is an estimation technique under which historical claim data, such as the number and amount of claims are grouped into the lag time periods in which claims were incurred and the time periods in which they were paid. The development method uses these groupings to create a claims processing or lag development pattern, which is used to determine

completion factors to help estimate the unpaid portion of incurred claims and come up with the ultimate value of claims. The paid claims are then subtracted from the ultimate value of claims by admission month to estimate the IBNP reserves.

20.2 Provision for Health Benefits - Denied Claims

The Provision for Health Benefits – Denied Claims amounted to 2,513,182,884 as at December 31, 2023. They are claims determined to be invalid and unworthy of payment due to an absolute deficiency that cannot be remedied through return to Health Facility or due to a finding of an unmet requirement. However, Health Facilities have filed a Motion for Reconsideration hence, it is probable that payment shall be made. The amount is actuarially computed.

21. OTHER PAYABLES

This account consists the following:

	2023	2022 (As Restated)
Undistributed collections	2,424,370,183	1,005,835,986
Due to Non-government organization/civil society	433,329	433,329
Other payables – others	33,344,822	28,881,887
Total	2,458,148,334	1,035,151,202

21.1 Undistributed Collections refer to unidentified premium collections, remittances of ACAs and accreditation fees directly credited to PhilHealth bank accounts as of reporting period.

21.2 Due to Non-Government Organization represents various reimbursement of PHIC Board of directors for meals and accommodation.

21.3 Other Payables – Others include employees' association dues for PhilHealth White, amortization of loan availments from PHICEA, and PROs Cooperative deduction from the salary of the PhilHealth employees.

22. DEFERRED CREDITS/UNEARNED INCOME

This account consists the following:

	2023	2022
Advance premium from direct contribution members	279,714,321	412,694,954
Premium contribution for NHTS enrollment (CYs 2013 and 2014)	516,844,200	516,844,200
Accreditation fees – ACAs	420,801	852,469
Accreditation fees – HCPs	2,793,000	2,873,500
Subsidy from LGUs	18,150,027	18,943,227
Others	306,300	366,300
Total	818,228,649	952,574,650

Other deferred credits/unearned income account refers to the payment of premium contribution, accreditation fees received in advance by the Corporation or income received not pertaining to the current year.

23. LEASE PAYABLE

Lease Payable amounting to P451.812 million and P535.743 million as at December 31, 2023 and December 31, 2022, respectively, are lease payable of the Corporation mostly office and warehouse rental ranging from one to five years.

24. LEAVE BENEFITS PAYABLE

This provision is measured at the best estimate of the amount needed to settle them at the end of the reporting period. The obligation is measured at its "actual expected value". Charges to this account are disbursements for terminal leave pay and monetization.

Balance, 12/31/2022	807,441,849
Less: Terminal Leave Pay	(18,304,097)
Add : Increase in Provision	24,726,491
Total	813,864,243
Monetization	604,091
Leave Benefits Payable, 12/31/2023	814,468,334

2023			
Region	Regular	Casual	Total
Head Office	176,982,487	15,577,760	192,560,247
NCR	52,448,664	26,988,901	79,437,565
CAR	32,658,554	5,929,276	38,587,830
I	29,988,559	11,153,305	41,141,864
II	28,966,812	6,126,667	35,093,479
III	35,136,109	20,134,585	55,270,694
IV-A	48,468,068	22,512,960	70,981,028
IV-B	17,595,448	11,814,399	29,409,847
V	23,073,433	10,945,966	34,019,399
VI	19,917,016	12,426,651	32,343,667
VII	24,394,134	9,306,641	33,700,775
VIII	23,847,723	6,320,960	30,168,683
IX	22,106,855	5,888,962	27,995,817
X	14,287,022	6,413,386	20,700,408
XI	14,447,154	6,841,220	21,288,374
XII	14,297,456	8,050,748	22,348,204
BARMM	10,969,078	7,013,715	17,982,793
CARAGA	23,748,674	7,084,895	30,833,569
Total	613,333,246	200,530,997	813,864,243

2022			
Region	Regular	Casual	Total
Head Office	173,762,626	16,207,735	189,970,361
NCR	29,403,524	5,548,446	34,951,970
CAR	28,019,916	10,824,679	38,844,595
I	26,386,532	6,052,306	32,438,838
II	31,569,213	19,306,507	50,875,720
III	52,046,743	26,414,311	78,461,054
IV-A	42,970,145	21,747,350	64,717,495
IV-B	17,759,287	12,029,271	29,788,558
V	23,759,902	10,323,360	34,083,262
VI	26,342,834	17,387,037	43,729,871
VII	24,185,684	10,690,757	34,876,441
VIII	21,893,581	5,758,197	27,651,778
IX	18,262,368	4,979,708	23,242,076
X	13,932,801	5,792,399	19,725,200
XI	14,312,649	7,378,536	21,691,185
XII	15,646,703	7,914,434	23,561,137
BARMM	21,122,259	6,073,422	27,195,681
CARAGA	10,327,750	21,308,877	31,636,627
Total	591,704,517	215,737,332	807,441,849

The provision of P813.864 million and P807.442 million represents money value of the earned leave credits as of December 31, 2023 and December 31, 2022 respectively of officers and employees of PhilHealth.

25. PROVISION FOR INSURANCE CONTRACT LIABILITIES

25.1 A liability for contractual benefits that are expected to be incurred in the future is recorded when the premiums are recognized. The liability is determined using assumptions considered appropriate for the policies in force. The actuarial valuation is based on PFRS 4 and projected / estimated by the Actuary which was subjected to deliberation and approval of the PhilHealth Board.

25.2 The following are the Actuarial assumptions used in the computation of ICL for the year 2022 and 2023:

Requisite Information	Assumptions Used	
	CY 2022	CY 2023
Valuation	Closed group valuation	Closed group valuation
Mortality rates	In accordance with the Philippine Inter Company Mortality Study	Graduated rates & age-last-birthday per 2017 PICM
Morbidity rates	Average of the past 3-years on Utilization Rates and Average Value per Claim disaggregated by age and sex	On-Level (based 2019-2023)
Discount rates	Bloomberg Valuation Rate, which is increasing by 0.5% (or 50 basis points) every 5 years after the 25th year up to a max of 8% *BVAL Rates were used as recommended by the PhilHealth Board but the Actuary's recommendation was to use the IC Spot Rates	IC 2023 Spot Rates
Inflation rate/Increase in Benefit Payout	6%	3.2% - Historical Trend (Medical Trend & PCB Trend)
Salary increase rate	3%	4.4%
On premium increase	Premium as stated in RA 11223	Premium as stated in RA 11223
On benefits	KONSULTA gradually transitioning to Comprehensive OPB	KONSULTA gradually transitioning to Comprehensive OPB
Average PCB Cost	Php 1,500 per member	Php 587 per member
Operating expense	7.5%	5.0% (Expense Loading)
Age at Retirement	60	60
Collection Efficiency	No increase	Gradual increase to 100% by 2026 for private sector
Frequency	Historical Indication	Historical Indication
Severity	Historical Indication	Historical Indication

Assumption	Particulars	
	CY 2022	CY 2023
Include GAA	Includes the government subsidy of 79.06B in the Inflows	Includes the government subsidy of 61.5B in the inflows
Lifespan	According to the latest WHO data published in 2019 life expectancy in the Philippines is: Male 67.3, female 75.5 and total life expectancy is 71.2. For the Direct Contributors, the liabilities beyond the average lifespan will not be considered when using this assumption. *WHO Life Expectancy in the Phil. was used as recommended by the PhilHealth Board but the Actuary's recommendation was to use the PICM	2017 Phil. Intercompany Mortality Study by Life Insurance Committee of the Actuarial Society of the Philippines.
Margin for Adverse Deviation (MFAD)	None	10% to reflect the degree of uncertainty of the best estimate assumption of Net ICL.

25.3 The above assumptions are for the interim until the complete personnel complement of the Office of the Actuary is in place as well as the setting up of the system that can address the needed information to be able to comply the requirements of PFRS 4.

25.4 Further, table below itemize the effect of each assumption on the Insurance Contract Liabilities (ICL):

The change in the estimated Insurance Contract Liabilities (ICL) compared to last year's estimate are due to the following:

Increase in outflows / future benefit payments

1. Increase in morbidity rates
2. Inclusion of future benefit payments to members past average lifespan
3. Reduction in discount rates
4. Increased number of claimants
5. Treatment of projection for dependents

Decrease in inflows / future contributions

1. Reduction in Government subsidy
2. Reduction in estimated annual contributions (affected by the number of contributing members, average contribution per member, etc.)
3. Decrease in expense loading
4. New assumption for primary care benefit
5. Reduction in medical trend

25.5 Following the assumptions aforementioned, the ICL as approved by the BOD to be recorded for CY 2023 and 2022 are as follows:

Actuarial Estimates	2023	2022
Discounted Outflows	5,564,470,507,002	5,037,679,508,012
Discounted Inflows	4,519,328,789,724	4,770,806,195,481
Net ICL	1,045,142,217,278	266,873,312,531
MfAD	104,514,221,728	-
Net ICL including MfAD	1,149,656,439,006	266,873,312,531

Actuarial Estimates	2023	2022
Discounted Outflows	5,564,470,507,002	5,037,679,508,012
Discounted Inflows	4,519,328,789,724	4,770,806,195,481
Net ICL	1,045,142,217,278	266,873,312,531
MfAD	104,514,221,728	-
ICL, end of the year	1,149,656,439,006	266,873,312,531
ICL, beginning of the year	266,873,312,531	339,309,737,739
Change in ICL	882,783,126,475	(72,436,425,208)

25.6 The Actuarial Services and Risk Management Sector performed Liability Adequacy Test as required by PFRS 4, using the same assumptions as stated above and in Note 2.22 as required by IC, the result of which is P1.1450 trillion. The increase by P882.783 billion is recognized as Other Losses to be consistent on the recommended by COA GAS to take up the decrease in 2022 as Other Gains in compliance to PAS 8, change in accounting estimates.

26. MEMBERS' EQUITY

Members' Equity consists the following:

	2023	2022 (As Restated)
Reserve fund	464,286,992,149	275,785,094,946
Provision for ICL	(1,127,993,822,270)	(245,210,695,795)
Total Members' Equity	(663,706,830,121)	30,574,399,151

26.1 Reserve Fund

	2023	2022
Reserve at January 1	275,785,094,946	191,498,004,527
Net Income	174,064,380,907	79,038,542,242
Prior year's adjustments	14,437,516,296	5,248,548,177
Reserve Fund	464,286,992,149	275,785,094,946

The Reserve Fund is recorded in compliance with Office Order No. 0145, s. of 2012 which is based on the provisions of Section 27 of RA No. 7875 as amended by RA No. 10606, which states that the Corporation shall set aside a portion of its accumulated revenues not needed to meet the cost of the current year's expenditures as reserved funds: Provided, that the total amount of reserves shall not exceed a ceiling of P560.55 billion, the amount actuarially estimated for two years' projected program expenditures: Provided further; that whenever actual reserves exceed the required ceiling at the end of the Corporation's Fiscal Year, the excess of the Corporation's Reserve Fund shall be used to increase the program's benefits, decrease the member's contributions and augment the health facilities enhancement program of the DOH.

The remaining portion of the Reserve Fund that is not needed to meet the current expenditure obligations or used for the abovementioned programs shall be placed in investments to earn an average annual income at prevailing rates of interest and shall be known as the "Investment Reserve Fund".

26.1 The difference from the ICL amount in the liability account amounting to P21,662,616,736 pertains to Non-Paying Members reserves previously set up before providing reserves for all members.

26.2 The resultant negative figure for the Members' Equity amounting to 664,315,295,196 is the net effect of change in accounting estimate in performing the adequacy test as required by PFRS 4. It is computed as the expected net of present value of expenses (claims + expenses) and present value of collection plus margin for adverse deviation. The Increase in the present value of outflow and decrease in the present value of inflow contributed in the increase of ICL thereby reducing the Members' Equity.

To counter the negative effect, there should be an increase in the projected inflow vs the projected outflow, among others.

27. PREMIUM CONTRIBUTIONS

Details of Premium Contributions are as follows:

	2023	2022 (As Restated)
Direct Contributors	157,353,034,317	139,474,920,706
Indirect Contributors	78,805,615,998	80,064,452,340
Total	236,158,650,315	219,539,373,046

27.1 Below is the breakdown of the Direct Contributors:

	2023	2022 (As Restated)
Government	42,302,905,260	35,528,325,450.00
Private	105,907,437,477	95,206,050,767.00
Kasambahay	82,417,469	101,203,951.00
Family Driver	47,517	46,083.00
Migrant Worker - Land Based	246,390,099	267,637,186.00
Migrant Worker - Sea Based	346,071	628,763.00
Filipino with Dual Citizenship / Living Abroad	714,961	719,566.00
Women about to give birth	47,550	128,200.00
Professional Practitioner	52,268,514	50,613,694.00
Self-Earning Individual	7,498,713,197	6,975,275,789.00

Self-Earning Individual - Sole Proprietor	5,270,827	4,150,488.00
Self-Earning Individual - Group Enrollment Scheme	1,232,547,552	1,315,491,475.00
Foreign National	22,816,098	23,782,984.00
Others	1,111,725	866,310.00
Direct Contributors	157,353,034,317	139,474,920,706

27.2 Below is the breakdown of the Indirect Contributors:

	2023	2022 (As Restated)
Indigents – NHTS	35,256,167,000	38,237,655,000
Senior Citizens	42,931,355,000	41,318,550,000
Special Government Programs – PAMANA	–	13,600,800
Sponsored	618,093,998	494,646,540
Indirect Contributors	78,805,615,998	80,064,452,340

27.3 The amounts collected come from the following members in accordance with Title III Section 5 of the Implementing Rules and Regulations of RA No. 7875 as amended otherwise known as the National Health Insurance Act of 2013 to wit:

- a. Income from the Direct Contributors come from the premium contributions of the following:
 1. Government employees;
 2. Private employees;
 3. All other workers rendering services, whether in government or private offices, such as job order contractors, project-based contractors and the likes;
 4. Owners of micro-enterprises;
 5. Owners of small, medium and large enterprises;
 6. Household Help – as defined in RA No. 10361 on "Kasambahay Law";
 7. Family Drivers;
 8. Migrant Workers – as defined in RA No. 10022 (Migrant Workers Act) and RA No. 10801 or Overseas Workers and Welfare Administration (OWWA) Act;
 9. Informal Sector;
 10. Self-earning individuals;
 11. Professional practitioners;
 12. Filipinos with Dual Citizenship;
 13. Naturalized Filipino Citizens;
 14. Citizens of other countries working and/or residing in the Philippines;
 15. Women about to give birth;
 16. Foreign Retirees (Registered with Philippine Retirement Authority); and
 17. All Filipinos aged 21 years and above who have the capacity to pay premiums.
- b. Income from the Indirect Contributors come from the premium contributions of the following:
 1. Indigent-National Household Targeting System (NHTS), Senior Citizen, Persons with Disability (PWD) and Point of Service (POS) as appropriated in the General Appropriations Act (GAA);
 2. Sponsored members paid by another individual, government agency, or private entity according to the rules prescribed by the Corporation; and
 3. Special government programs as appropriated in the GAA.

28. INTEREST AND OTHER INCOME

Interest and other income account is as follows:

	2023	2022 (As Restated)
Interest Income	20,700,881,752	11,544,397,570
Other Income	815,345,117	698,494,582
Total	21,516,226,869	12,242,892,152

28.1 Below is the breakdown of the Interest Income:

	2023	2022 (As Restated)
Investment securities at amortized cost	12,163,006,708	9,920,279,268
SSDs and Investment in Time Deposit	1,029,695,126	318,231,985
Investment in Time Deposit	7,506,556,770	1,304,226,368
Savings and current deposits	1,623,148	1,659,949
Interest Income	20,700,881,752	11,544,397,570

28.2 Below is the breakdown of the Other Income:

	2023	2022 (As Restated)
Accreditation fees – HCPs	32,124,718	28,758,144
Fines and penalties	700,636,775	516,628,467
Rent income	714,698	638,791
Gain on foreign exchange	158,488	259,837
Gain on sale of unserviceable property	322,548	229,676
Gain on sale of property, plant and equipment	14,100	–
Other gain	6,500	23,319
Reversal of Impairment loss	48,676,694	111,670,121
Miscellaneous income	32,690,596	40,286,227
Other Income	815,345,117	698,494,582

Reversal of Impairment Loss is credited when the allowance is being adjusted due to excessive provision or reversal of the allowance when collection was made.

Miscellaneous Income consists of income ranging from P100,000 and above. This includes income from proceeds from disposal of unserviceable properties, sale of valueless records, forfeited performance security and refund of benefit payment collected from delinquent employers.

Rent Income is an income earned from the portion of PRO III office building being leased out to a private entity.

28.3 Other Gains is credited to recognize gains which are not classified under any specific gain accounts. The decrease in the estimated Provision of ICL is credited to this account while an increase is charged to the benefit expense account.

	2023	2022
ICL, end of the year	–	266,873,312,531
ICL, beginning of the year	–	339,309,737,739
Total	–	(72,436,425,208)

Under PAS 8 on Accounting Policies, Changes in Accounting Estimates and Errors, a change in accounting estimate is an adjustment of the carrying amount of an asset or a liability, or the amount of the periodic consumption of an asset, that results from the assessment of the present

status of, and expected future benefits and obligations associated with, assets and liabilities. Changes in accounting estimates result from new information or new developments and, accordingly, are not correction of errors. Paragraph 36 states that:

The effect of a change in an accounting estimate, other than a change to which paragraph 37 applies, shall be recognized prospectively by including it in profit or loss in:

- (a) the period of the change, if the change affects that period only; or
- (b) the period of the change and future periods, if the change affects both.

Further, Paragraph 37 states that:

To the extent that a change in an accounting estimate gives rise to changes in assets and liabilities, or relates to an item of equity, it shall be recognized by adjusting the carrying amount of the related asset, liability or equity item in the period of the change.

29. BENEFIT CLAIMS EXPENSES

Benefit Claims Expenses for 2023 and 2022 are recognized at the time of admission. This account is composed of the following:

	2023	2022
Members' Benefits for Direct Contributors	34,473,633,624	86,970,165,112
Members' Benefits for Indirect Contributors	39,303,979,133	56,092,416,694
Interim financing mechanism	–	269,004,561
PhilHealth konsultasyon sulit at tama (KONSULTA) package	429,575,508	228,816,027
Total	74,207,188,265	143,560,402,394

The decrease in benefit expense is due to the adjustment made for Incurred But Not yet Paid (IBNP) as computed by Actuary. The adjustment is a change in accounting estimate and per PAS 8, change in accounting estimate shall be taken up on the period of change. Benefit expense before the adjustment is at 116.6B. After effecting the adjustment of 40.84B, it is now at 75.77B.

29.1 Below is the breakdown of the Members' Benefits for Direct Contributors:

	2023	2022
Government	3,804,023,192	11,240,792,473
Private	8,520,948,761	28,281,918,690
Kasambahay	96,929,967	29,050,204
Family Driver	47,965,885	339,285
Migrant Worker - Land Based	493,975,614	2,096,764,677
Migrant Worker - Sea Based	2,003,835,137	97,704,869
Filipino with Dual Citizenship / Living Abroad	9,182,592	20,676,523
Professional Practitioner	429,309,094	2,522,476
Self-Earning Individual	7,843,820,839	33,250,132,225
Self-Earning Individual - Sole Proprietor	33,707,908	3,382,479
Self-Earning Individual - Group Enrollment	1,880,720,143	522,316,293
Foreign National	248,912,462	11,636,237
Lifetime Members	7,500,485,892	11,412,646,513
Others	1,559,816,138	282,168
Direct Contributors	34,473,633,624	86,970,165,112

29.2 Below is the breakdown of the Members' Benefits for Indirect

Contributors:

	2023	2022
Senior citizens	4,800,801,526	28,695,167,679
Indigent - NHTS	5,639,985,224	15,879,870,523
Sponsored	29,575,647,889	11,517,378,492
Indirect Contributors	40,016,434,639	56,092,416,694

30. PERSONNEL SERVICES

Personnel services account includes:

	2023	2022
Salaries and wages	2,670,502,725	2,382,652,094
Other compensation	2,055,352,059	2,043,349,862
Statutory contributions	576,032,091	540,139,105
Other personnel services	46,679,099	6,441,831
Total	5,348,565,974	4,972,582,892

31. OTHER OPERATING EXPENSES

This account is composed of the following:

	2023	2022 (As Restated)
MOOE	2,642,826,368	1,938,498,856
Financial Expenses	48,668,167	96,958,546
Non-cash expenses	1,363,247,503	2,175,280,268
Total	4,054,742,038	4,210,737,670

This account represents the administrative costs which must be within the limits prescribed in Section 72, Financial Management of the Revised Implementing Rules and Regulations of RA No. 7875, as amended by RA No. 9241 and RA No. 10606, known as the "National Health Insurance Act of 2013."

31.1 Below is the breakdown of the MOOE:

	2023	2022
Traveling Expenses	162,175,554	79,178,933
Training and Scholarship Expenses	39,601,308	30,352,268
Supplies and Materials Expenses	181,243,116	168,139,330
Semi-Expendable Expenses	73,074,412	55,612,980
Water Expenses	10,242,182	7,210,995
Electricity Expenses	180,138,901	182,746,728
Communication Expenses	202,510,701	165,938,346
Awards / Rewards, Prizes and Indemnities	7,392,355	9,522,334
Research, Exploration and Development Expenses	34,082,608	15,962,748
Confidential, Intelligence and Extraordinary Expenses	5,769,098	5,827,007
Auditing Services	82,765,175	64,857,701
Consultancy Services	2,616,840	545,638
Other Professional Services	580,012,226	269,224,985
Janitorial Services	91,744,080	84,774,362
Security Services	189,313,995	174,932,949
Repairs & Maintenance	30,942,286	24,052,827
Taxes, Duties and Licenses	5,894,034	415,558
Fidelity Bond and Insurance Expenses	25,092,528	23,846,399
Advertising Expenses	33,695,710	22,225,401

Marketing and Promotional Expenses	90,081,687	37,324,541
Printing and Publication Expenses	9,728,110	9,949,857
Representation Expenses	61,224,248	61,685,774
Transportation and Delivery Expenses	2,768,029	2,228,221
Rent / Lease Expenses	325,222,188	282,550,111
Membership Dues and Contri. to Organizations	9,846,471	10,150,844
Subscription Expenses	11,269,684	31,989,759
Donations	400,163	1,654,337
Major Events Expenses	29,749,394	20,175,880
Other MOOE – Others	164,229,285	95,422,043
Total	2,642,826,368	1,938,498,856

31.2 Below is the breakdown of the Financial Expenses:

	2023	2022
Interest Expenses	19,037,558	22,873,017
Bank Charges	86,851	23,830
Other financial charges	29,543,758	74,061,699
Total	48,668,167	96,958,546

Interest Expenses include the effect of applying PFRS 16.

Other financial charges pertain to transaction fees/service fees of ACAs and Accredited Collecting Banks, rental fees of National Registry of Scripless Securities facility and other financial charges.

31.3 Below is the breakdown of the Non-Cash Expenses:

	2023	2022
	(As Restated)	
Depreciation	495,792,936	528,861,606
Depreciation - buildings & other structures	1,417,764	1,290,437
Depreciation - machinery and equipment	158,862,502	209,046,869
Depreciation - furniture and fixtures and books	4,350,217	4,492,150
Depreciation - transportation equipment	36,501,497	14,493,515
Depreciation - leased assets improvements	22,202,461	26,922,126
Depreciation - Right-of-Use-Assets	272,458,495	272,589,986
Depreciation - other property, plant & equipment	-	26,523
Amortization - Intangible assets	5,468,226	10,206,770
Impairment Loss	861,069,539	1,633,657,912
Impairment loss - loans and receivables	833,389,410	1,539,716,014
Impairment loss - other receivables	19,965,748	10,520,375
Impairment loss - financial assets held to maturity	980,984	328,963
Impairment loss - Inventories	274,575	1,177,736
Impairment loss - property, plant & equipment	171,654	962
Impairment loss - intangible assets	6,287,168	707,370
Impairment loss - other assets	-	81,206,492

Losses	916,802	2,553,980
Loss on sale of property, plant, & equipment	-	477,140
Loss on FOREX	175,952	90,807
Loss on assets	608,932	268,607
Loss on sale assets	131,918	-
Loss on Sale of Unserviceable Property	-	1,717,426
Total	1,426,592,076	2,175,280,268

31.4 Other Losses is used to recognize the amount of losses not falling under any of the specific loss account. The account was debited for the increase in the estimated provision ICL for CY 2023 while other gains account will be credited for any decrease in the ICL.

	2023	2022
ICL, end of the year	1,149,656,439,006	-
ICL, beginning of the year	266,873,312,531	-
Increase/Change in ICL	882,783,126,475	-

Under PAS 8 on Accounting Policies, Changes in Accounting Estimates and Errors, a change in accounting estimate is an adjustment of the carrying amount of an asset or a liability, or the amount of the periodic consumption of an asset, that results from the assessment of the present status of, and expected future benefits and obligations associated with, assets and liabilities. Changes in accounting estimates result from new information or new developments and, accordingly, are not correction of errors. Paragraph 36 states that:

The effect of a change in an accounting estimate, other than a change to which paragraph 37 applies, shall be recognized prospectively by including it in profit or loss in:

- the period of the change, if the change affects that period only; or
- the period of the change and future periods, if the change affects both.

Further, Paragraph 37 states that:

To the extent that a change in an accounting estimate gives rise to changes in assets and liabilities, or relates to an item of equity, it shall be recognized by adjusting the carrying amount of the related asset, liability or equity item in the period of the change.

32. RELATED PARTY DISCLOSURES

Compensation of key management personnel.

The key management personnel refer to the executive team, with the rank of Senior Vice President up to PCEO. These individuals have the authority and responsibility for planning, directing, and controlling the activities of the Corporation.

The aggregate compensation of the executive officers for CYs 2023 and 2022 is as follows:

	2023	2022
Salaries and wages	64,900,031	71,450,250
Terminal benefits	-	794,063
Total	64,900,031	72,244,313

The BODs for CYs 2023 and 2022 received the following:

	2023	2022
Honorarium/per diem	7,184,000	8,523,200
Total	7,184,000	8,523,200

33. RESTATEMENTS AND CHANGES IN PRESENTATION FORMAT OF CERTAIN ACCOUNTS IN THE FINANCIAL STATEMENT

Below are restatements and changes in presentation of certain accounts in the financial statements For CY 2022 as recommended by the Commission on Audit to adhere in the Revised Chart of Accounts (RCA):

	December 31, 2022		
	Audited	Restatement/Change in Presentation Format	As Restated/New Presentation Format
Assets			
Current Assets			
Cash & cash equivalents	6,423,703,001	–	6,423,703,001
Investment in time deposits	126,426,398,154	–	126,426,398,154
Receivables, net	34,577,099,066	3,386,973,507	37,964,072,573
Other receivable, IRM	8,230,950	–	8,230,950
Inventories	122,846,004	–	122,846,004
Other current assets	28,369,588	–	28,369,588
Total Current Assets	167,586,646,763	3,386,973,507	170,973,620,270
Non-Current Assets			
Investment securities at amortized cost	281,181,882,805	–	281,181,882,805
Investment Properties	–	855,663,291	855,663,291
Property and equipment, net	1,680,050,270	(855,663,291)	824,386,979
Right-of-use of assets	505,427,949	–	505,427,949
Intangible assets, net	264,741,753	–	264,741,753
Other non-current assets, net	216,173,173	(22,902,889)	193,270,284
Total Non-Current Assets	283,848,275,950	(22,902,889)	283,825,373,061
TOTAL ASSETS	451,434,922,713	3,364,070,618	454,798,993,331
LIABILITIES AND EQUITY			
Current Liabilities			
Financial liabilities	57,634,055,213	(1,246,153,471)	56,387,901,742
Inter-agency payables	278,434,503	437,653,553	716,088,056
Trust liabilities	1,009,113,076	194,500	1,009,307,576
Provision for Health Benefits	95,098,573,409	808,499,918	95,907,073,327
Other payables	2,243,373,710	(1,208,222,508)	1,035,151,202
Total Current Assets	156,263,549,911	(1,208,028,008)	155,055,521,903
Non-Current Assets			
Unearned income	911,858,175	40,716,475	952,574,650
Lease payable	535,743,247	–	535,743,247
Leave benefits payable	807,441,849	–	807,441,849
Provision for ICL	266,873,312,531	–	266,873,312,531
Total Non-Current Liabilities	269,128,355,802	40,716,475	269,169,072,277
TOTAL LIABILITIES	425,391,905,713	(1,167,311,533)	424,224,594,180
EQUITY			
Members' Equity	26,043,017,000	4,531,382,151	30,574,399,151
Total Equity	26,043,017,000	4,531,382,151	30,574,399,151
TOTAL LIABILITIES AND EQUITY	451,434,922,713	3,364,070,618	454,798,993,331

December 31, 2022			
	Audited	Restatement	As Restated
Premium contributions	216,779,856,065	2,759,516,981	219,539,373,046
Less: Benefit claims expenses	143,560,402,394	–	143,560,402,394
GROSS MARGIN FROM OPERATIONS	73,219,453,671	2,759,516,981	75,978,970,652
Operating expenses			
Personnel services	4,972,582,892	–	4,972,582,892
Other operating expenses	4,253,755,675	(43,018,005)	4,210,737,670
TOTAL OPERATING EXPENSES	9,226,338,567	(43,018,005)	9,183,320,562
NET OPERATING INCOME (LOSS)	63,993,115,104	2,802,534,986	66,795,650,090
Add: Interest and other income	11,853,163,646	389,728,506	12,242,892,152
NET INCOME BEFORE CHANGE IN ICL	75,846,278,750	3,192,263,492	79,038,542,242
Other Gains	72,436,425,208	–	72,436,425,208
Other Comprehensive Income	–	–	–
NET INCOME	148,282,703,958	3,192,263,492	151,474,967,450
RESERVE FUND			
Reserve at January 1	191,498,004,527	–	191,498,004,527
Net Income	75,846,278,750	3,192,263,492	79,038,542,242
Prior year adjustment	3,909,429,518	1,339,118,659	5,248,548,177
Reserve Fund at December 31	271,253,712,795	4,531,382,151	275,785,094,946
Provision for insurance contract liabilities (ICL)	(245,210,695,795)	–	(245,210,695,795)
TOTAL MEMBERS' EQUITY	26,043,017,000	4,531,382,151	30,574,399,151

34. OTHER SIGNIFICANT AND RELEVANT INFORMATION

34.1 By virtue of Special Provision 1(d), XLIII of the Republic Act No. 11975 or General Appropriations Act (GAA) for FY 2024 which directs the Department of Finance (DOF) to issue guidelines to implement the collection of Unprogrammed Appropriation sourced from the "[F]und balance of the GOCC from any remainder resulting from the review and reduction of their reserve funds to reasonable levels taking into account the disbursement from prior years, the DOF issued DC No. 003-2024 dated Feb. 27, 2024 mandating all GOCCs (including PhilHealth) to return its excess reserves.

As such, the following events and actions of the Management were made in order to comply with the aforesaid directive:

- PhilHealth sought for legal opinion from the Office of the Government Corporate Counsel (OGCC) and the Governance Commission for GOCCs (GCG) on the applicability of the Special Provision 1(d), XLIII of the GAA to PhilHealth in the light of the provisions on Reserve Fund under RA 7875 (NHIP) and the RA 11223 or the UHC Act. In addition, PhilHealth also sought guidance from the Commission of Audit (COA) on the accounting treatment of the supposed remittance.
- On April 16, 2024, PhilHealth received OGCC opinion No. 055, s. 2024 dated April 11, 2024 expressing that the remittance to the BTR of the Fund Balance would not violate the Universal Health Care Act (UHCA) as the amount remitted is understood not to constitute a portion of the Reserve Fund. They concluded that it would be proper and prudent for PhilHealth to abide by the DOF Circular which implements Section XLIII (1)(d) of the GAA.
- On April 25, 2024, PhilHealth received DOF letter dated April 24, 2024 directing PhilHealth to remit the 89.9 billion pesos the Bureau of Treasury (BTR) within 15 calendar days from receipt of their notice. The collection of fund was approved during the

Cabinet meeting on April 3, 2024. The table below shows how the 89.9 billion was computed by the DOF, to wit:

Government Subsidy vs Utilization						
	2023	2022	2021	2020	2019	2018
Premium for Indirect Contribution	78.8	80.1	80.2	63.4	69.4	59.7
Member Benefit for Indirect Contributors	0.0	56.1	53.1	64.1	70.5	58
Net Flow	38.8	24.0	27.1	-0.7	-1.1	1.7

89.9 billion

- On May 2, 2024 the Governance Commission for Government Owned or Controlled Corporation (GCG) issued its opinion, stating that they defer to the expertise of the DOF, being the agency mandated by law to implement Special Provision 1(d), XLIII of the 2024 GAA.
- Based on the DOF computation, it was determined that the 2-year requirement constituting the Reserve Fund is equivalent to P280.6 billion. After considering the new computation, a reduction of 183.1 billion of the reserve fund (463.7-280.6) and shall be transferred to the Surplus Account.
- Hence, on May 8, 2024, after ensuring that there is fund balance to comply with the DOF directive, the Management sought for the Board approval on the transfer of government subsidy amounting to 89.9 billion for premium contribution (CY 2021-2023) that are more than the benefit expenses of indirect contributors. This was then approved by the Board upon confirmation from Management that the said unutilized subsidies does not include PhilHealth premium contribution from direct contributors.

- The releases shall be synchronized with investment maturities to avoid pretermination fees. Hence, on May 10, 2024, the Execom approved the schedule of remittance of the 89.9 Billion to the Bureau of Treasury as follows:

1st tranche: May 10, 2024 – 20 billion
 2nd tranche: August 21, 2024 – 10 billion
 3rd tranche: October 16, 2024 – 30 billion
 4th tranche: March 31, 2025 – 29.9 billion

34.2 Republic Act 11975, also known as the GAA for FY 2024 was signed into law by President Ferdinand R. Marcos Jr. which takes effect on January 1, 2024. The National Government has appropriated a total of 61.514 billion to subsidize the programs of Philhealth. The breakdown of the appropriated subsidy is shown below.

Particulars	Amount
Subsidy for the National Health Insurance Program	61,453,404,000
Special Purpose Insurance Coverage	61,229,000
Total	61,514,633,000

34.3 As provided for in chapter IX section 37 of the R.A. 11223, fifty percent (50%) of the National Government share from the income of the Philippine Amusement Gaming Corporation (PAGCOR) as provided for in Presidential Decree No. 1869, as amended: Provided, That the funds raised for this purpose shall be transferred to PhilHealth at the end of each quarter subject to the usual budgeting, accounting and auditing rules and regulations: Provided, further, That the funds shall be used by PhilHealth to improve its benefit packages and Forty percent (40%) of the Charity Fund, net of Documentary Stamp Tax Payments, and mandatory contributions of the Philippine Charity Sweepstakes Office (PCSO) as provided for in Republic Act No. 1169, as amended: Provided, That the funds raised for this purpose shall be transferred to PhilHealth at the end of each quarter subject to the usual budgeting, accounting, and auditing rules and regulations: Provided, further, That the funds shall be used by PhilHealth to improve its benefit packages.

As of the reporting date, SARO-BMB-C-23-0026482 dated December 5, 2023 amounting to P21,170,000,000 for the improvement of benefit packages was released. However, No Notice of Cash Allocation (NCA) has yet to be issued.

34.4 Identified Strategies and Cybersecurity Action Plans to mitigate risk arising from the “Medusa” ransomware attack.

As the Corporation recognizes the critical importance of robust security measures to enhance its resilience against evolving threats (e.g. Medusa ransomware), proactive measures shall be undertaken to enhance our security posture. As such, we need to manage and reduce the risk to our cyber and physical infrastructure through the implementation of the following identified strategies and cybersecurity action plans:

1. Cyber Security Projects

Database Security Suite. This solution ensures the security of the production database through transparent data encryption. As background, the said project “safeguard[s] the Corporation’s data from unauthorized access, use, disclosure, disruption, modification, perusal, inspection, recording or destruction, even from privileged users, that is 100% compatible with existing systems and database. This means that the Corporation will be assured that problems due to error/s in the existing database encryption software will be corrected due to updated software including patches, upgrades and remote services.” The said project was initially processed last 2023, and finally implemented this April 2024.

Vulnerability Assessment and Penetration Testing (VAPT). The VAPT is one of the most reliable methods for performing “Risk Assessment” in accordance with globally accepted Security Standards and best practices, which requires the expertise of a third party to do an independent assessment of PhilHealth’s security posture by undertaking technical reviews such as Penetration Testing (PT) and

Vulnerability Assessment (VA). In a more plain language, VAPT is best described as “a proactive “hacking” activity in which [the organization] hack[s] [their] [own] infrastructure before hackers come looking for loopholes.” Hence, this project has the end goal of gaining better understanding of potential threats and vulnerabilities, identify existing inherent and potential risks and vulnerabilities, evaluate the existing security measures and validate the effectiveness and efficiency of existing defenses. The documents pertaining to this project were already endorsed to the SBAC for procurement.

Managed Security Operations Center (MSOC). A managed service, which aims to have a better visibility on PhilHealth’s cyber security posture by providing real-time monitoring. The same was already presented before the IT Governance Committee. Further, in a memorandum dated 15 March 2024 to the IPPSD-ITMD, this Office already requested the inclusion of the said project to the Information System Strategic Plan (ISSP) for CY 2024-2026 of the Corporation. In their response, the IPPSD-ITMD required the submission of a copy of the Secretary Certificate, the same was already coordinated with the Corporate Secretary.

These projects will redound to the cyber security posture of the Corporation.

The Cybersecurity Action Plan will be further developed and calibrated to the status and needs of PhilHealth in view of the engagement of the Cyber Security consultant.

2. Compliance to the Statutory Requirement i.e. Data Privacy Act and related issuance

For being the designated organic office to provide support to the Data Protection Officer, InfoSec immediately complied with the statutory requirement prescribed by the Data Privacy Act (DPA) and its related issuance, along with the directives of the National Privacy Commission. This includes submission of reports, notification to the Commission, and other similar activities.

3. Capacitating PhilHealth Personnel and Officials through the Security Education Training Awareness (SETA) Program

It is indisputable that the human component is the weakest link in the information security and cybersecurity chain. To remediate the same, it is incumbent for the Corporation to bolster its human firewall by capacitating its employees. In view thereof, InfoSec initiated a set of activities covered by the Corporate Personnel Order (CPO), authorizing its personnel to conduct activities that promote information security and data privacy and protection awareness in several regional offices and LHIOs.

4. Philhealth cyber and physical infrastructure is proactively protected and secured through the following:

- Maintaining an updated Inventory of Enterprise and Software Assets
- Strengthen Data Protection through developing processes and technical controls to identify, classify, securely handle, retain, and dispose data. Encrypt data on end-user devices containing sensitive data, on removable media, in transit, and at rest.
- Secure configuration of assets and software
- Improve Account Management by conducting periodic audits
- Improve Access Control Management
- Strengthen Audit Log Management through log collection and analysis to detect malicious activity quickly.
- Strengthen Malware Defenses
- Improve Network and Infrastructure Management
- Improve Network Monitoring and Defense
- Improve Service Provider Management
- Improve Application Software Security
- Adoption of National Security-and-Privacy-by-Design Framework
- Improve Physical and Environmental Security
- Risk Management

- Adoption and implementation of the Six-stage Cybersecurity Incident Response Model and
- Business Continuity and Disaster Recovery program -this will be further developed and calibrated to the status and needs of PhilHealth in view of the engagement of the Cybersecurity consultant to be implemented immediately.

34.5 PhilHealth as Commercial Public Sector Entity

PhilHealth is now a CPSE per COA Resolution No. 2020-013 dated January 31, 2020, re: Renaming Government Business Entities (GBEs) and Non-Government Business Entities (Non-GBEs) into Commercial Public Sector Entities (CPSEs) and Non-Commercial Public Sector Entities (Non-CPSEs) pursuant to the 2018 Edition of the Handbook of International Public Sector Accounting Pronouncements (HIPSAP) Published by the International Federation of Accountants (IFAC) and the International Public Sector Accounting Standard Board (IPSASB) and its adoption of the PFRSs as its financial reporting framework.

Formerly, PhilHealth is a GBE per COA Resolution No. 2018-001 dated January 20, 2018, re: Reclassification of the Philippine Health Insurance Corporation from a Non-Government Business Enterprise (Non-GBE) to a Government Business Enterprise (GBE) and its adoption of the PFRS as its financial reporting framework.

34.6 POS Program

The DBM has issued the following Special Allotment Release Order (SAROs) for the payment of cost of availment for CY2017 benefit claims of financially incapable families under UHC through POS Program, chargeable against PhilHealth's authorized appropriation under RA No. 10924, FY 2017.

The release of the corresponding cash allocation is subject to PhilHealth's submission of the names of CY2017 POS patients together with the actual amount of claims with the required premium contributions to DBM.

SARO No.	Amount of SARO	DBM Releases from CYs 2017 to 2023
SARO-BMB-C-17-0023166	91,333,530	91,333,530
SARO-BMB-C-17-0025119	103,350,096	103,350,096
SARO-BMB-C-17-0025794	2,805,316,374	1,196,668,091
Total	3,000,000,000	1,391,351,717

34.7 PhilHealth Supplemental Benefits

On December 28, 2018, the DBM issued SARO No. SARO-BMB-C-18-0035076 amounting to P8.597 billion which include the locally funded project "PhilHealth Supplemental Benefits" for all government employees of the Executive Branch amounting to P3.5 billion. No Notice of Cash Allotment received yet for the P3.5 billion as at September 30, 2023.

34.8 Arrears of the National Government as an Employer

The information below, though not recorded in PhilHealth's books of accounts, are deemed necessary to be disclosed

PhilHealth had adjusted premium contribution of the Employed Sector in CY 2013 through PhilHealth Circular No. 057, s. of 2012 which prescribes P875.00 per month as the maximum contribution shared equally by the Employer and the Employee at P437.50 each. However, DBM has only allocated P312.50 or a 40 per cent discrepancy. The Corporation had formally billed the DBM of the estimated National Government Employer Premium Differential and requested allocation of the unappropriated balances for the following periods:

Arrears	Amount
CYs 2001-2012	9,664,042,012
CY 2013	330,691,801
CY 2014	330,691,801
CY 2015	330,691,801
CY 2016	330,691,801
Total	10,986,809,216

34.9 Assigned lot to PRO III

A parcel of lot with a total area of 1,831 sq. m. located within the Government Center, Barangay Maimpis, City of San Fernando, Pampanga was assigned to PRO III per Deed of Assignment from Regional Government Center of San Fernando, Pampanga through a MOA executed by and between the RDC III and the PhilHealth. A warehouse was constructed in this lot with a carrying amount of P2.520 million in the financial statements.

34.10 Non-remittance of GSIS Premium for Disallowed Salary Adjustments

The GSIS premium amounting to P20.605 million was part and parcel of the salary adjustments given to the Corporation's employees, but subsequently disallowed by COA. The said amount is still outstanding and not yet remitted to GSIS per Opinion No. 56, Series of 2018 from the OGCC dated March 26, 2018 stating that PhilHealth should keep the amount originally intended to be remitted to GSIS as premiums corresponding to the adjusted salaries of its employees without prejudice to its remittance in the event the ND is lifted.

34.11 Contingent Liability

Under contingent liability is the employer's share for Philhealth Provident Fund (PPF) amounting to 1.208 billion for the years 2019, 2020 and 2021. The establishment of the PPF needs the approval of the President of the Philippines as observed through COA AOM hence, the timing and control of the approval is now beyond the control of the corporation. As of even date, the appeal is now with the Office of the President.

With other Government-Owned and Controlled Corporations and Government Financial Institutions that were able to secure approval for their Provident Fund, we are optimistic that the approval will be granted in due time.

34.12 Contingent Asset

Contingent assets are possible assets whose existence will be confirmed by the occurrence or non-occurrence of uncertain future events that are not wholly within the control of the entity. Contingent assets are not recognised, but they are disclosed when it is more likely than not that an inflow of benefits will occur.

Under contingent assets are legal cases filed by Philhealth against erring HCPs and employers which are more likely than not that an inflow of benefit will occur amounting to P55,311,21. It also include notice of disallowances issued by the Commission on Audit which are on appeal with the Supreme Court amounting to P7.143 billion.

34.13 Outstanding Legal Cases filed by PhilHealth

	No. of Cases	Amount
Complaints Against		
Employers	128	103,103,023.81
Health Care Professionals (HCPs)	2,289	95,610,338.11
Total	2,417	198,713,361.92

Head Office

Complaints Against	Settled	Probable	Possible	Remote	Total
HCPs*	-	-	-	95,570,338.11	95,570,338.11
Total	-	-	-	95,570,338.11	95,570,338.11

*Outcome of certain cases cannot be estimated due to pending court decision, thus, only total amount is indicated.

NCR

Complaints Against	Settled	Probable	Possible	Remote	Total
Employers	-	16,439,115.47	40,678,934.58	-	57,118,050.05
Total	-	16,439,115.47	40,678,934.58	-	57,118,050.05

CAR

Complaints Against	Settled	Probable	Possible	Remote	Total
HCPs*	-	40,000.00	-	-	40,000.00
Total	-	40,000.00	-	-	40,000.00

*Outcome of certain cases cannot be estimated due to pending court decision, thus, only total amount is indicated.

PRO II

Complaints Against	Settled	Probable	Possible	Remote	Total
Employers	1,629,554.84	1,081,470.60	-	-	1,081,470.60
Total	1,629,554.84	1,081,470.60	-	-	1,081,470.60

PRO III

Complaints Against	Settled	Probable	Possible	Remote	Total
Employers	-	35,260,701.13	-	-	35,260,701.13
Total	-	35,260,701.13	-	-	35,260,701.13

PRO IV-B

Complaints Against	Settled	Probable	Possible	Remote	Total
Employers	-	1,370,279.40	-	-	1,370,279.40
Total	-	1,370,279.40	-	-	1,370,279.40

PRO VI

Complaints Against	Settled	Probable	Possible	Remote	Total
Employers	-	-	1,152,695	-	1,152,695
Total	-	-	1,152,695	-	1,152,695

PRO VII

Complaints Against	Settled	Probable	Possible	Remote	Total
Employers	-	1,003,244.71	-	559,315.51	1,562,560.22
Total	-	1,003,244.71	-	559,315.51	1,562,560.22

PRO X

Complaints Against	Settled	Probable	Possible	Remote	Total
Employers	-	156,400.00	2,483,389.41	-	2,639,789.41
Total	-	156,400.00	2,483,389.41	-	2,639,789.41

PRO XI

Complaints Against	Settled	Probable	Possible	Remote	Total
Employers	-	-	136,535.33	2,333,502.67	2,470,038.00
Total	-	-	136,535.33	2,333,502.67	2,470,038.00

PRO XII

Complaints Against	Settled	Probable	Possible	Remote	Total
Employers	-	-	447,440.00	-	447,440.00
Total	-	-	447,440.00	-	447,440.00

34.14 Number and amount of Notice of Disallowances:

	No. of Disallowances	Amount	Estimated Outcome
Head Office	91	1,841,712,940.33	Probable
CAR	38	288,089,860.75	Probable
I	271	227,524,901.32	Probable
II	28	339,129,375.90	Probable
III	51	610,474,672.94	Probable
IV-A	68	285,331,163.63	Probable
IV-B	219	358,781,931.90	Probable
V	155	459,793,950.59	Probable
VI	129	598,080,882.36	Probable
VII	13	283,597,209.22	Probable
VIII	29	275,401,451.96	Probable
IX	45	253,054,131.53	Probable
X	78	471,336,385.04	Probable
XI	54	347,572,118.12	Probable
XII	8	56,600,689.75	Probable
CARAGA	151	408,040,850.98	Probable
BARMM	8	38,298,209.70	Probable

34.15 Number and amount of Outstanding Legal Cases Filed Against PhilHealth:

	No. of Disallowances	Amount	Estimated Outcome
Head Office	62	32,147,895.26	Remote
Total	62	32,147,895.26	

35. SUPPLEMENTARY TAX INFORMATION UNDER REVENUE REGULATION (RR) NO. 15 - 2010

The taxes, duties and licenses fees paid and accrued during the taxable year required under RR No. 15-2010 are as follows:

	2023	2022
Real estate tax, license and permit taxes	547,134	376,957
Taxes on compensation and benefits	393,008,075	355,210,350
Government Money Payments	84,079,046	61,063,635
Expanded withholding taxes	1324495769	1,679,376,992
Total	1,802130,024	2,096,027,934

Other Taxes, Duties and Licenses amounted to P547,134 and P376,957 for CYs 2023 and 2022, respectively, are taxes paid for real property taxes and other fees paid to regulatory entities.



BOARD OF DIRECTORS



Dr. Teodoro J. Herbosa
Secretary
Department of Health (DOH)



Emmanuel R. Ledesma, Jr.
President and Chief Executive Officer
Office of the President and Chief Executive Officer



Rexlon T. Gatchalian
Secretary
*Department of Social Welfare
and Development (DSWD)*



Bienvenido E. Laguesma
Secretary
*Department of Labor
and Employment (DOLE)*



Benjamin E. Diokno
Secretary
Department of Finance (DOF)



Amenah F. Pangandaman
Secretary
Department of Budget Management (DBM)



Atty. Lora L. Mangasar
Corporate Secretary
Office of the Corporate Secretary



Atty. Mauro Anthony B. Cabading, III
Corporate Legal Counsel
Office of the Corporate Legal Counsel



BGen. Marlene R. Padua, AFP (Ret.)

Board Member - Health Care Providers Sector

Education

Health Service Management, PhD in Nursing-University of the Philippine Baguio, University of the Philippines Manila • Master of Arts in Nursing-University of Asia and the Pacific • Pre-Nursing/Graduate Nurse/Bachelor of Science in Nursing-St. Louis University College of Arts and Science General Hospital School of Nursing- Philippine Women's University • Secondary Education-St. Louis University High School Baguio City • Basic Education-St. Louis University Elementary School Baguio City

Before her stint as a member of the PhilHealth Board as a representative for the Health Care Provider Sector, BGen. Padua served as Chair of the Advisory Council of the PNP Health Service PATROL Plan of the Philippine National Police Health Service in 2017. BGen. Padua also served as Dean of the College of Nursing at Arellano University in Pasig in 2012. She was a faculty member and research coordinator at Florentino Cayco Memorial School of Graduate Studies in 2009. BGen. Padua also served as Chief Nurse in the Nurse Corps. of the Armed Forces of the Philippines from 2004-2007; Chief Nurse of the Philippine Army from 2003-2004; Deputy Chief Nurse of the Philippine Navy from 2001-2003; Assistant Chief Nurse for Clinical Service Fort Bonifacio General Hospital in 1999-2000. BGen. Padua also served as Command Nurse of the 4th Infantry Division/Camp Evangelista Station Hospital in Cagayan De Oro from 1998-1999. She also served as Staff Nurse from 1987 to 1998 at the Coney Island Hospital, Brooklyn, New York, USA; Nurse- In-Charge from 1984 to 1986 at the Bonifacio Naval Station Dispensary and Chief Registrar from 1983 to 1984 and Head Nurse from 1973 to 1983 in the AFP Medical Center. With her prolific contributions to Nursing and health care, she handles the role of Philhealth's Health Care Provider Sector with ease and confidence.



Dr. Jack G. Arroyo

Board Member - Expert Panel

Dr. Jack G. Arroyo, Jr., M.D., B.Sc. is a pioneering and innovative medical professional with a diverse range of experiences. During his residency, he was elected President of the Glaucoma Research Foundation and upgraded the Eye Referral Center, the first comprehensive eye center in the Philippines. He also trained in refractive surgery under Dr. James J. Saiz in California. He established the first privately owned ophthalmological center in the Philippines, the American Eye Correction Center, which later became the American Eye Center. In May 1995, he performed the first LASIK procedure in Asia. Dr. Arroyo has had a significant impact on the field of ophthalmology in the Philippines. He founded the first sister society of the American Society of Cataract and Refractive Surgery in the world and the Philippine Society of Cataract and Refractive Surgery. He shared his expertise with different audiences through speaking engagements on Cataract and Refractive Surgery. With almost 20 years of practice, he has performed over 34,000 eye surgeries and has been recognized for his contributions to the field. In addition to his medical career, Dr. Arroyo has been involved in various other roles, including serving as an independent director on the Board of Directors of San Miguel Global Power Holdings Company, as a member of the Board of Directors of PhilHealth representing local chief executives and the self-employed sector, and as a founding member of an emergency care institution, Lifeline Arrows Inc. He has also been politically active, supporting campaigns of Congressmen and Senators in the Philippines. Dr. Arroyo's extensive experience and contributions in various fields and his role as Philhealth's board member highlight his leadership and commitment to making a difference in his profession.



Alejandro Labrado Cabading, CPA

Expert Panel

Education

BSC Major in Accountancy-Ateneo De Zamboanga 1980 -1984 • AB Political Science-University Western Mindanao State University • Claret School of Zamboanga City • Canelar Elementary School.

Mr. Cabading was a proprietor at the Alejandro L. Cabading Accounting Office from 1995 before he was appointed member of the PhilHealth Board expert panel. Mr. Cabading served as a college instructor at Ateneo De Zamboanga University from 1994 to 1997 and at Zamboanga Arturo Eustaquio College from 1990 to 1994. He also served as a Branch Accountant at Marsman Inc., Pharmaceutical Co. from 1992 to 1994 and was a Production Assistant at the Garment Export Co., Creative Lines Inc. from May 1990 to Dec 1990. Mr. Cabading is a member of the Philippine Institute of Certified Public Accountants and Employers' Confederation of the Philippines. His experience and expertise make him an integral part of the Philhealth's board.



Dr. Rene Elias C. Lopez

Board Member - Employers Group

Leadership Overview

Dr. Rene Elias C. Lopez is an incumbent member of the Board of Directors of the Philippine National Oil Company Renewables Corporation. His professional affiliations include being a Member of the Philippine Medical Association and the Davao Medical Society and a Consultant in General Surgery at Davao Doctors Hospital, San Pedro Hospital, Medical Mission Group Hospital, Inc., and Brokenshire Hospital.

Public Health Experience

Dr. Lopez was a Medical Officer at the Department of Surgery of Davao Medical Center from 1991 to 1995. In 1996, he became a Medical Specialist/Regional Medical Training Officer and Regional Coordinator at the Department of Health Regional Office XI, which lasted for six years. From 2009 to 2019, Dr. Lopez was a Quarantine Medical Officer at the Bureau of Quarantine, Port of Davao, DOH.

With true leadership and expertise in Public Health, Dr. Lopez handles his role as Philhealth's board member with confidence and steadfast commitment.



Dr. Jason Ronald N. Valdez

Board Member - Direct Contributors Group

Dr. Jason Valdez is a graduate of De La Salle University's accelerated medical program, attaining his degree in Human Biology from DLSU-Manila and his doctor of medicine degree from DLSU-Health Sciences Campus. He passed the physician licensure examination in the Philippines in 2007 and the U.S. Medical Licensure Examination in 2009. Adding a feather in his cap, Dr. Valdez pursued further studies at the University of the Philippines, earning his Master's degree in Public Health. He also completed the academic units in Hospital Administration and has served in various positions in the healthcare industry, not merely in the clinical setting, but also the management and operation of health institutions. He is a member of the International Maritime Health Association and has been involved in the industry for 8 years. His advocacy is a holistic health and wellness promotion in the career of every seafarer while on board and ashore or both. He aims to benefit both the seafarer, with a long and healthy seafaring career, and the ship owners, with an increasingly productive workforce.

His dedication to building a productive workforce has helped him achieve a lasting impression in Philhealth's Direct Contributors Group.



EXECUTIVE OFFICERS



EXECUTIVE OFFICERS

EMMANUEL R. LEDESMA, JR.
President and Chief Executive Officer

ATTY. ELI DINO D. SANTOS
Executive Vice President and Chief Operating Officer



SENIOR VICE PRESIDENTS

*From left to right: **SVP Renato L. Limsiaco, Jr.**, Fund Management Sector | **SVP Dennis S. Mas**, Management Services Sector | **SVP Atty. Jose Mari F. Tolentino**, Legal Sector*



SENIOR VICE PRESIDENTS

*From left to right: **Acting SVP and Head Nerissa R. Santiago**, OSVP for Actuarial Services and Risk Management Sector, Project Management Team for Risk Management | **SVP Israel Francis A. Pargas, MD**, Health Finance Policy Sector | **Acting SVP Clementine A. Bautista, MD**, PhilHealth UHC Surge Team | **SVP-CIO Engr. Jovita V. Aragona**, Information Management Sector*



VICE PRESIDENTS

*From left to right: **VP Francisco Z. Soria, Jr., MD**, Quality Assurance Group | **VP Lemuel T. Untalan**, Member Management Group | **VP Atty. Jay R. Villegas**, Arbitration Office | **Acting VP and SM Rey T. Baleña**, Corporate Affairs Group and Corporate Communications Department | **Acting VP Yolanda M. De Leon**, Office of the Actuary*



VICE PRESIDENTS

*From left to right: **Acting VP Anne Marie C. San Andres**, Internal Audit Group | **VP Walter R. Bacareza**, Area 1&2 | **VP Atty. Alfredo B. Pineda, II**, Area III | **VP Gregorio C. Rulloda**, Area IV | **VP Bernadette C. Lico, MD**, PRO-NCR*



SENIOR MANAGERS

*From left to right: **SM Atty. Alex B. Cañaveral**, Financial Audit Department | **SM Atty. Rogelio A. Pocallan, Jr.**, Internal Legal Department | **SM Atty. Andrei C. Samson**, Prosecution Department*



SENIOR MANAGERS

*From left to right: **SM Edwin M. Oriña**, Corporate Planning Department | **Acting SM Nadine B. Navarro**, Corporate Marketing Department | **SM Arsenia B. Torres**, Social Health Insurance Academy | **SM Atty. Francis Jay E. Remigio**, Physical Resources and Infrastructure Department*



SENIOR MANAGERS

*From left to right: **Acting SM Atty. Roy A. Maranan**, Protest and Appeals Review Department | **SM Atty. Ernesto P. Barbado, Jr.**, Fact Finding Investigation and Enforcement | **SM Atty. Michael Troy A. Polintan**, Operation Audit Department*



SENIOR MANAGERS

*From left to right: **SM Rex Paul R. Recoter**, Formal Sector - Member Management Group | **SM Melanie C. Santillan, MD**, Benefits Development and Research Department | **RVP William O. Chavez**, Special Programs Department | **SM Cheryl W. Peña**, Human Resource Department*



SENIOR MANAGERS

*From left to right: **SM Rizza Majella L. Herrera, MD**, Accreditation Department | **SM Cherie Carmen B. Divina**, Comptrollership Department | **SM Atty. Ma. Emily P. Roque**, Treasury Department | **SM Lambert S. David, MD**, Standards and Monitoring Department*



SENIOR MANAGERS

*From left to right: **Acting SM Annie Rose B. Gaffud**, Corporate Information Security Department | **Acting SM Nelson S. De Vera**, Information Technology and Management Department | **Acting VP and SM Henry V. Almanon, PRO-III**, Organization and Systems Development Office | **Acting SM Leilani Mae S. Formadero**, International and Local Engagement Department*



REGIONAL VICE PRESIDENTS

From left to right: RVP Dominga A. Gadgad, PRO-CAR | RVP Dennis B. Adre, PRO-II | RVP BGen. Llewellyn R. Binasoy (Ret.), PRO-II | RVP Danilo M. Reyes, MD, PRO-IVA | RVP Atty. Jerry F. Ibay, PRO-IVB



REGIONAL VICE PRESIDENTS

*From left to right: **RVP Alberto C. Manduriao, PRO-V** | **Acting RVP Janet A. Monteverde, PRO-VI** (not in photo) | **RVP Marjorie A. Cabrieto, PRO-VII** | **RVP Ronald S. Jabay, PRO-VIII** | **RVP Paolo Johann C. Perez, PRO-IX***



REGIONAL VICE PRESIDENTS

*From left to right: **Acting RVP Delio A. Aseron, II**, PRO-X | **RVP Harvey L. Carcedo**, PRO-XI | **RVP Hector Zenon P. Malate, MD**, PRO-XII | **RVP Michael Jibson C. Hernandez**, PRO-CARAGA | **RVP Masiding M. Alonto, Jr.**, PRO-BARMM*



BRANCH MANAGERS

*From left to right: **Acting BM Arlan M. Granali**, PRO III-Branch B | **Acting BM Rowena S. Zabat-San Mateo, MD**, PRO III-Branch A | **BM Raul B. Tuquero, MD**, PRO NCR South Branch | **Acting BM Mae R. Dizon**, PRO NCR North Branch | **BM Atty. Recto M. Panti**, PRO NCR Central Branch*

DIRECTORY

Head Office

709 Citystate Centre Building, Shaw Boulevard
Oranbo, Pasig City
(02) 866-225-88

PRO CAR - Baguio

Regional Office
SNOBT, Inc. Building, No. 19 Leonard Wood Road,
2600 Baguio City
(074) 444-5345; 444-8361; 444-9862
car@philhealth.gov.ph

PRO I - Dagupan

Regional Office
Akia Building, Old De Venecia Highway
Dagupan City, Pangasinan
(075) 515-1111
philhealthregion1@gmail.com

PRO II - Tuguegarao

Regional Office
The Builder's Place, Del Rosario St. Tuguegarao City,
Cagayan 3500
(078) 255-1342; (0917) 8357544
info.pro2@philhealth.gov.ph

PRO III-A - San Fernando

Regional Office
G/F PhilHealth Bldg., Lazatin Blvd., San Agustin,
San Fernando City, Pampanga C-2000
(045) 961-1977; (045) 961-3949 loc. 4330

PRO III-B - Malolos

Regional Office
The Cabanas Mall of Malolos 2nd and 3rd floors N4
Bldg.
Km. 44/45 MacArthur Highway Longos, Malolos
City, Bulacan
(044) 796-1559; (044) 796-1560 loc. 4400

PRO NCR

Regional Office
18th to 19th Flr (Penthouse), Panorama
Technocenter,
1029 Epifanio de los Santos Ave, Proj. 7, Quezon
City

PRO NCR North - Manila

Branch Office
Ten Commandments Building, 689 Rizal Avenue
Extension, Gracepark, Caloocan City

Local Health Insurance Office

Manila

JARS Building, 1810 J.P. Laurel St., San Miguel,
Manila
(02) 8521-7321
so.manila@philhealth.gov.ph

Caloocan

3rd Floor, Victory Central Mall, Rizal Avenue
Extension, Gracepark, Caloocan City
(02) 8365-0464

Valenzuela

4/F Puregold, Paso de Blas Road Cor. East Service
Road North, Valenzuela City
(02) 8277-4863

Mandaluyong

3rd Floor, 500 Shaw Zentrum Mall, Shaw Boulevard,
Mandaluyong City
(02) 8532-0449

PRO NCR Central - Quezon City

Branch Office
Corporate 145 Building, 145 Mother Ignacia St.,
Brgy. South Triangle, Quezon City, 1103

Local Health Insurance Office

South Triangle, Quezon City

Lower Ground Floor, Corporate Building 145
Mother Ignacia, Barangay South Triangle, Quezon
City
(02) 8332-1557

Fairview, Quezon City

OWS Bldg., Blk 237, Lot 19, Neopolitan IV, Britanny
Subd.,
Barangay Pasong Putik, Quezon City, 1118
(02) 8356-7461

Rizal

Fibertex Building, Corner Don Mariano Subdivision,
Ortigas Extension, San Juan, Cainta, Rizal 1900
(02) 8997-8377

PRO NCR South - Pasig

Branch Office
8007 Pioneer Street, Kapitolyo, Pasig City

Local Health Insurance Office

Pasig

Ground Floor, Iriz One Corporate Centre
35 Meralco Avenue, Cor. Segundo Street, San
Antonio, Pasig City

Taguig

Global Satellite Office, 7th Floor
SM Aura Tower, Bonifacio Global City, Taguig

Makati

2326 Chino Roces Ave., Extension, Brgy. Magallanes,
Makati City

Las Piñas

Mega Accent Bldg. 479 Alabang-Zapote Road, Brgy.
Almanza Uno, Las Piñas City

Parañaque

HRDC Bldg., KM 16 Acsie rd. cor. West Service
Road, Brgy. Marcelo Green, Parañaque City

PRO IV-A - Lucena

Regional Office
Lucena Grand Central Terminal, Brgy. Ilayang Dupay,
Lucena City
(042) 373-7554
member.pro4a@philhealth.gov.ph

PRO IV-B - Batangas

Regional Office
Xentro Mall Batangas City, Diversion Road, Brgy.
Alangilan,
Batangas City, Batangas
(043) 723-8822

PRO V - Legazpi

Regional Office
ANST IV Bldg., Benny S. Imperial Drive,
Legazpi City, Albay
Healthline: (052) 481-5596

PRO VI - Iloilo

Regional Office
Gaisano Capital Building, Luna Street, Lapaz, Iloilo
City
(033) 501-9160 to 62 loc. 100/102
region6@philhealth.gov.ph

PRO VII - Cebu

Regional Office
7th and 8th Floor, Skytower, N. Escario Street corner
Acacia Street,
Cebu City 6000
(032) 233-3281; (032) 233-3284

PRO VIII - Tacloban

Regional Office
Brgy. 24, P. Burgos Street, Tacloban City, Leyte
(053)325-3563; (053) 523-1195 (Fax)
info.pro8@philhealth.gov.ph

PRO IX - Zamboanga

Regional Office
BGIDC Corporate Center, Gov. Lim Ave.,
Zamboanga City
(062) 992-2739 (fax)
region9@philhealth.gov.ph

PRO X - Cagayan De Oro

Regional Office
8F Gateway Tower 2, Limketkai Center,
C.M. Recto Avenue, Cagayan de Oro City
(088) 859-0225
region10@philhealth.gov.ph

PRO XI - Davao

Regional Office
Valgosons Bldg., Bolton Ext., Poblacion District,
Davao City
(082) 295-2133 loc. 6314 / (082) 295-0151
region11@philhealth.gov.ph;
pro11.orvp@gmail.com

PRO XII - Koronadal

Regional Office
CSA I Building Cor. Zulueta Street, General Santos
Drive, Koronadal City
(083) 228-9731 to 34 (fax); (083) 228-4733
region12@philhealth.gov.ph
admin.pro12@philhealth.gov.ph

PRO CARAGA - Butuan

Regional Office
Lynzee's Building, 766 J. Rosales Avenue,
Butuan City
(085) 342-0900; (085) 816-0019; (085) 225-7026
loc. 101-103
caraga@philhealth.gov.ph

PRO BARMM - Marawi

Regional Office
Kouzbari Business Complex, Alibin Talib Street, New
Capitol HTs,
Marawi Poblacion, Marawi City
pro.armm@philhealth.gov.ph | phic_armm@yahoo.com

PhilHealth Action Center

New Hotline: (02) 866-225-88

Available 24/7 including weekends and holidays
www.philhealth.gov.ph

Mobile Hotlines: Call and text (24/7)

Smart: 0998-857-2957; 0968-865-4670 Globe:
0917-127-5987; 0917-110-9812

To request for callback via preferred mobile hotline, text:
"PHICallback [space] Mobile Number to be called
[space] details of your concern"

Callback schedule is from 8 am - 8 pm, 12 hours by 7
days, including weekends and holidays. Ask agent for
details to make sure the callback is from PhilHealth.

 actioncenter@philhealth.gov.ph

 PhilHealthofficial

 teamphilhealth

“A Well-equipped Workforce is the Cornerstone of Exceptional Service”

PhilHealth calls for the public's cooperation to curtail fraud and unethical acts done towards the National Health Insurance Fund. We encourage whistleblowers with any information pertaining to the Corporation, its officials and personnel to contact us through:

whistleblower@philhealth.gov.ph; actioncenter@philhealth.gov.ph; opceo@philhealth.gov.ph

or by mail addressed to the **Office of the Corporate Secretary Room 1711, 17th Floor Citystate Centre, 709 Shaw Boulevard, Pasig City.**

Complete confidentiality of reports as well as protection provided by law will be given to all whistleblowers.



